



AfroCentric
GROUP

Healthier Together

YEAR ENDED DECEMBER

2025

INTEGRATED ANNUAL REPORT

About AfroCentric

AfroCentric Group (**the Group**) is a Broad-Based Black Economic Empowerment (**B-BBEE**) level 1 investment holding company listed on the Johannesburg Stock Exchange (**JSE**). The Group is focused on advancing access to affordable healthcare across Southern Africa. Through a diversified portfolio of subsidiaries, the Group delivers integrated health management solutions, including medical scheme administration, managed care, medicine provision, wellness services and digital health innovations.

The Group operates across public and private sector environments and remains focused on strengthening operational execution, leveraging technology and clinical innovation, and building partnerships that support inclusive, value-based healthcare.

Sanlam Life Insurance Limited holds a controlling stake in the AfroCentric Group, fostering synergies and creating opportunities for both entities.

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Creating systems value (our integrated report at a glance)

In a world defined by deep interconnections, value no longer belongs only within the walls of an organisation. For AfroCentric, this understanding resonates with our purpose in healthcare. We do not optimise isolated parts of the system; we work across funding, delivery, access and outcomes to strengthen the system as a whole. This means asking not only what value we create for our stakeholders, but how our decisions contribute to the resilience and wellbeing of the healthcare system that sustains communities, partners and the economy.

Why we exist (see page 18)

We exist to enhance quality of life by transforming healthcare and strengthening the systems in which health, wellbeing and economic participation take place.

South Africa's healthcare system is under growing strain. Rising costs, increasing chronic disease, affordability pressures, regulatory and National Health Insurance (NHI) uncertainty, and heightened digital and cybersecurity risks are placing pressure on health outcomes, social equity and economic resilience (see page 31). These realities reinforce the need for a more integrated, preventative and outcomes-led approach to care – one that improves access, keeps people healthier for longer and uses resources more effectively across the system.

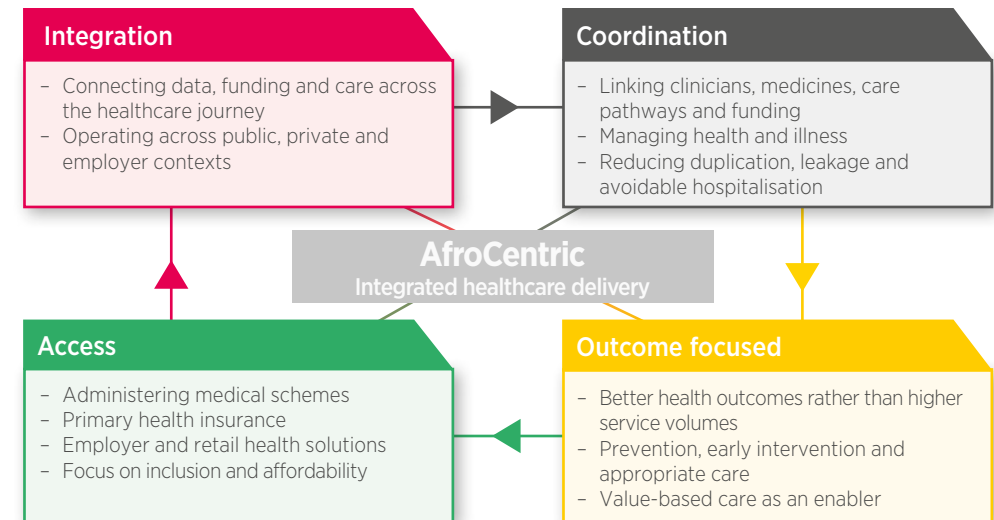
Why we are uniquely positioned (see page 21)

Our integrated position across the healthcare journey gives us visibility across members, clinicians, medicines, claims and underlying risk drivers. Combined with our Level 1 B-BBEE status, transformed ownership and long-term partnership with Sanlam, we are well-positioned to connect data, funding and clinical decision-making, at scale.

Our experience in delivering large-scale public-private healthcare programmes, particularly in chronic medicine access, further positions us to support coordinated, outcomes-led care in an evolving NHI environment.

Our integrated business model (see page 28)

We operate as an integrated healthcare delivery system that coordinates care, funding and delivery across the healthcare journey. Our businesses function as an interconnected platform – linking prevention, primary care, managed care, medicines and specialist services to improve access, reduce fragmentation and support better health outcomes.



These activities operate as an interconnected system — enabling, reinforcing and scaling integrated healthcare.

Our strategy for transforming healthcare (see page 58)

Our strategy is to strengthen and scale integrated healthcare delivery, shifting the system towards outcomes-led care that improves health, enhances affordability and supports long-term sustainability.



Building a winning health offering



Driving clinical innovation



Investing in data, digital, and technology



Evolving our culture and operating model

What this delivers

By strengthening the healthcare system rather than isolated components within it, AfroCentric contributes to outcomes that extend beyond organisational performance. A more integrated health system supports improved health outcomes and affordability, enables economic participation and strengthens social wellbeing, while recognising that environmental resilience underpins long-term human health.

 Value created/capital enhanced/metric increase
  Value eroded/capital reduced/metric decrease
  Value preservation/metric stable

How our performance reflects systems value

Following the year end change in 2024 from 30 June to 31 December, the comparative figures presented are in respect of the six months period ended 31 December 2024 and as such may not be comparable to the current reporting period.

Economic (see page 64)

Social (see page 76)

Environmental (see page 86)

System pressures and long-term choices		
Balancing healthcare affordability with scheme sustainability through disciplined utilisation management, clinical risk oversight and long-term investment in managed care and system capability	Balancing scale, access and coordination to support equitable healthcare delivery across public and private settings within a transforming system	Balancing uninterrupted healthcare service delivery with responsible resource use and environmental resilience
Organisational indicators reflecting system impact		
 94.0% revenue increase (2024: 51.8% revenue decrease)  82.3% operating profit increase (2024: 63.8% decrease)  (64.8%) return on equity (2024: 3.6% decrease)  52.1% weighted scheme solvency (2024: 54.8%)	 4.09 million lives under management (2024: 4.08 million)  8.6% average scheme contribution increase (2024: 11.27%)  Level 1 B-BBEE status (2024: Level 1)  18.3 million scripts dispensed, of which 93% were for the public sector (2024: 9.3 million; 90% public sector)  5 707 Group employees (2024: 5 899)	 72 251 kl water consumption (2024: 24 880 kl)  8 096 MWh electricity consumption (2024: 4 706 MWh)  708 MWh self-generated electricity – solar (2024: 247 MWh)  12 671 litres of diesel (used in generators) (2024: 8 512 litres)

Outlook

AfroCentric enters 2026 in a more complex and evolving healthcare environment that requires disciplined execution and continued refinement of our operating model. While the strategic intent underpinning Vision 2030 remains intact, management’s near-term focus is on strengthening the Group’s foundations while continuing to build the capabilities that differentiate our long-term strategy. We will deepen collaboration with partners, invest selectively in capabilities that support value-based care, and respond with agility to regulatory and market developments shaping the sector. Through this approach, AfroCentric remains committed to contributing to a more integrated, affordable and sustainable healthcare system, supported by our diversified healthcare assets, strong partnerships and prudent capital management.

About this report

Our integrated report provides information about AfroCentric for the year ended 31 December 2025.

This report is intended for **current and prospective investors and stakeholders interested in our Group.**

It provides information on our governance, material risks and opportunities, performance against strategy, societal impact, and future prospects.

Integrated thinking

Against the backdrop of a particularly challenging year, integrated thinking proved more critical than ever, equipping the Group to assess the interconnected impacts of strategic decisions across all capitals. This report has accordingly been prepared to deliver balanced, reliable, transparent and relevant information to providers of financial capital and other stakeholders, demonstrating how the Group applied integrated thinking to execute its strategy, manage risk and navigate trade-offs, while actively seeking to create and preserve value and minimise value erosion through its chosen business model.

Central to this approach is a deepening focus on **systems value**, recognising that value creation extends well beyond organisational performance to encompass the resilience and sustainability of the broader systems in which we operate. As the Group matures its integrated thinking practices and progresses toward full alignment with King V™, greater focus is being placed on how strategic decisions, partnerships and trade-offs work together to strengthen the broader environment, and, most critically, the healthcare ecosystem the Group is committed to improving.

By considering the relationships between the capitals we use and affect, and the trade-offs inherent in our strategic choices, we manage our resources to provide integrated healthcare solutions for our clients and improve access to sustainable, affordable and high-quality healthcare for their members. These considerations guided key strategic decisions throughout the year, including adjustments to our operating model and strategic priorities, which are further discussed in the Chairman's and Chief Executive Officer's (CEO's) reviews on pages 6 and 54, respectively, as well as in the strategy and strategic trade-offs sections on page 61.

Consistent with the principles of integrated thinking and conciseness, the scope of this report has been refined to sharpen its narrative focus on value creation, preservation and erosion. While it continues to present our material environmental, social and governance (ESG) information within an integrated thinking framework, certain detailed compliance disclosures have been moved to supplementary publications. The full governance report, full remuneration report and notice of the annual general meeting are now presented separately, allowing this report to sustain a clear value-creation narrative while ensuring that comprehensive compliance information remains fully accessible to stakeholders.

Materiality

This integrated report discloses information about matters that substantively affect our ability to create value over the short (12 months), medium (three years) and long term (five years and beyond) (refer to page 46 for more detail).

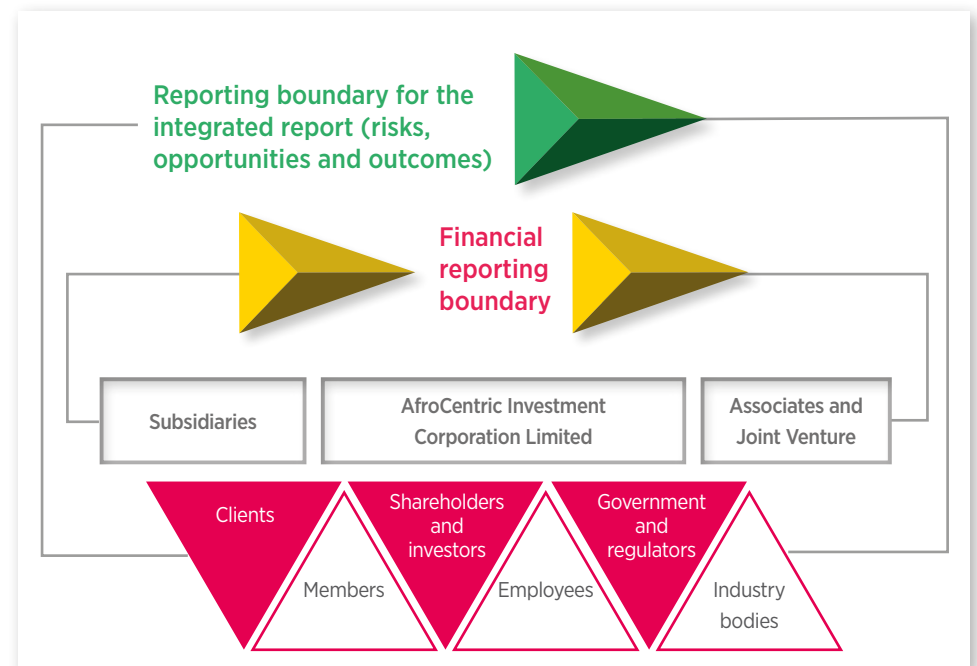
To demonstrate our commitment to **sustainable development**, we have included Sustainable Development Goals (SDGs) – those that relate to our strategy and to which we can most significantly contribute. However, based on our strategic intent, our primary contribution is to **SDG 3 – Good Health and Wellbeing – specifically targeting 3.8.**



Reporting scope and boundary

During the prior year, AfroCentric transitioned its financial year-end from 30 June to 31 December to align with Sanlam's reporting cycle. This report therefore covers the period from 1 January 2025 to 31 December 2025. Material events post year-end up until 31 March 2026, when the Board approved this report, are also included. Our reporting approach in terms of scope, boundary, and measurement methods remains materially consistent with prior reports. There are no restatements in the Annual Financial Statements.

The reporting boundary for this integrated report is informed by the Group's financial reporting boundary, as well as reflecting significant ESG risks, opportunities and outcomes, including those arising from our interactions with key external entities and stakeholders. This approach ensures that the report captures the matters that materially influence AfroCentric's ability to create and preserve value or those that could lead to value erosion over the short, medium and long term, in line with the principles of integrated reporting.



Navigating our report

Our strategic focus areas



Our capitals



Our suite of reports

Integrated report

Date of issue
8 April 2026

What's disclosed in this report

Our integrated report is our primary report and is addressed to our providers of capital and other interested stakeholders. It details the Group's strategy and our performance against it, including demonstrating the Group's journey in relation to value creation, preservation and erosion.

Annual financial statements

Date of issue
4 March 2026

What's disclosed in this report

The Group's annual financial statements (**AFS**) provide comprehensive information pertaining to the Group's audited financial statements, prepared in accordance with IFRS® Accounting Standards.

Governance and remuneration report

Date of issue
8 April 2026

What's disclosed in this report

This report contains detailed statutory, governance, remuneration, tax and shareholder-related disclosures that support compliance, transparency and accountability. It includes the full governance report, the full remuneration report, the Group's tax disclosures and the notice of the annual general meeting, together with other information relevant to shareholder engagement and oversight.

Key regulatory and reporting frameworks

- Integrated Reporting Framework
- JSE Listings Requirements
- JSE Sustainability Disclosure Guidance
- King IV™ Report on Corporate Governance for South Africa, 2016 (**King IV™**)¹
- King V™ Report on Corporate Governance for South Africa, 2025 (**King V™**)¹
- Companies Act 71 of 2008, as amended (**Companies Act**)
- The South African Institute of Chartered Accountants' Financial Reporting Guides, as issued by the Accounting Practices committee
- Financial Pronouncements as issued by the Financial Reporting Standards Council
- International Financial Reporting Standards (**IFRS® Accounting Standards**)

Assurance

- Management and governance oversight
- Board oversight and approval
- B-BBEE verification

Key regulatory and reporting frameworks

- Integrated Reporting Framework
- JSE Listings Requirements
- King IV™
- King V™
- Companies Act
- The South African Institute of Chartered Accountants' Financial Reporting Guides, as issued by the Accounting Practices committee
- Financial Pronouncements as issued by the Financial Reporting Standards Council
- IFRS® Accounting Standards

Assurance

- Internal controls
- Management and governance oversight
- Internal audit
- Unmodified external audit opinion (KPMG Inc.)

Key regulatory and reporting frameworks

- King IV™
- King V™
- Companies Act 71 of 2008, as amended
- JSE Listings Requirements
- JSE Sustainability Disclosure Guidance (where applicable)
- Remuneration governance requirements and best practice guidelines

Assurance

- Management and governance oversight
- Board and committee oversight
- Independent assurance where required or appropriate

Available @ www.afrocentric.za.com

¹ Initiatives are underway to align with King V™.

Outlook

Outlook information is considered to be that which details the challenges, opportunities and uncertainties we are likely to encounter in pursuing our strategy, as well as the potential implications for our business model and future performance. Outlook information can be found throughout this report; however, the majority of this information is detailed in the following sections:

Chairman's review

Page 6

CEO's review

Page 54

Our performance

Page 63

Forward-looking statements

Certain statements in this document may constitute forward-looking statements. Such statements involve known and unknown risks, uncertainties and other important factors that could cause the actual performance or achievements of AfroCentric and its subsidiaries to materially differ from future results, performance or achievements expressed or implied by such forward-looking statements.

The Group undertakes no obligation to update publicly or release any revisions to these forward-looking statements to reflect events or circumstances after publication of this report or to reflect the occurrence of anticipated events. These have not been reviewed or reported on by the Group's external auditors.

Process disclosure and assurance approach

Our integrated reporting process began with reviewing the material matters for reporting. Content gathering for the report included engagements across the business and at the Board level and drawing from Board and Board committee submissions. We apply a combined assurance approach to the Group's operations. Therefore, internal controls, management assurance, compliance and internal audit reviews, as well as the services of independent external assurance providers, support the accuracy of the disclosures within our published reports. The executive responsible for the report is our Group CFO. In line with their mandate, the integrated report is reviewed and recommended to the Board for approval by the Audit and Risk committee. The Board takes final responsibility for approving the Group's external disclosures.

Our material matters

Page 46

Our strategy and approach

Page 58

Chief Financial Officer's (CFO's) review

Page 64

Board approval

The Board acknowledges its responsibility for the integrity of this report. The report addresses all material matters, offering a balanced perspective on the Group's strategy and its relationship to AfroCentric's ability to create and preserve value, while guarding against value erosion, in the short, medium, and long term. The report addresses the use of and effects on the capitals and how the availability of these capitals impacts the Group's strategy and business model. Furthermore, we believe this report demonstrates how the Group seeks to fulfil its purpose and, in this way, creates sustainable value for all its stakeholders. Our opinion is that this integrated report presents a fair and balanced view of AfroCentric's performance and outlook. We, as the Board, believe that this report has been prepared in accordance with the Integrated Reporting Framework.

This report was approved for release on 31 March 2026.

Prof. Anna Mokgokong Chairman Non-executive Director	Bruno Fernandes Lead Independent Non-executive Director
Paul Hanratty Non-executive Director	Alice le Roux Independent Non-executive Director
Joe Madungandaba Non-executive Director	Kanyisa Mkhize Non-executive Director
Charlotte Mokoena Independent Non-executive Director	Karabo Morule Independent Non-executive Director
Dr Nkateko Munisi Non-executive Director	Thato Moloele Group CFO
Gerald van Wyk Group CEO	

Feedback

We are committed to communicating effectively with our stakeholders, and we value feedback on this report. Any questions or requests for additional information relating to our report can be directed to Gift Dlamini at investor-relations@afrocentrichealth.com or telephonically at +27 (0)11 671 2475.

Who we are



Chairman's review

AfroCentric operates at the intersection of healthcare need, system reform and long-term stewardship. As pressures on affordability, access and outcomes continue to intensify, the Group remains focused on shaping a healthcare ecosystem that is resilient, inclusive and capable of delivering sustainable value over time.

Prof. Anna Mokgokong
Chairman



The year under review unfolded within a demanding environment for both the healthcare sector and the Group. Alongside ongoing system pressures, AfroCentric navigated a number of operational and strategic developments that required careful stewardship, disciplined governance and clear leadership. In this context, the Board remained firmly guided by the Group's long-term strategic direction and purpose.

Navigating change within a dynamic healthcare landscape

The healthcare environment is defined by complexity, pressure and ongoing transition. Economic constraints, rising healthcare demand, affordability challenges, utilisation trends and regulatory uncertainty continue to test the resilience of healthcare systems and funding models.

Further detail on the external drivers shaping the Group's operating environment is set out in the Operating context section on page 31.

Against this backdrop, AfroCentric remains firmly committed to the principles underpinning universal healthcare and the progressive expansion of access to quality, affordable care. In a country marked by deep inequality in health outcomes, this commitment carries both social responsibility and long-term economic importance. We believe that improving access requires a healthcare system that is sustainable, outcomes-led and capable of delivering care at scale. This conviction continues to guide the Group's strategic direction and contribution to healthcare reform.

In line with this commitment, the Board remains focused on constructive engagement with policymakers, regulators and industry stakeholders. We continue to advocate for a collaborative public-private healthcare framework that protects continuity of care, leverages existing capacity and supports sustainable reform over time. AfroCentric's integrated healthcare capabilities, scale and experience position the Group to contribute meaningfully to this dialogue while remaining agile in the face of change.

Navigating the Bonitas developments with governance and care

The Board has given careful and sustained attention to developments relating to the Bonitas Medical Scheme (**Bonitas**) during the year. Medscheme has maintained a long-standing relationship with Bonitas over several decades, during which the scheme has grown into one of the country's largest medical schemes, supported by Medscheme's extensive administrative and managed care capabilities.



A key area of focus has been the continued sharpening of the Group's strategic positioning. The Board supports the deliberate prioritisation of managed healthcare as the core engine of value creation, underpinned by clinical expertise, data-driven insights and integrated care models.



Over this period, the value delivered to Bonitas has been measurable across solvency, affordability, growth, retention, service levels, governance, and clinical outcomes. Notably, Medscheme played a primary role in supporting Bonitas' solvency recovery through disciplined managed care interventions and evidence-based models — a key indicator of long-term sustainability for members. Service levels have also remained consistently high, even in increasingly complex delivery environments, with no service-level agreement breaches over an extended period. This performance contributed to Bonitas being recognised as the top medical aid scheme for excellence in customer experience in the Ask Afrika Orange Index Awards in both 2024 and 2025.

During the year, Bonitas conducted a request for proposal (**RFP**) process for administration and managed care services and awarded the contracts to third parties, while the Council for Medical Schemes (**CMS**) initiated a forensic investigation following earlier regulatory enquiries and media reporting concerning procurement processes. Medscheme launched an application in the High Court to prevent the award of the contracts or their implementation pending the conclusion of the CMS forensic investigation. These developments have understandably attracted significant attention across the healthcare sector.

The Board's position is clear: governance, fairness and due process are fundamental to the integrity and sustainability of the healthcare system. Where regulatory investigations and legal processes are underway, they must be allowed to proceed through the appropriate channels and reach their conclusions in accordance with the rule of law.

At the same time, the Board's responsibility to members and clients remains paramount. Until contractual arrangements change, the continued delivery of services to scheme members must remain professional, responsible and uninterrupted, with any transition managed to protect continuity of care and minimise disruption.

Recognising the potential implications of these developments for the organisation, the Board has directed management to implement a structured programme to reposition the business for revised scale. The programme focuses on stabilising the organisation, resetting the cost base, simplifying the portfolio and redesigning the operating model to ensure long-term sustainability.

Importantly, the Board acknowledges the human impact of these developments. We extend our sincere appreciation to employees who have supported the Bonitas relationship over many years with professionalism and care, including those who may be affected by the potential loss of this contract. Their contribution has been instrumental to the value delivered and is deeply valued by the Group.

While the circumstances require careful stewardship and disciplined execution, the Board remains confident in the strength of the Group's core healthcare capabilities and its continued role in supporting millions of beneficiaries across multiple schemes.

Sharpening strategic focus through disciplined governance

During the year, the Board maintained a strong focus on strategic execution, governance discipline and organisational resilience. Building on the strategic refresh undertaken in the prior period, attention shifted decisively from strategy articulation to delivery, supported by strengthened oversight structures and clearer accountability.

In parallel, the Group undertook selective disposals and prioritised partnerships to streamline its portfolio and ensure that capital and leadership attention are concentrated where AfroCentric can deliver the greatest long-term impact. These actions reflect strategic discipline and a commitment to sustainability rather than short-term scale.

Central to this approach is the Group's partnership with Sanlam. Alignment of reporting cycles, operating models and strategic priorities has strengthened long-term stability while creating opportunities to deepen integration across healthcare funding, administration and care delivery. The Board continues to view this partnership as a cornerstone of AfroCentric's ability to deliver sustainable value over time.

Governance remains a defining strength of the Group. The Board and its committees exercised rigorous oversight across risk management, financial performance, regulatory compliance and ethical conduct. In an environment characterised by heightened scrutiny and complexity, this discipline is essential to maintaining stakeholder trust and safeguarding long-term value.



AfroCentric's ability to fulfil its purpose ultimately depends on its people. The Board continues to place significant emphasis on leadership depth, succession planning and the cultivation of a high-performance, values-driven culture – particularly in a demanding operating environment where skills, resilience and ethical conduct are essential.



Strengthening leadership and stewarding our people

The Board was strengthened during the year by the appointment of Ms Charlotte Mokoena in October 2025. Ms Mokoena also serves on the Nomination committee, Remuneration committee and the Social and Ethics committee. She brings extensive experience as a senior human resources executive, and her insight into people, culture and organisational effectiveness is already adding value to Board deliberations. The Board warmly welcomes her contribution to the Group's governance and long-term stewardship.

Ms Marinda Dippenaar resigned from the Board with effect from 6 March 2026. She also resigned from the Investment committee. The Board would like to thank her for her contribution.

Ensuring that AfroCentric remains a place where people can thrive is both a moral responsibility and a strategic imperative. Ongoing attention to employee wellbeing, engagement and ethical conduct remains essential, particularly in a demanding operating environment.

Upholding integrity through regulatory and legal stewardship

Trust remains foundational to the functioning of the healthcare system. During the year, CMS published the findings of the Section 59 Inquiry into aspects of fraud, waste and abuse detection across the industry. While the findings are not legally binding and did not result in sanctions, they contributed to important public discourse regarding fairness, transparency and procedural integrity.

The Board notes that Medscheme had already taken proactive steps prior to the publication of the report to strengthen governance controls, enhance oversight mechanisms and mitigate any potential risk of bias or unintended discrimination in its processes. These actions reflect the Group's ongoing commitment to ethical conduct, continuous improvement and regulatory alignment.

The Board also notes the resolution of the long-standing litigation involving Neil Harvey and Associates, which has spanned more than 18 years. The findings of the Appeal Tribunal affirm Medscheme's position and bring welcome closure to a matter that has required sustained legal and governance attention over an extended period. The case is being taken on review by NHA.

Positioning the Group for sustainable long-term value

Looking ahead, the healthcare environment is likely to remain complex and contested. Affordability pressures, regulatory developments, technological change, workforce constraints and increased utilisation will continue to shape the sector.

AfroCentric's long-term strategy remains clear. The Group is focused on strengthening integrated healthcare delivery, advancing clinical and digital innovation, deepening partnerships and operating with discipline and integrity.

While the near-term environment will require careful stewardship and disciplined execution, the Board remains confident in the strength of the Group's underlying capabilities and the scale of the healthcare platform that continues to serve more than three million beneficiaries across multiple schemes.

For more detailed strategic outlook information, please refer to our strategy and approach section on page 58.

I would like to thank my fellow Board members for their commitment, insight and stewardship. I also extend my appreciation to the executive team and all AfroCentric employees for their professionalism and dedication during a demanding year.

Finally, I thank our shareholders, clients, partners and stakeholders for their continued trust and engagement.

With a clear purpose, strong governance and a disciplined focus on execution, AfroCentric remains well positioned to navigate the period ahead and contribute meaningfully to the evolution of healthcare in Southern Africa.



Prof. Anna Mokgokong

Chairman

Who governs us

Corporate governance plays a critical role in enabling AfroCentric to create sustainable value over the short, medium and long term while protecting against value erosion. Our governance approach supports effective decision-making, responsible leadership, and oversight of strategy, risks, and opportunities that influence our ability to deliver on our purpose and strategic objectives. Through ethical leadership, robust governance structures, and a culture of accountability, the Board ensures that the Group's strategy, performance, and prospects are considered in the context of the broader operating environment and the interests of our stakeholders.

Overview of critical corporate governance practices

Periodic review of the Group's governance framework, including the Group Delegation of Authority Framework

Risk appetite set and monitored

Annual review of Board leadership structure

Combined assurance approach

Annual Board and committee evaluations

Annual review of Board policies including the declaration of interest policy, ethics policy and Group Code of Conduct

MOI includes provisions for the protection of shareholder rights and the equitable treatment of shareholders

Safeguards are in place to monitor transactions between the Company and our significant shareholders

Annual review of the malus and clawback policy to align with prevailing market practices

Succession planning and rotation

Inclusive and balanced stakeholder engagement programme

Risk management through an Enterprise Risk Management (**ERM**) framework

Annual assessment of the Lead Independent Director

Board-approved policy on gifts and gratuities

No voting rights ceilings

Shareholders' right to call special meetings

Related-party transactions monitored and transparently disclosed

No supermajority provisions in governing documents

Training for the Board to enhance capabilities on an ongoing basis

Review the Group's remuneration policy

Evaluate Board members' interests and independence

Board leadership and governance structure

The Board is the highest governing authority of the Group and is responsible for corporate governance and setting strategic direction. It formulates the Group's strategy in line with the organisational vision and values and ensures that all business decisions and judgements are made with reasonable care, skill and diligence.

The Board's leadership and judgement direct the Group to sustainable growth. It acts in the best interests of the business and its stakeholders and is responsible to shareholders for creating and delivering sustainable shareholder value by managing the Group's businesses. In providing overall strategic direction, the Board ensures that management strikes an appropriate balance between long-term growth and short-term objectives.

As at 31 December 2025, AfroCentric has a unitary Board structure with two Executive Directors, six Non-executive Directors and four Independent Non-executive Directors. Marinda Dippenaar (Non-executive Director) resigned with effect from 6 March 2026. The Board's members have appropriate industry knowledge, qualifications and sufficient diverse experience to discharge their duties effectively.

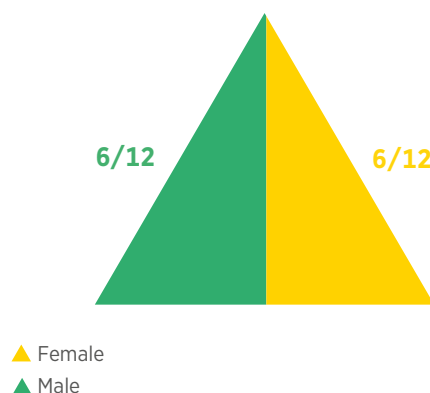
The roles of the Chairman and the CEO are separate. The Board is led by a Non-executive Chairman, while the operational management of the Group is the responsibility of the CEO. To strengthen good corporate governance, an Independent Lead Director has been appointed to ensure independence and objective oversight.

The Board Charter defines the Board's powers and responsibilities and is reviewed annually. The charter aligns substantially with the JSE Listings Requirements, the Companies Act and King IV™, and initiatives are underway to align the Group's practices with King V™.

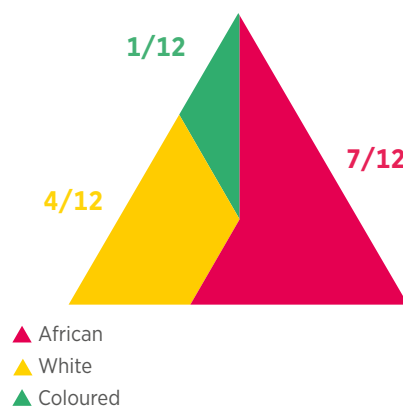
Board composition, independence and capability

As at 31 December 2025, AfroCentric's Board comprises a diverse group of directors whose collective experience and expertise support effective oversight of the Group's strategy and operations. Diversity criteria include gender, age, ethnicity and geographic background, as well as the skills and qualifications required to guide the organisation.

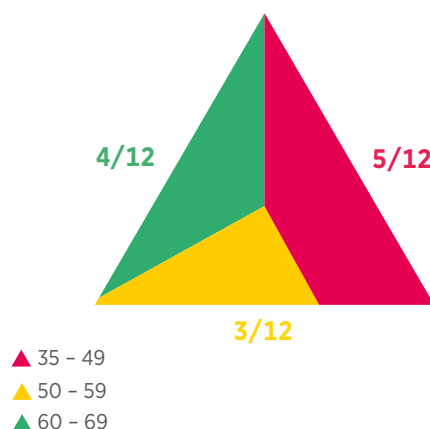
Gender – 2025



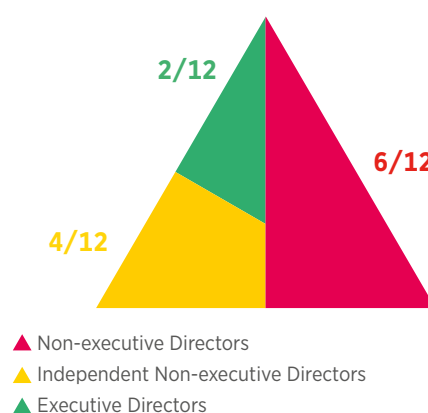
Diversity – 2025



Age – 2025



Independence – 2025



The Nomination committee reviews directors' independence in accordance with guidelines from King IV™, the Companies Act and the Board's constitutional documents. These assessments consider factors such as previous affiliations, financial interests and roles held both inside and outside the organisation to determine whether directors are able to exercise objective judgement.

The Company's diversity policy is available on the Company's website at <https://www.afrocentric.za.com/sustainability/>

In 2025, the Board found that all Independent Non-executive Directors remained independent in character, conduct and Board contributions. Maintaining this independence is essential to ensuring objective oversight and safeguarding stakeholders' interests.

The Board possesses a broad and complementary range of expertise that supports the Group's strategic ambitions. Key areas of experience across the Board include:

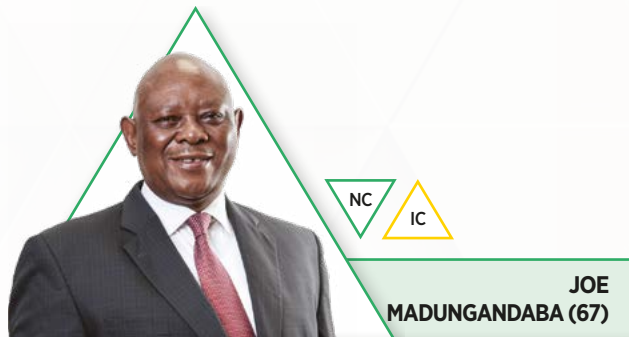
Strategy
Healthcare
Finance, accounting, auditing and advisory
Leadership
Business development
Corporate governance, legal and compliance
Investor relations
Human resources
Mergers and acquisitions
Investments
Risk management
Health technology
Stakeholder relations and communications
Sustainability and social impact
Innovation

This diverse skill set enables the Group to remain agile in a volatile economic environment. The Board views its current mix of expertise as a competitive advantage that strengthens strategic oversight and supports management in delivering sustainable value.

Board of Directors



Chairman
Qualifications: BSc, MBChB, DCom (HC)
Appointed: 10 June 2010
Expertise: Business development, governance, health, investor relations, leadership and strategy



Qualifications: MDP, CPA(SA)
Appointed: 10 June 2010
Expertise: Business development, finance, human resources, investor relations and strategy



Qualifications: MBChB
Appointed: 7 December 2015
Expertise: Business development, health and safety management and human resources

** With effect from 24 February 2026, Dr Nkateko Munisi resigned from his position as a member of the Investment Committee and was appointed as a member of the Remuneration Committee.*



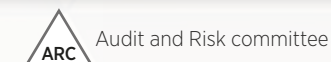
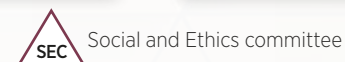
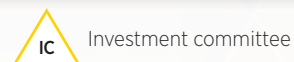
Qualifications: MBA, BBusSci (Hons)
Appointed: 20 June 2022
Expertise: Business leadership and strategy



Qualifications: BBusSci (Hons)
Appointed: 15 June 2023
Expertise: Business leadership and strategy



Qualifications: BCompt (Hons), CA(SA)
Appointed: 15 June 2023
Expertise: Business development, investment, mergers, and acquisitions



Detailed biographies of our Board can be found at www.afrocentric.za.com/about-us/our-leadership/

¹ Resigned as a Non-executive Director and member of the Investment committee with effect from 6 March 2026.

Independent Non-executive Directors



BRUNO FERNANDES (58)

Lead Independent Director
Qualifications: BCom, BAcc, BCom (Hons) (InvM), CA(SA)
Appointed: 23 November 2018
Expertise: Accounting, advisory-related services, corporate governance and oversight, finance, risk management, and investment banking



KARABO MORULE (44)

Qualifications: BBusSci (Actuarial Science, Finance), PDip (Actuarial Science), AMP, Fellow with the IFOA and ASSA
Appointed: 31 December 2024
Expertise: Leadership, strategy, sustainability, corporate governance, compliance, risk management, finance, and innovation



ALICE LE ROUX (52)

Qualifications: BAcc, BCompt (Hons), CA(SA), RA
Appointed: 25 May 2020
Expertise: Accounting, risk management, corporate governance, finance, human resources, and auditing



CHARLOTTE MOKOENA (60)

Qualifications: BA (Hons), BSocSci, MCom in Leadership, PDip in Training and Performance Management
Appointed: 1 October 2025
Expertise: Business leadership, human resources, stakeholder relations and communication, and social impact

Executive Directors



GERALD VAN WYK (42)

Group Chief Executive Officer (CEO)
Qualifications: BCom Risk Management, Senior Management Development Programme, General Management Programme
Appointed: 1 November 2023
Expertise: Business development, business leadership, strategy, and stakeholder relationships
Committees: Invitee to committees



THATO MOLOELE (38)

Group Chief Financial Officer (CFO)
Qualification: BAccSci, HDipAcc, CA(SA), MBA
Appointed: 1 January 2025*
Expertise: Finance and mergers and acquisitions, accounting, risk management, and auditing
Committees: Invitee to committees

* Appointed as an Executive Director with effect from 1 January 2025 and took office as CFO with effect from 1 February 2025.

Executive committee

The Executive committee ensures that the Board's strategic objectives and policies are implemented effectively throughout the Group.

Detailed biographies of our Executive committee can be found at www.afrocentric.za.com/about-us/our-leadership/



GERALD VAN WYK (42)

Group CEO

BCom Risk Management (University of South Africa), Senior Management Development Programme (University of Stellenbosch), General Management Programme (Harvard Business School)



THATO MOLOELE (38)

Group CFO

Bachelor of Accounting Science (University of the Witwatersrand), a Higher Diploma in Accounting Science (University of the Witwatersrand), CA(SA) and an MBA (University of Manchester)



LINDIWE MIYAMBU (52)

Chief People and Marketing Officer

Bachelor of Education (National University of Lesotho), Certificate in Labour Relations (Global Business Solutions), Post Graduate Diploma in Labour Relations (Graduate Institute of Management and Technology)



AKLAAQ MAHMOOD (60)

Group Executive: Diversified Investments and Africa

BAcc (University of South Africa), MBA (University of Stellenbosch)



ANDREW SCHWULST (60)

CEO: Medscheme

BCom Accounting and Finance (University of Cape Town), MBA (Bond University)



MUJEEB BRAY (53)

COO: Medscheme

BCom Business Administration and Management, General (University of Western Cape), Leadership Advance Programme (University of Cape Town), High Performance Peak Programme (London Business School), CEO Programme (Said Business School – University of Oxford)



SATISH ANTONY (42)

Chief Analytics and Strategy Officer

BCom (Honours) Actuarial Science University of Stellenbosch, MBA Executive Management (University of Cape Town), Fellow of the Institute and Faculty of Actuaries (UK)



ASHLEY SINGH (44)

Chief Information Officer (CIO) Health and Corporate

BSc Computer Science (Natal University), MBA (Henley Business School), PGDIP (Honours) Leadership (Stellenbosch Business School) Agile Leadership Harvard Certification



MONWABISI KULA (44)

Chief Risk Officer

MBA (Wits Business School), PGDip Business Administration (Wits Business School), Adv. Dip Acc Sci (University of South Africa), Cert. Financial Services Auditor (Institute of Internal Auditors SA), BCom (Walter Sisulu University)



JEREMY YATT (61)

Chief Growth Officer

BA LLB (University of Witwatersrand) and completed an EDP (Executive Development Programme) Gordon's Institute of Business School. He is also an admitted attorney in the High Court of South Africa.

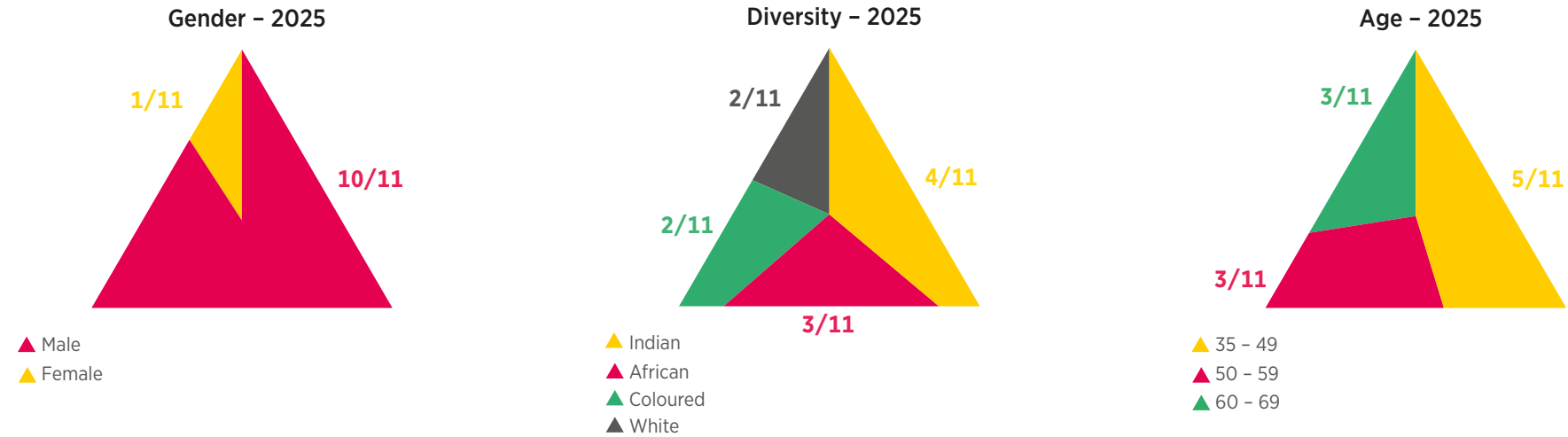


DAVID CAROLUS (54)

Interim Group Executive: Corporate Solutions

BSc. Eng (Mech) (University of Cape Town), Programme in Financial and Investments (FMIPR) (University of South Africa), Hons BB&A (University of Stellenbosch), MBA (University of Stellenbosch), MPhil Futures Studies (University of Stellenbosch)

Executive committee diversity



Board oversight of strategy and value creation

The Board provides ethical and effective leadership by overseeing the Group’s strategy, monitoring performance and ensuring that key decisions support AfroCentric’s ability to create sustainable value over the short, medium and long term while protecting against value erosion. During the year, the Board deliberated on a range of strategic and governance matters aligned with the Group’s refreshed strategy and evolving operating environment.

Through these deliberations, the Board oversaw strategy execution, leadership succession, portfolio optimisation, regulatory developments, ESG integration and financial performance to ensure that the Group remains positioned to deliver on its strategic objectives.

Strategy execution and portfolio optimisation			Leadership succession and Board composition		
Key Board oversight and decisions			Key Board oversight and decisions		
Oversaw execution of the refreshed strategy focused on strengthening the Group’s health offering and driving clinical innovation.			Oversaw leadership succession, including the appointment of Thato Moloele as Group CFO and the appointment of Charlotte Mokoena as an Independent Non-executive Director.		
Approved the disposal of AfroCentric Distribution Services group (ADS group) and Wellworx to Sanlam Life, and the proposed sale of the Activo Group to FHC Proprietary Limited.					
Strategic rationale/value creation impact			Strategic rationale/value creation impact		
Portfolio optimisation and alignment of the Group’s business portfolio with its long-term strategic priorities.			Strengthens Board independence, diversity and skills to support strategy execution and governance oversight.		
Strategic objectives supported	Governance lens	More information	Strategic objectives supported	Governance lens	More information
	Strategy oversight	Refer to page 58		Governance effectiveness	Refer to our separate Governance and Remuneration report.

Risk oversight and strategic review

Key Board oversight and decisions

Reviewed key risks, adjusted risk ratings and tolerance levels where required, and linked principal risks to material matters to ensure alignment between risk oversight and strategic priorities.

Creation of a Chief Risk Officer role and oversaw the appointment of a suitable candidate into the role.

Strategic rationale/value creation impact

Ensures proactive risk management and resilience in a complex healthcare environment.

Strategic objectives supported	Governance lens	More information
   	Risk governance	Refer to page 40

Strategic partnership with Sanlam

Key Board oversight and decisions

Approved the disposal of the ADS group and Wellworx to Sanlam Life as part of strengthening collaboration with our core strategic partner.

Strategic rationale/value creation impact

Supports the development of an integrated healthcare value chain and expanded healthcare offering.

Strategic objectives supported	Governance lens	More information
  	Strategic partnerships	Refer to page 58

Industry and regulatory developments

Key Board oversight and decisions

Oversaw the Group's response to developments relating to the Bonitas administration and managed care RFP process and related legal proceedings.

Strategic rationale/value creation impact

Ensures proactive governance oversight of regulatory and industry developments affecting the Group.

Strategic objectives supported	Governance lens	More information
  	Regulatory oversight	Refer to our Chairman and CEO reviews on page 6 and 54 respectively.

Economic, Social and Governance (ESG) and transformation oversight

Key Board oversight and decisions

Monitored ESG integration and transformation progress, including initiatives to sustain the Level 1 B-BBEE status for AfroCentric Health and AfroCentric Investment Corporation Limited.

Strategic rationale/value creation impact

Strengthens stakeholder trust, sustainability and long-term value creation.

Strategic objectives supported	Governance lens	More information
	ESG governance	Refer to page 76

Financial stewardship and governance frameworks

Key Board oversight and decisions

Approved the Annual Financial Statements and Integrated Report, and an updated Delegation of Authority Framework to strengthen accountability and decision-making.

Selected earnings per share and headline earnings per share as key performance metrics for trading statement purposes.

Strategic rationale/value creation impact

Ensures sound financial stewardship and strong governance frameworks.

Strategic objectives supported	Governance lens	More information
	Financial governance	Refer to our reporting suite.

Socioeconomic impact

Key Board oversight and decisions

Considered enterprise and supplier development and corporate social investment initiatives, including bursary programmes and contributions to health organisations.

Strategic rationale/value creation impact

Supports the Group's role in strengthening healthcare access and social development.

Strategic objectives supported	Governance lens	More information
	Stakeholder value	Refer to page 29

Strategic developments subsequent to the reporting period

Key Board oversight and decisions

Deliberated on the outcome of the Bonitas RFP process within the context of the ongoing legal proceedings. The Board also approved a structured turnaround programme aimed at stabilising the business and repositioning the Group in response to potential changes in scale.

Strategic rationale/value creation impact

Ensures strategic resilience and proactive response to a significant industry development.

Strategic objectives supported	Governance lens	More information
	Strategic oversight	Refer to page 6

Governance of risk, ethics and responsible business practices

The Board is committed to maintaining high ethical standards across the Group. The Social and Ethics committee oversees the Group's Code of Ethics, monitors anti-corruption initiatives, and assesses management's implementation of ESG-related policies and disclosures.

AfroCentric maintains a zero-tolerance approach to unethical conduct. Mechanisms for reporting concerns include a declaration of conflicts of interest process, formal codes of conduct and an independent whistleblowing hotline through which employees and stakeholders can report fraud, corruption or unethical behaviour without fear of reprisal.

The Board also oversees the Group's compliance environment. The legislative compliance universe is reviewed, monitored and reported to ensure alignment with applicable laws and regulations. During the reporting period, there were no significant ESG-related incidents, regulatory breaches or fines.

Board committees supporting oversight

The Board has established several committees to assist in fulfilling its oversight responsibilities. Each committee operates under a Board-approved charter and reports regularly to the Board.

Key committees include:

- **Audit and Risk committee**, which provides independent oversight of financial reporting, internal controls, risk management and combined assurance processes as well as information technology governance via the ICT Steering committee, a sub-committee to the Audit and Risk committee
- **Investment committee**, which evaluates capital allocation decisions and considers investment opportunities, mergers and acquisitions and material transactions
- **Nomination committee**, which oversees Board composition, succession planning and director appointments
- **Remuneration committee**, which ensures that executive remuneration supports the Group's strategic objectives and attracts and retains key talent
- **Social and Ethics committee**, which monitors responsible corporate citizenship, ethical conduct and ESG performance

Through these committees, the Board strengthens its ability to provide independent oversight and ensure effective governance across the organisation.

Remuneration aligned with performance

The Remuneration committee determines the remuneration philosophy of the Group, including the remuneration of Executive Directors in accordance with AfroCentric's remuneration policy. Executive remuneration is based on a pay-for-performance principle, whereby remuneration outcomes are linked to the Group's balanced scorecard and the achievement of strategic objectives.

Non-executive Directors do not hold service contracts, and their remuneration is subject to shareholder approval at the annual general meeting.

As noted in the introduction to this report, certain detailed compliance disclosures have been moved to supplementary publications to maintain a clear and concise focus on value creation within this integrated report. Further detailed governance, remuneration, tax and other shareholder-related disclosures are therefore available in the **Governance and remuneration report**, available online at <https://www.afrocentric.za.com/investor-centre/>.

About us

AfroCentric is a B-BBEE level 1 JSE-listed investment holding company providing services and products to the healthcare sector. The Group was founded in 2008 on the core philosophy of promoting transformation and empowerment.

What we do

We increase access to sustainable, affordable and quality healthcare through our operating subsidiaries by providing health administration, health risk management, and a range of complementary solutions to our medical scheme clients and their members across the healthcare value chain.

A primary lever in our growth strategy is building a winning health offering in partnership with Sanlam.



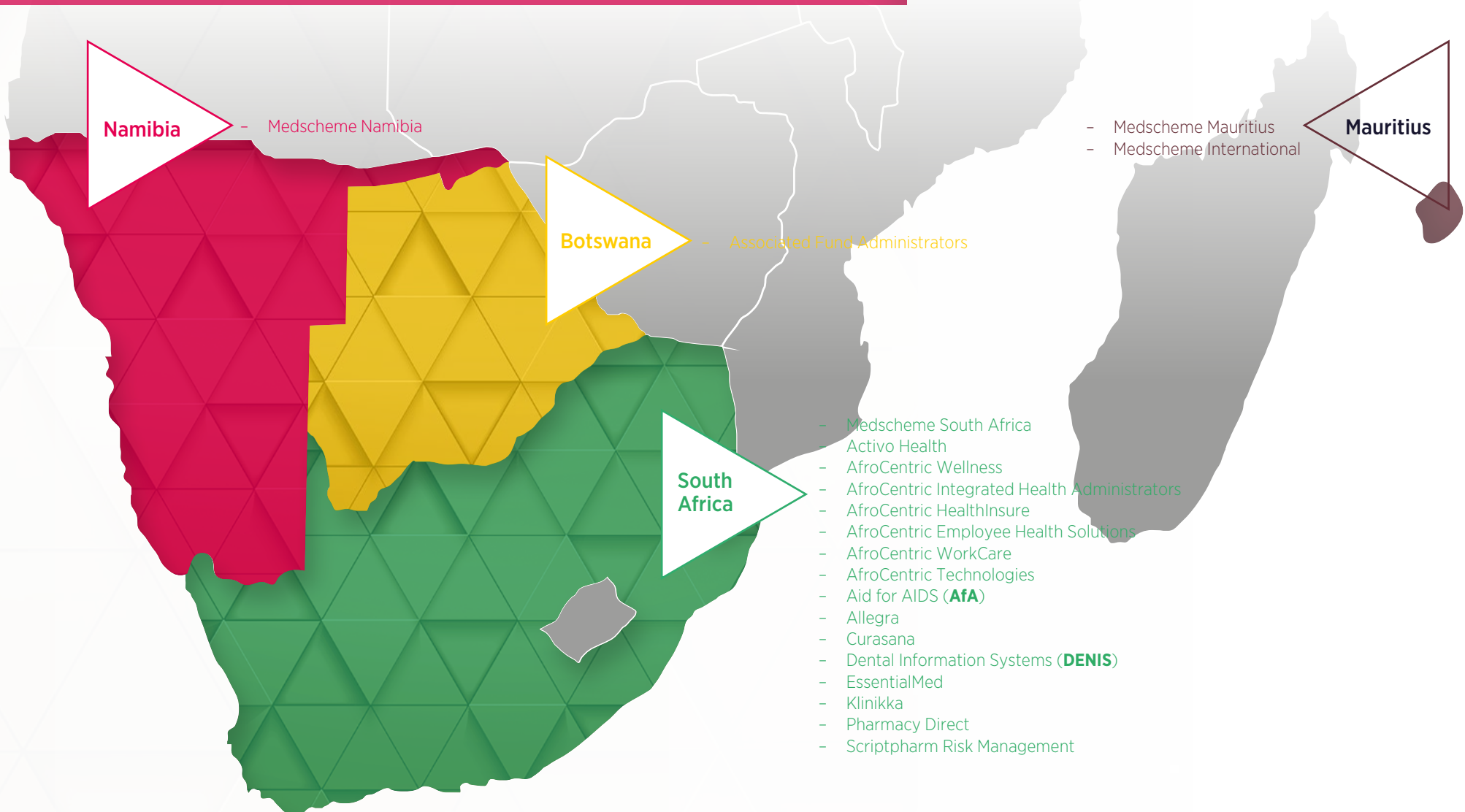
We execute our vision and purpose through our strategy

To expand and diversify AfroCentric's presence in the broader healthcare industry to achieve our ultimate goal of transforming healthcare.

For more information, refer to page 58.

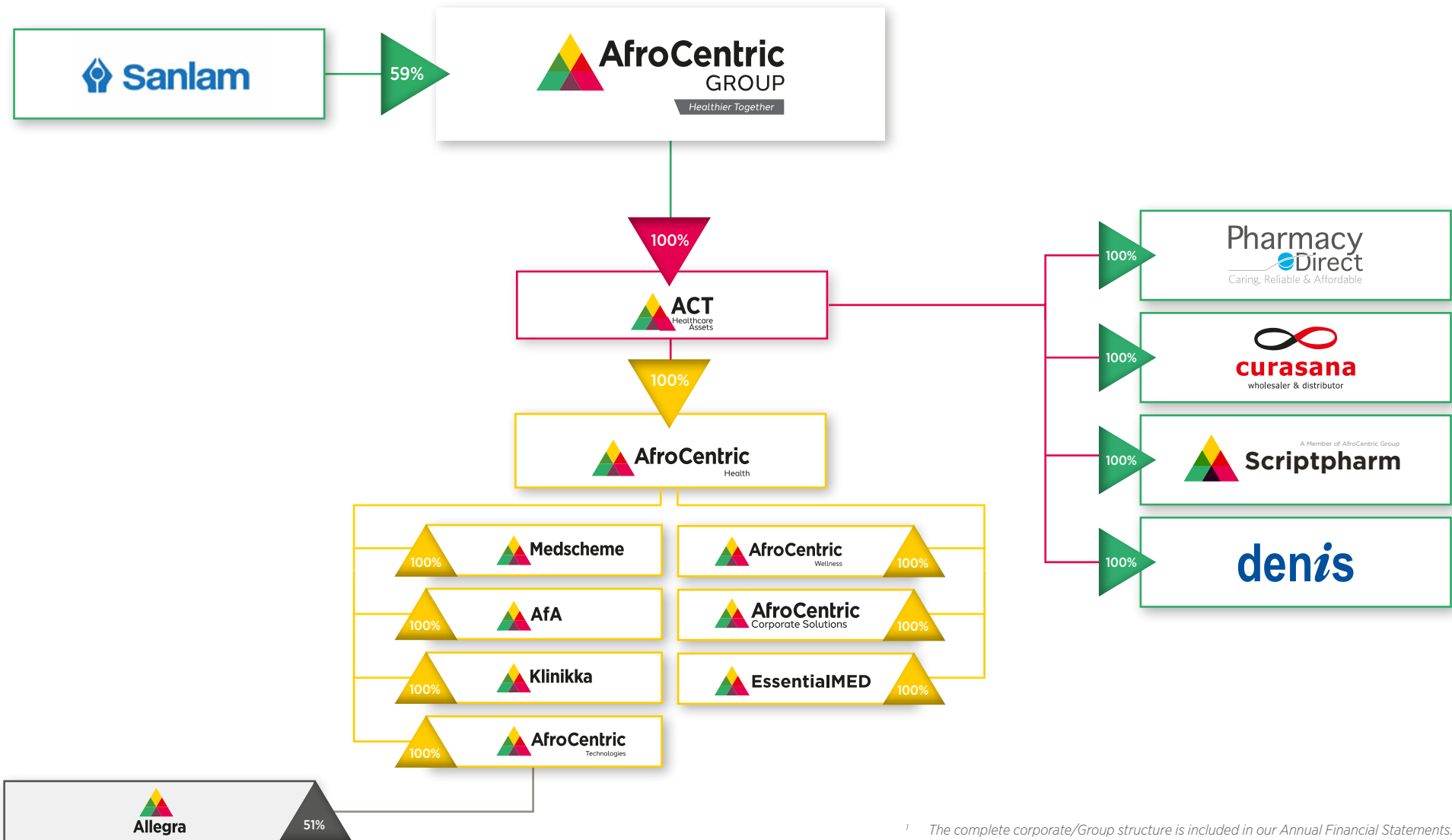
Where we operate

We operate in four Southern African countries.



Organisational overview

Group structure¹



¹ The complete corporate/Group structure is included in our Annual Financial Statements.

What sets us apart

Our diversified business model and growth strategy enable us to achieve sustainable value creation and preservation across the healthcare sector by leveraging our competitive advantages while mitigating risks to prevent value erosion.

Strong market positioning



Market leader in managed care

An integrated healthcare business and market leader in managed care.



Largest distributor of chronic medicine

The largest distributor of chronic medicine to public hospitals, clinics and distribution points.



Proven leadership in medical scheme administration

One of the largest health administration businesses in the South African healthcare sector.

An established record of accomplishment as a medical scheme administrator –

4.09 million

lives under management (14.5% market share).

Leading provider in prescription dispensing



18.3 million

scripts dispensed during the year – public sector makes up 93% of the total prescriptions dispensed.

Leveraging partnerships

Our integration into **Sanlam's comprehensive ecosystem** represents a competitive edge, reaffirming our commitment to providing affordable medical aid and health insurance solutions.



Integration into Sanlam's ecosystem provides a **unique competitive edge**.



Access to Sanlam's **expansive distribution network** and diverse financial products.



Enhanced ability to serve customers with **improved offerings and service quality**.

Diversified across complementary healthcare services

Pharmaceutical business contribution

Pharmaceutical companies contributed **46.3%** in revenue, as well as **15%** contribution to operating profit.



Strategic diversification

Diversification makes us more sustainable in a changing healthcare environment.



Optimising healthcare costs

Our business model enables the Group to optimise healthcare costs to deliver on our mission.

What sets us apart *continued*

Multiple growth drivers



Partnership expansion: Continued focus on key partnerships with schemes, intermediaries, and service providers to grow our networks and deliver a compelling data and digitally enabled healthcare proposition.



Digital innovation: Advancing digital solutions to support disease management, enhance patient outcomes, and improve operational efficiency.



People and culture development: Implementing targeted initiatives to enhance employee value, foster cultural alignment, and support a high-performance environment.



Enhancing Group governance: Aligning and strengthening all subsidiaries with the Group's risk, compliance, and governance frameworks to ensure a consistent and effective approach.



Operating model evolution: Refining our business focus by re-evaluating existing models, enhancing core operations, and deepening integration around Medscheme to optimise efficiencies and service delivery.

Competent and experienced management



Leadership expertise

Experienced and empowered leadership team with healthcare expertise.



Intellectual capital

Market-leading intellectual capital with clinical, actuarial, data analytics and health intelligence competence and capabilities.



Data-driven strategies

Data capabilities support our strategy to eliminate inefficiencies within the healthcare industry and optimise the healthcare value chain.

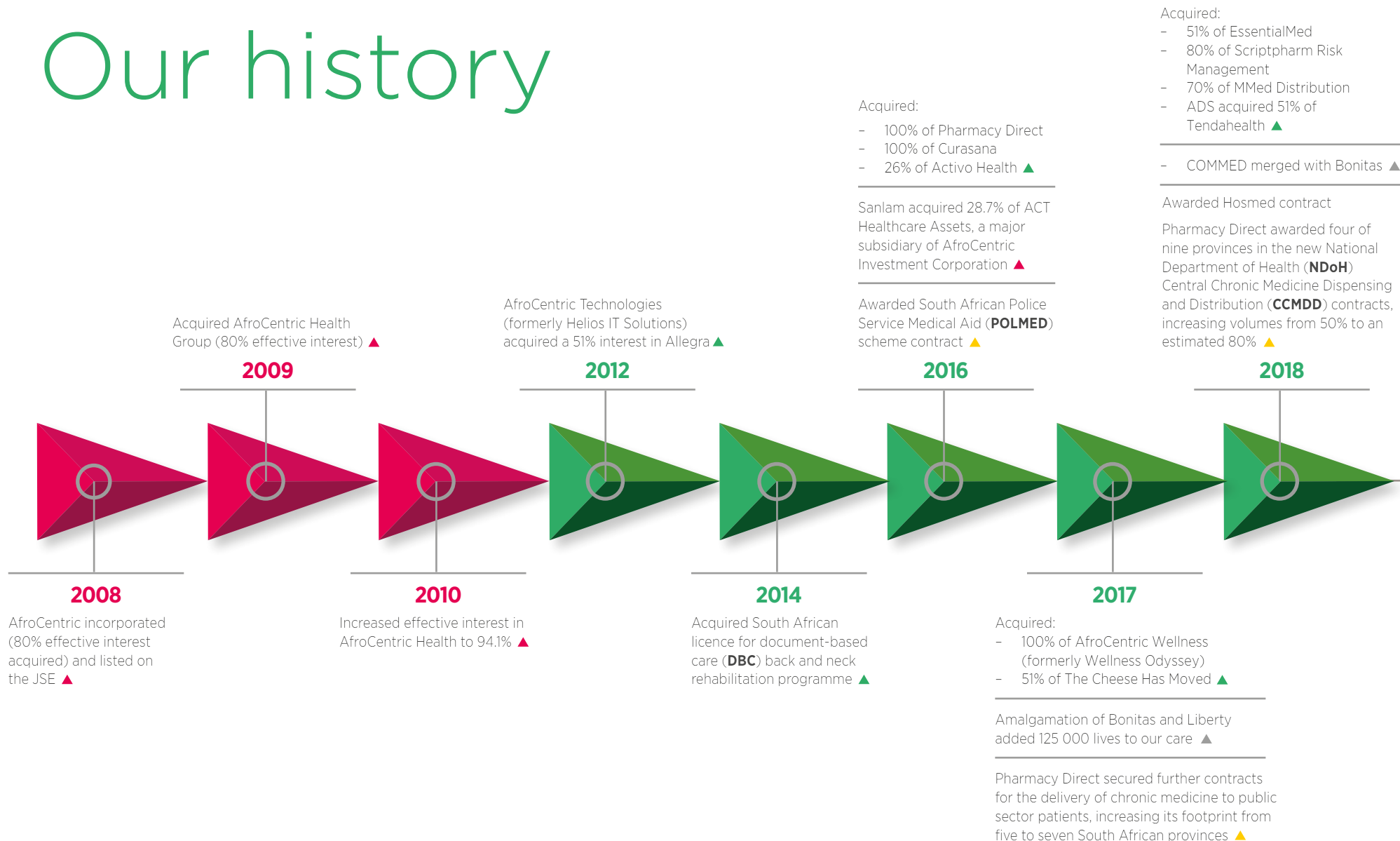


Incentivising long-term value

Long-term incentives are in place for strategic employees.



Our history



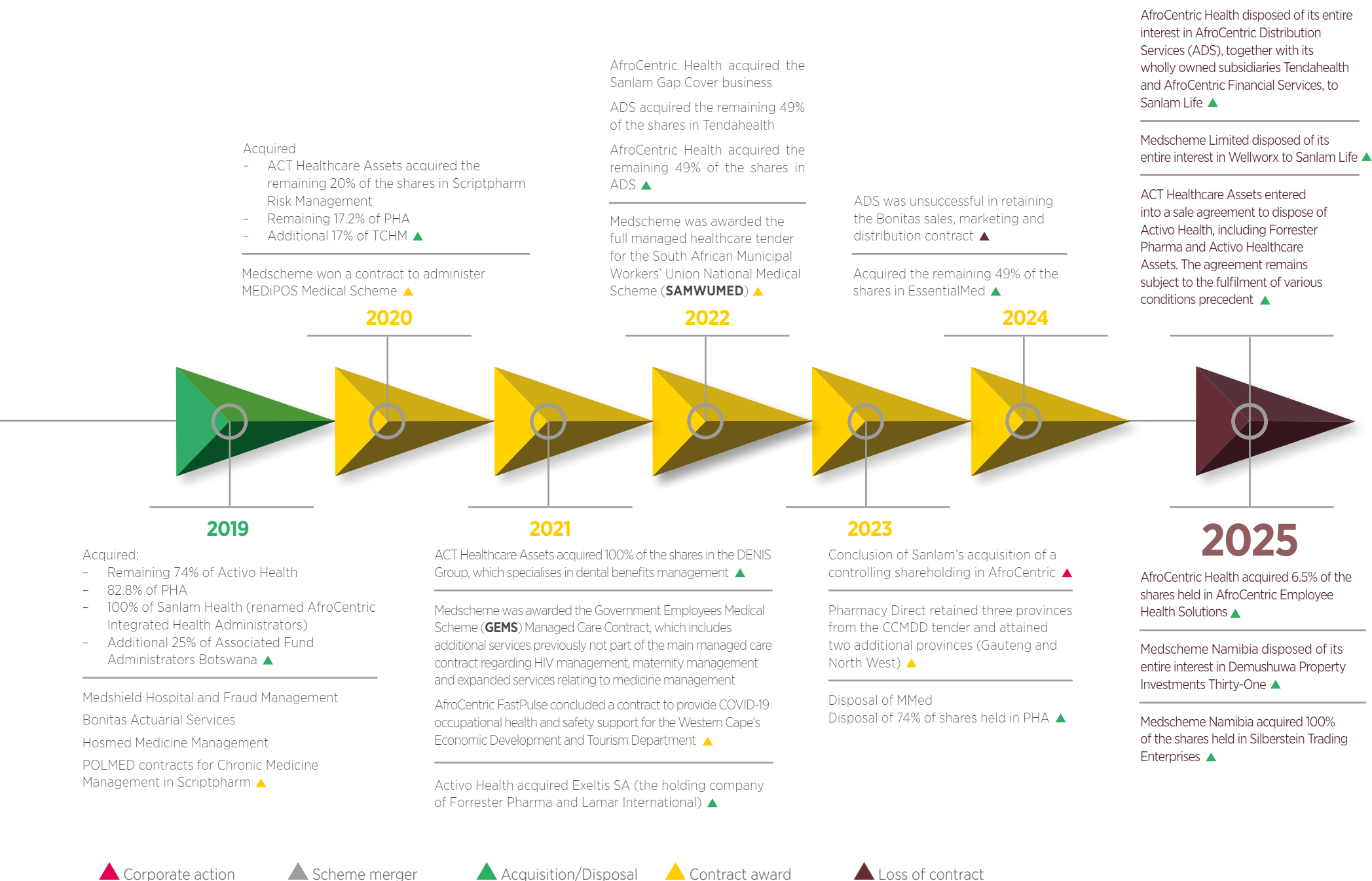
▲ Corporate action

▲ Scheme merger

▲ Acquisition/Disposal

▲ Contract award

▲ Loss of contract



Our operations

AfroCentric is the most diversified healthcare company in Southern Africa. To leverage our integrated model, our business is organised and managed into three primary clusters: 1) medical scheme administration, risk management and technology, 2) pharmaceutical, and 3) corporate solutions.

Medical scheme administration, risk management and technology

Transforming healthcare, together

We partner with medical schemes to manage healthcare benefits, pay claims and use advanced technology and risk management to improve access to affordable quality care

Areas of strength and differentiation

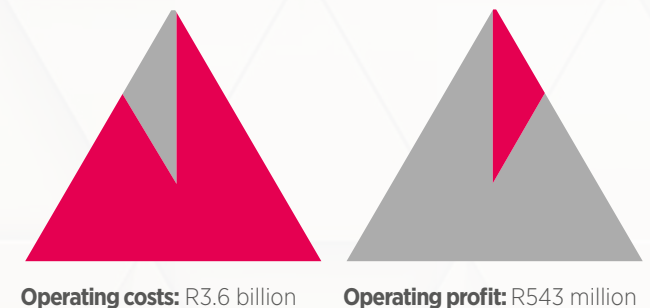
- Largely constituting Medscheme, the largest managed care organisation in South Africa
- Manages over four million lives
- Utilises hyper-automation, robotic process automation, artificial intelligence (AI), and machine learning

Outputs (products and services)

- **Claims processing:** Automated claims adjudication and high straight-through processing rates
- **Customer service:** Enhanced digital platforms for member interaction, including online and mobile technologies
- **Health risk management:** Programmes that identify and manage clinical and financial risks to lower healthcare costs
- **VirtualCare:** Provides safe and affordable virtual consultations with registered medical professionals
- **Hospital benefit management:** Digital engagement system for hospital authorisations and claims
- **Healthcare Professional Portal (HCP):** Our HCP portal is a web-based digital tool designed to help healthcare practitioners and their staff simplify and speed up many administrative tasks

Contribution to Group

Revenue: **R3.8 billion** (51.4%)



Pharmaceutical

Transforming healthcare, together

We manage the cost, supply and delivery of medicines across the healthcare system, improving affordability and access while ensuring patients receive the treatment they need

Areas of strength and differentiation

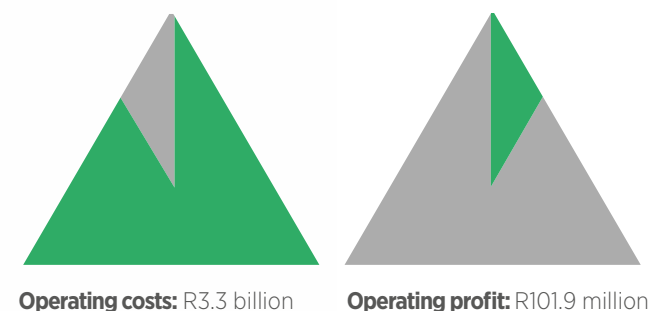
- Management of full medicine spend for our client schemes
- Comprehensive presence in the pharmaceutical value chain, including manufacturing, wholesale, importing, marketing, distribution and courier services

Outputs (products and services)

- **Wholesale and distribution:** Delivering medicines to urban and rural areas through Pharmacy Direct
- **Medicine cost management:** Scriptpharm manages medication and provides drug utilisation reviews

Contribution to Group

Revenue: **R3.4 billion** (46.3%)



Corporate solutions

Transforming healthcare, together

We support employers with workplace healthcare, wellness, and insurance solutions that promote healthier employees, reduce healthcare costs and improve access to care

Areas of strength and differentiation

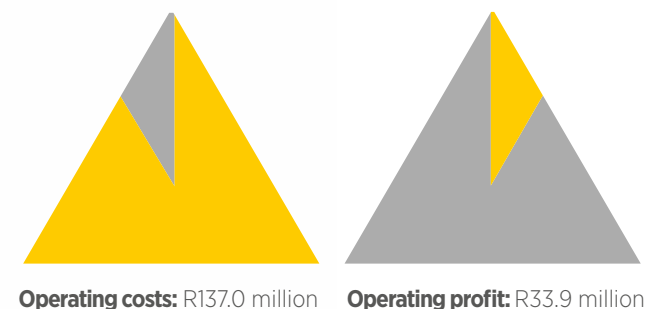
- Integrated employee wellness and occupational health services
- Primary care and gap cover solutions
- Focused on reducing employer healthcare costs

Outputs (products and services)

- **Health and wellness programmes:** Integrated solutions for employee health and wellness
- **On-site clinics:** Nurse-led primary healthcare clinics
- **Disease management:** Programmes for managing non-communicable diseases
- **Wellness initiatives:** Programmes to enhance employee wellbeing and productivity
- **Sanlam Gap Cover and Primary Healthcare Insurance**

Contribution to Group

Revenue: **R170.9 million** (2.3%)



Our business model

The resources we rely on

Capitals

Availability, quality, and affordability

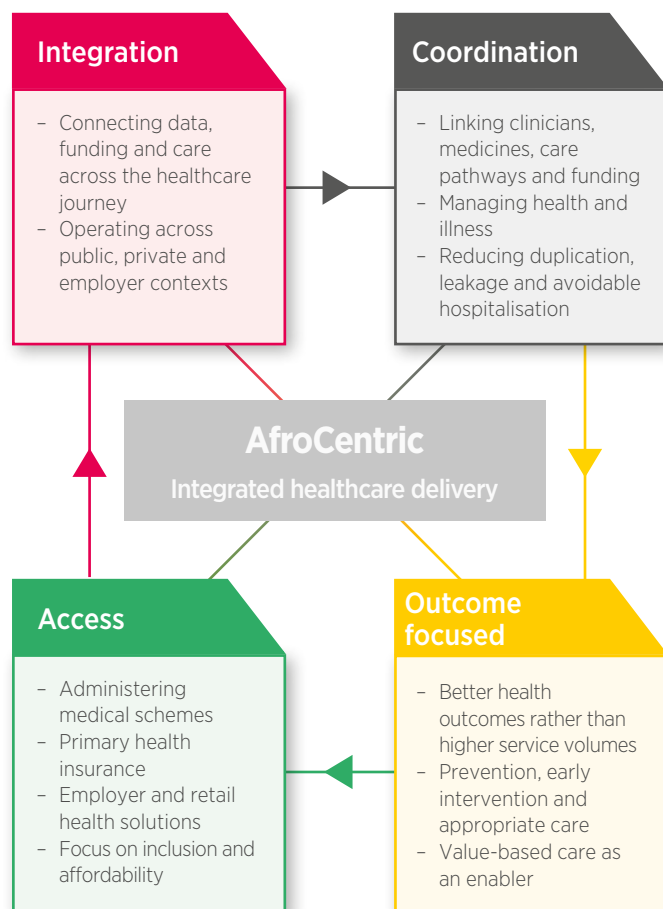
Inputs

 Financial	Disciplined financial and liquidity management remains essential in a constrained economic environment characterised by affordability pressure and heightened cost sensitivity across the healthcare system. The Group continues to focus on prudent capital allocation, cost management and balance sheet resilience to support sustainable operations and investment in strategic priorities. Ongoing collaboration with Sanlam, including on technology enablement, is expected to support systems stability, operational efficiencies and longer-term cost optimisation, strengthening the Group's financial resilience.	The pool of funds (equity) the Group relies on, including funds reinvested in the Group, return on investments and revenue generated from services – R1.9 billion net asset value (2024: R3.3 billion) – (R448) million* reduction in retained earnings * Value includes the impact of the total impairment raised. Please see <i>Our economic performance</i> on page 64 for more information.
 Intellectual	The pace of innovation across the healthcare sector continues to accelerate, increasing the importance of adaptable systems, data capability and continuous learning. The Group continues to invest in strengthening its digital platforms, software environments and process maturity, supported by collaboration and knowledge-sharing across the broader Sanlam ecosystem. These efforts enhance decision-making, service quality and innovation capability, supporting relevance and sustainability in an increasingly data-driven healthcare environment.	The knowledge-based intangible assets such as systems, processes, customised IT, licences, business and industry knowledge, fraud management software, and medical administration knowledge – R117.1 million spent on software and IT system development (2024: R53.7 million) Please see <i>Our economic performance</i> on page 64 for more information.
 Manufactured	Ongoing investment in physical infrastructure, facilities and core technology platforms remains critical to operational efficiency, service reliability and client confidence. While such investments may place short-term pressure on costs, they are expected to enhance service quality, scalability and affordability over time through improved efficiency and reduced operational risk. Modern, resilient infrastructure underpins the Group's ability to deliver consistent healthcare services in a competitive and evolving operating environment.	The physical assets that are available for use within the Group's operations, including buildings, facilities, equipment and IT infrastructure (hardware and physical components) – R64.9 million spent on physical infrastructure modernisation (2024: R64.5 million) Please see <i>Our economic performance</i> on page 64 for more information.
 Human	The healthcare system continues to face significant workforce constraints, with shortages, ageing profiles and skills pressures affecting service capacity and the delivery of quality care across both public and private sectors. These challenges are amplified by rising healthcare demand, increased care intensity and growing expectations of clinical quality and compliance. For AfroCentric, this context underscores the importance of supporting a capable and resilient healthcare workforce, both within the Group and across the broader healthcare ecosystem.	Our employees' and contractors' skills, wellness and motivation: – 5 707 employees (2024: 5 899), with R6.8 million invested in training and R2.9 million in bursaries (2024: R1.6 million and 0.3 million, respectively) Please see <i>Our social performance</i> on page 76 for more information.
 Social and relationship	Strong, trusted relationships with clients, regulators, partners, service providers and the communities in which the Group operates remain fundamental to value creation. Effective stakeholder engagement supports service delivery, governance outcomes and collaboration across the healthcare ecosystem, particularly during periods of heightened industry scrutiny, regulatory uncertainty and system-level reform.	AfroCentric fosters strong relationships with its material stakeholders by actively managing stakeholder engagement and maintaining its social licence to operate. Please see <i>Our stakeholders</i> on page 33 and <i>Our social performance</i> on page 76 for more information.
 Natural	Environmental health is increasingly recognised as a foundational determinant of human health, influencing disease burden, healthcare demand and long-term system sustainability. In South Africa, a coal-intensive energy mix and growing resource constraints continue to place pressure on environmental and public health outcomes. Within this context, the Group seeks to reduce its environmental footprint and strengthen operational resilience through targeted initiatives, including the transition to solar energy solutions where feasible and the implementation of borehole infrastructure to support responsible water management.	– Water consumption: 72 251 kl (2024: 24 880 kl) – Electricity consumption: 8 096 MWh (2024: 4 706 MWh) – Diesel consumption (used in generators): 12 671 litres (2024: 8 512 litres) – Petrol (used in Company-owned vehicles): 63 548 litres (2024: 86 802 litres) Please see <i>Our environmental performance</i> on page 86 for more information.

What we do and produce

Activities

AfroCentric's activities focus on broadening healthcare access by expanding our presence, integrating our operations and optimising costs across the value chain to measurably improve access to quality care.



Outputs

Our products and services

Enhanced access to healthcare through health administration and risk management services

- We provide efficient, client-centric **administration services** to leading medical schemes in Southern Africa
- We provide **health risk management** by identifying and managing clinical and financial risks for our client schemes in a coordinated manner to improve the members' clinical outcomes

Complementary services

- Coordinated day-to-day healthcare provision
- Medicine delivery

Products

- Medicine and vitamins

Waste products

- Direct emissions of **188 tCO₂e** (2024: 261 tCO₂e)
- Indirect emissions of **93 835 tCO₂e** (2024: 3 975 tCO₂e)

Outcomes for overarching goals



Enhanced quality of life and stakeholder value added

Stable and reliable healthcare provision supports continuity of care and financial protection for members, contributing to improved health security and quality of life. While the Group experienced financial pressure during the year, disciplined liquidity management and balance sheet resilience helped sustain healthcare delivery and member support.

- Headline earnings per share of **13.92 cents** (2024: 3.80 cents)
- R607 million** in cash reserves (2024: R348 million)
- 33.2%** decrease in share price (2024: 30.9% decrease)
- Operating profit increased by **82.3%** (2024: 63.8% decrease)
- Total revenue increased by **94%** (2024: 51.8% decrease)

Affordable healthcare

Improved system efficiency and decision-support reduce administrative friction and unnecessary cost escalation, supporting more affordable access to healthcare

Widely accessible healthcare

Scale, infrastructure and integrated platforms enable sustained access to healthcare services across diverse population groups

- 4.09 million** lives under administration (2024: 4.08 million)
- 15 client medical schemes** administered (2024: 15)

Stakeholder quality of life

Engagement and development across the healthcare value chain support inclusive participation, economic resilience and long-term system sustainability

- R16.5 million** invested in enterprise and supplier development (2024: R7.7 million)
- R5.8 million** invested in socioeconomic development (2024: R2.2 million)

Sustainable healthcare

Responsible resource use supports resilient healthcare operations while reducing environmental impact over the long term

- Environmental impact monitoring and management
- Optimised energy, water and paper use
- 708 MWh** of self-generated solar electricity (2024: 247 MWh)

For detailed economic, social, and environmental performance, see *Our performance* chapter starting on page 63.

Our business in context



Operating context

AfroCentric operates within interconnected economic, social and institutional systems that shape how healthcare is funded, accessed and delivered in South Africa. Conditions across the economy, healthcare funding landscape, population health profile, technology environment and workforce availability influence the availability, affordability and quality of the capitals we rely on (see Our business model on page 28).

These system dynamics affect stakeholders directly through access, cost and continuity of care, and indirectly through the resilience and sustainability of the healthcare ecosystem within the wider socioeconomic context.

Economic conditions and affordability pressures

South Africa's macroeconomic environment continues to be characterised by constrained growth and persistent household affordability pressure. While headline inflation moderated during 2024 and remained near the lower end of the target band through much of 2025, healthcare affordability remains under pressure as households and employers balance medical scheme contributions against broader cost-of-living demands.

This environment increases price sensitivity among members and employers, influencing benefit selection, retention and demand for lower-cost healthcare solutions. For healthcare funders and administrators, these pressures reinforce the importance of affordability-focused design, utilisation management and cost discipline.

Material matters

- Access to healthcare and medicine
- Sustainable financial performance
- Member satisfaction
- Economic, transformation, political, geopolitical and societal risk

Strategic levers



Healthcare access, utilisation and market dynamics

South Africa's healthcare system continues to operate within a structural imbalance between healthcare need and access. Demand for care is rising due to demographic and epidemiological trends, while access to private healthcare remains limited to a relatively small portion of the population.

Within the private healthcare environment, modest membership growth and an ageing beneficiary profile are translating into higher utilisation and escalating costs. At the same time, industry solvency remains above regulatory minimums, reflecting underlying financial resilience despite sustained cost pressures.

Policy developments aimed at expanding access form an important part of this context. While NHI is intended to broaden access over time, ongoing uncertainty regarding its design, sequencing and implementation continues to affect the medical schemes industry. In parallel, recent Council for Medical Schemes (CMS) recommendations regarding Low-Cost Benefit Options (LCBOs) and exempt Primary Health Insurance (PHI) products have introduced further uncertainty for affordability-focused funding models.

For AfroCentric, these dynamics reinforce the need for a diversified and adaptable healthcare services model, supported by strong governance and operational resilience, to support access, affordability and continuity of care across evolving funding arrangements.

Material matters

- Access to healthcare and medicine
- Business continuity and adaptation
- Sustainable financial performance
- Member satisfaction
- Ethical business conduct and good governance
- Economic, transformation, political, geopolitical and societal risk

Strategic levers



Technology and data

Technology is increasingly central to how healthcare is administered and how value is delivered through insight, monitoring and more integrated care pathways. At the same time, system resilience, cyber risk and data governance have become non-negotiable operational foundations.

The disruption to the National Health Laboratory Service operations following a cyber-attack in June 2024 highlighted the critical dependency of healthcare delivery on secure, resilient information systems. In parallel, national policy direction continues to emphasise interoperability and safe health information sharing, as reflected in South Africa's National Digital Health Strategy.

Material matters

- Access to healthcare and medicine
- Business continuity and adaptation
- Digital transformation and resilience
- Cybersecurity and information security

Strategic levers



Workforce

Workforce constraints remain a material challenge for the healthcare system, influencing service capacity, quality of care and the pace of reform implementation. Nursing shortages for example, present a risk to system resilience and the delivery of universal health coverage objectives.

For the Group, this context reinforces the importance of attracting, developing and retaining critical skills across clinical, digital and operational domains to support sustainable service delivery and system resilience.

Material matters

- Access to healthcare and medicine
- Employee wellbeing and retention
- Ethical business conduct and good governance

Strategic levers



AfroCentric is positioned to respond to these system dynamics through a diversified and adaptable healthcare services model, supported by strong governance, integrated capabilities and a focus on operational resilience, as the Group continues to support access, affordability and sustainable healthcare delivery.

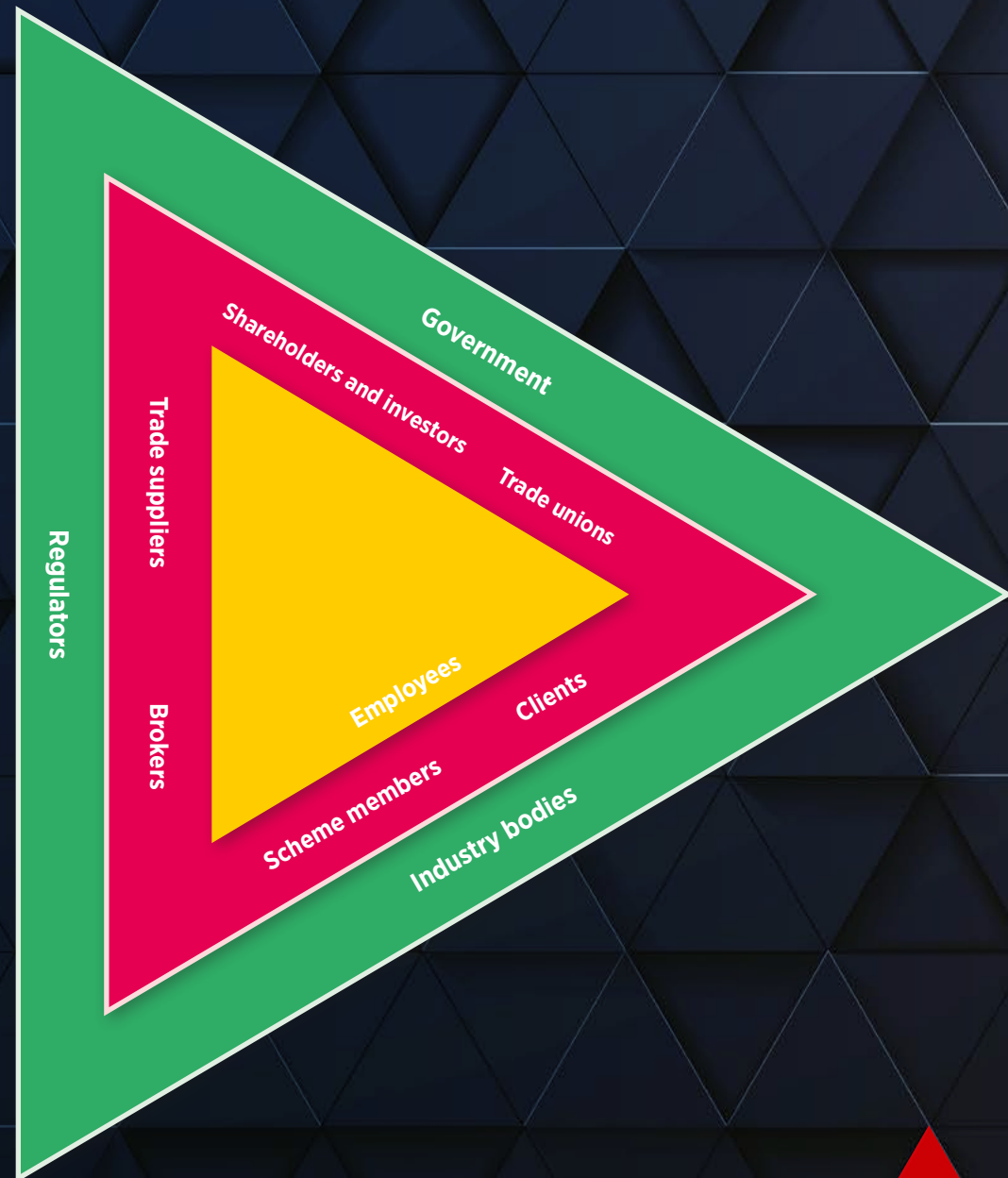
Our stakeholders

We exist to enhance the quality of our stakeholders' lives. This purpose guides how we engage, requiring us to understand, listen to and respond meaningfully to those who influence or are impacted by our activities.

AfroCentric recognises its stakeholders as individuals and groups with a legitimate interest in, or who are affected by, the Group's operations. These include clients, employees, members, partners, regulators, service providers, shareholders and broader societal stakeholders. We acknowledge that the nature and intensity of engagement differ across stakeholder groups, informed by their relative influence and interest.

To deliver on our purpose, we identify, analyse and categorise our stakeholders, enabling tailored engagement approaches that respond to their distinct needs, expectations and concerns. The Board and executive management adopt an inclusive and balanced approach to stakeholder management, engaging in a responsive, constructive, collaborative and transparent manner to address material matters and support sustainable value creation.

Governance oversight of stakeholder engagement is provided by the Social and Ethics committee.



Quality of relationship

Robust relationship characterised by transparency, trust, and mutual understanding



Good quality, value-enhancing relationship



Solid relationship but requires some enhancement to add value



Relationship established, but significant effort needed to enhance its quality



No existing relationship



Industry bodies



Bodies that oversee healthcare-related matters:

- Board of Healthcare Funders of Southern Africa (**BHF**)
- Hospital Association of South Africa
- Doctors' Health Council
- Public Health Enhancement Forum
- Business Unity South Africa (**BUSA**)
- Business Leadership South Africa (**BLSA**)
- Health Ombudsman
- South African Nursing Council

How we engage

- Participation in public and industry forums
- Integrated report and AFS
- Interactive Group website
- Publications

Key concerns raised

- Implementation of the NHI Act and the role of medical schemes
- The NHI Act does not align with the principles enshrined in the country's Constitution and does not reflect the healthcare industry's comments and concerns

Responding to our stakeholders' needs

Ongoing engagements with government and regulators regarding the NHI Act. AfroCentric continues to play a key role in driving and providing access to healthcare. Through our membership with the BHF and BUSA, we have been involved in the NHI Act discussions intended to navigate a way for the private sector to meet and partner with the public sector to address as a collective the healthcare challenges South Africa is facing

Value created for industry bodies

Contributing our technical expertise and in-depth knowledge to pertinent industry topics.

What it measures

AfroCentric participates in various engagements with industry bodies

This qualitative indicator reflects the Group's active participation in industry associations, forums and working groups. It demonstrates AfroCentric's contribution of technical expertise, thought leadership and practical insight to industry discussions, standard-setting and policy development across the healthcare sector.

Value created for us

Industry bodies provide vital opportunities to interact with broader industry players concerning critical interests in the healthcare sector. In some instances, they open communication channels to government and regulators, particularly when policymakers and regulators seek industry-wide positions.



Employees and trade unions



- National Education, Health and Allied Workers' Union (**NEHAWU**)

How we engage

- Ongoing dialogue through established channels (Employee listening strategy)
- Forums
- Company intranet and newsletters
- Leadership conference
- Surveys and employee presentations
- Onboarding throughout the employee life cycle
- Integrated report and AFS

Key concerns raised

- Access to affordable medical care
- Strengthening management and labour working relations
- Reward and recognition
- Wellbeing
- Wage increase

Responding to our stakeholders' needs

- Renewed collective agreement and related forums
- Intentional relationship building – interventions to strengthen engagement between management and labour to achieve common goals
- Introduction of Primary Health Insurance as an affordable medical benefit option
- Ignite recognition programme
- Employee experience management has given prominence to the wellbeing programmes – ensuring a focus on mental health to boost engagement and productivity

Value created for our employees

A purpose-driven work culture that fosters inclusivity and high performance.

Passionate individuals who pursue innovation, excellence and career advancement through continued professional development and focused executive programmes.

What it measures



17.2%

Employee turnover
(2024: 15.3%)

Employee turnover measures the proportion of employees who leave the Group during the reporting period. It provides insight into workforce stability, employee engagement and the Group's ability to retain critical skills in a competitive healthcare environment.



86.8%

of the workforce are black
(2024: 86.0%)

This metric reflects the racial composition of the Group's workforce and indicates progress in advancing employment equity, transformation and inclusivity in line with national priorities and stakeholder expectations.



69.5%

of the workforce are women
(2024: 69.4%)

This metric measures gender representation across the workforce. The Group is committed to gender diversity, inclusive employment practices and equal participation in the workplace.



58.7%

of the workforce are black women
(2024: 62.2%)

This metric highlights the representation of black women within the workforce, providing insight into the intersection of race and gender transformation and the effectiveness of inclusive talent practices.



61.1%

black representation at senior management level
(2024: 59.3%)

This metric measures the proportion of black employees in senior management roles and reflects progress in building a diverse leadership pipeline, succession planning and transformation at decision-making levels.



962

Employees undertook 3 663 online courses
(2024: 440)

This metric reflects participation in online learning and development initiatives and indicates the Group's investment in continuous learning, skills development and capability building across the organisation.



R6.8 million

was spent on developing our employees
(2024: R1.4 million)

This metric reflects expenditure on employee development, including training, professional development and leadership programmes. It demonstrates the Group's commitment to building capability, enhancing performance and supporting long-term organisational sustainability.

Value created for us

Our people are a key differentiator and enable our continued success. AfroCentric competes for skilled and experienced global and local healthcare employees. Retention of critical skills, particularly actuarial, information technology (**IT**) and medical specialists, is imperative to our continued success.



Our clients – medical schemes



Our clients are our direct link to the end-users of our services, which are the medical scheme members.

Our schemes include:

- Open schemes: Bonitas, Fedhealth, Medshield
- Closed schemes: AECL, Barloworld, GEMS, Horizon, MBMed, ParMed, POLMED, SABC, SAMWUMED and Sisonke Health Medical Scheme
- Outside SA schemes: NAMMED, Namibia Health Plan

How we engage

- Presentations
- Meetings
- Email communication
- Symposia
- Voice of the customer survey
- Integrated report and AFS

Key concerns raised

- Scheme growth – attracting young and healthy member profile
- Member retention in the current economical context
- Addressing out of pocket expenses to address member concerns
- Fraud, waste and abuse
- Understanding our unique value proposition
- Sound claims management
- Quality service engagements with emphasis on knowledge management
- Digital transformation to reduce cost and enhance service
- The private sector sustainability brought about by the NHI Act impact

Responding to our stakeholders' needs

- Partnering with BHF to fast-track the LCBO framework approval
- ARVs drug prices decrease successfully negotiated with relevant pharmaceuticals companies
- Appointed a Chief Health Commercial Officer who will be responsible for driving and optimising the integration strategy to generate improved client value and returns for the Group
- Effective hospital collective negotiation model, provider network management and our disease management programmes continue to demonstrate Value-Based-Care (VBC)
- Supplemented administration and managed care services with additional contracts (SAMWUMED Integrated Claims Management)
- Medscheme hybrid EXPO connecting our people to our transformation journey experiences and client feedback through connect groups averaging over 2 000 employees in attendance weekly
- Our mobile app has evolved to be an intelligent extension of our medical aid administration platform at a member's fingertips
- NHI Act was signed into law in May 2024. Medscheme supports the principle that underpins movement towards universal health coverage and is continually engaging industry stakeholders on key considerations and implementation scenarios to business model sustainability

Value created for our clients

AfroCentric is committed to exceptional client service and continuous improvement in our service offerings.

We optimise the healthcare value chain by leveraging our market presence and size to reduce costs for our clients and their scheme members.



Customer effort score

91%

(2024: 83%)

Customer effort score reflects how easy it is for clients (typically medical schemes) to interact with AfroCentric, resolve queries, process claims or access services. A higher score indicates lower effort and a better client experience.



Average scheme contribution increase of

8.6%

(2024: 11.27%)

The average annual increase in medical scheme contributions for members across schemes administered by AfroCentric. Contribution increases directly affect affordability for members. Lower-than-market increases indicate effective cost containment and value delivery to clients and their members.



R293.6 million fraud, waste and abuse (FWA)

(2024: R278.4 million)

The total rand value of potentially fraudulent, wasteful or abusive healthcare claims identified through AfroCentric's detection processes.



Weighted scheme solvency of

52.1%

(2024: 54.8%)

The aggregate solvency position of administered schemes, weighted by size, reflecting their ability to meet current and future member claims. Solvency is a critical indicator of scheme sustainability, regulatory compliance and member protection.



R160.4 million FWA cases recovered

(2024: R155.5 million)

The rand value of funds successfully recovered following confirmed FWA investigations. Recovery demonstrates tangible financial benefit from governance and oversight processes and reinforces accountability in the healthcare system.

What it measures

What it measures

Value created for us

AfroCentric is committed to exceptional client service and continuous improvement in our service offerings.

We optimise the healthcare value chain by leveraging our market presence and size to reduce costs for our clients and their scheme members.



Our shareholders and investors



- Institutional and individual investors

How we engage

- Integrated report and AFS
- Corporate website
- Stock Exchange News Service (SENS) announcements
- Investor days and roadshows
- One-on-one meetings with executive management
- Interim and annual results presentations

Key concerns raised

- Sustainable value creation and preservation
- Consistency in the delivery of strategy
- Consistent financial performance
- Sound investment returns
- Working capital and capital allocation
- Protection of minority shareholders' rights in relation to Sanlam's majority shareholding
- ESG matters

Responding to our stakeholders' needs

- We communicate our value-creation story and strategy through our integrated report
- Enhanced our ESG reporting with additional elements being included during the year
- Enhanced the conciseness of our Integrated Annual Report by producing a separate Governance and Remuneration Report

Value created for our shareholders and investors

Sustainable returns over time derived from a purpose-driven business model.

Diversifying into other revenue streams through our various subsidiaries supports business sustainability.

Value created for us

Our providers of capital enable us to diversify our revenue sources and grow our business.



What it measures



Dividend yield of
0% (2024: 3.0%)

Dividend yield reflects the cash return generated for shareholders relative to the share price. It indicates the Group's ability to return value to shareholders while balancing reinvestment for long-term growth.



Headline earnings per share (HEPS) of
13.92 cents (2024: 3.80 cents)

HEPS measures the Group's underlying profitability attributable to ordinary shareholders, excluding non-recurring items, and is a key indicator of sustainable earnings performance.



Increase in revenue
94.0% (2024: 51.8% decrease)

Revenue growth reflects the Group's ability to grow its income base through organic growth, diversification and market demand for its services, supporting long-term financial sustainability.



Return on invested capital (ROIC)
(18.4%) (2024: 1.0% decrease)

ROIC measures how efficiently the Group generates returns from the capital invested in the business. It is a key indicator of value creation relative to capital employed.



Operating profit increased by
82.3% (2024: 63.8% decrease)

Operating profit reflects the profitability of the Group's core operations before financing and tax. It demonstrates operational efficiency, cost management and the effectiveness of the business model.



Net working capital
(52.9%) decrease year-on-year
(2024: 2.5% increase year-on-year)

Net working capital indicates the Group's short-term liquidity and efficiency in managing current assets and liabilities. Improvements reflect stronger cash flow management and operational discipline.



Return on equity (ROE) of
(64.8%) (2024: 3.6% decrease)

ROE reflects the return generated on shareholders' funds and indicates how effectively management is deploying equity capital to create value.

Pharmaceutical companies



Revenue contribution
46.3% (2024: 45.4%)

Revenue contribution from pharmaceutical subsidiaries reflects the scale of this segment within the Group's diversified operating model.

Contribution to operating profit
15% (2024: 40%)

Contribution to operating profit indicates the profitability of the pharmaceutical segment and its role in strengthening overall Group earnings and margins.

Stakeholder matters in focus

We seek to maintain high levels of corporate transparency. This builds and maintains trust with our stakeholders, growing social and relationship capital that enables value creation and preservation while protecting against value erosion. Below is a summary of matters that arose as areas of heightened stakeholder interest during the year.

Medscheme and Bonitas governance and legal proceedings

Governance and procurement matters relating to Bonitas Medical Scheme received media attention during the year and were subject to investigation by the CMS. In late 2025, Medscheme initiated legal proceedings in the High Court regarding the tender process for administration and managed care services. In early 2026, Bonitas concluded the RFP process by awarding contracts to alternative providers. Further discussion of this development appears in the Chairman’s review on page 6, the CEO’s review on page 54, and the strategy and approach section on page 58.

Stakeholders	Related material matters
- Government and regulators - Clients - Shareholders and investors	- Business continuity and adaptation - Legal, regulatory and compliance management - Sustainable financial performance - Employee wellbeing and retention

Fedhealth and Medshield potential amalgamation

During 2025, Fedhealth Medical Scheme and Medshield Medical Scheme engaged in constructive discussions regarding a potential amalgamation, reflecting broader consolidation trends in the medical schemes industry. The Boards of both schemes undertook thorough and independent actuarial assessments to evaluate whether an amalgamation would be in the best interests of their respective members.

Following careful consideration of these assessments, the Boards concluded that proceeding with a full amalgamation at this time would not optimally serve member needs, and it was mutually agreed to conclude the formal amalgamation discussions. Through Medscheme, we remain fully committed to delivering high-quality services to both Fedhealth and Medshield. Our existing contractual relationships remain firmly in place and we will continue to provide exceptional support and value to both schemes.

Stakeholders	Related material matters
- Government and regulators - Clients - Shareholders and investors	- Legal, regulatory and compliance management - Sustainable financial performance

NHI

AfroCentric supports the principles underpinning universal health coverage and the aim of improving access to quality healthcare. We continue to monitor legislative and policy developments around NHI, including ongoing legal and procedural challenges that may influence timing, implementation design and stakeholder confidence.

The BHF, together with other stakeholders, has instituted legal proceedings challenging the constitutionality and procedural validity of the NHI Act. During 2025, the High Court ordered the President to disclose the full record of decision underpinning the enactment of the NHI Act, enabling the applicants to advance their constitutional challenge. The matter remains ongoing.

AfroCentric believes the long-term success of healthcare reform depends on a collaborative, phased public-private approach that leverages existing capacity, protects continuity of care and maintains system sustainability. We remain committed to constructive engagement through relevant industry platforms while continuing to position our integrated model to support affordability and access in a changing policy environment.

Stakeholders	Related material matters
- Government and regulators - Clients - Shareholders and investors	- Access to healthcare and medicine - Business continuity and adaptation - Legal, regulatory and compliance management

Call for regulatory reform on LCBOs

In 2025, the CMS published recommendations opposing the continuation of LCBOs and proposing the phasing out of currently exempt PHI products. This was followed by a formal Government Gazette call for public comment by the NDoH. AfroCentric continues to engage with stakeholders and regulators to support sustainable, affordable healthcare solutions while monitoring the potential regulatory implications for the sector.

Stakeholders	Related material matters
- Government and regulators - Clients - Shareholders and investors	- Access to healthcare and medicine - Business continuity and adaptation - Legal, regulatory and compliance management

Neil Harvey and Associates (NHA)

Following an 18-year arbitration process, Medscheme welcomed the arbitrator’s decision dismissing all claims brought by NHA as baseless, with costs awarded in Medscheme’s favour. A review of the decision has been launched by NHA, and stakeholders will be updated as the process progresses.

Stakeholders	Related material matters
- Shareholders and investors	- Legal, regulatory and compliance management

Section 59 Inquiry

In July 2025, the CMS published the final report of the Section 59 Investigation, which examined allegations of unfair racial discrimination and procedural unfairness in the implementation of Fraud, Waste and Abuse (FWA) systems across parts of the medical schemes industry.

The panel’s findings, while not legally binding, have contributed to broader public and stakeholder discussion regarding fairness and potential bias in fraud detection practices. The review reflected on historical system-level outcomes, based on data from 2012 to 2019, and noted that certain approaches resulted in differential impacts on providers. The panel also recognised the material enhancements implemented by administrators, including Medscheme, since 2021 to strengthen governance, transparency and procedural fairness in FWA processes.

The report did not result in sanctions or penalties and did not make findings of intentional discrimination. While aspects of the report’s methodology have been questioned by industry participants, the Group remains committed to fair, lawful and transparent claims management practices and continues to assess regulatory developments arising from the inquiry.

Stakeholders	Related material matters
- Industry bodies - Clients - Shareholders and investors	- Legal, regulatory and compliance management - Ethical business conduct and good governance

Our risks and opportunities

A disciplined and mature risk management approach underpins the execution of our strategy and enables the proactive identification of opportunities across the Group.

Risk management at AfroCentric is overseen by the Board and its committees and is supported by established governance structures. These structures are chaired by members of the senior management team and operate with clear accountability to the relevant Board committees. The Group Risk Management function provides effective oversight, coordination and assurance to ensure that risk management processes are consistently applied and appropriately embedded across the organisation.

Through our robust risk management processes, we aim not only to mitigate potential threats but also to identify and capitalise on opportunities that support sustainable value creation.

Enterprise risk management (ERM) framework

AfroCentric's ERM framework is aligned with the principles of King IV™, the committee of Sponsoring Organizations of the Treadway Commission (COSO) ERM framework, and the International Organization for Standardization's ISO 31000:2018 risk management standard. The framework provides a structured, integrated and enterprise-wide approach to the identification, assessment, management and monitoring of risks and opportunities.

Insight into the Group's risk landscape is obtained through the systematic consideration of both external and internal factors that may positively or negatively influence the achievement of our strategic objectives.



Identification

Risk identification is an ongoing and structured process through which the Group identifies strategic, operational, financial, regulatory and emerging risks that may affect the achievement of its objectives. This process considers both internal and external factors, including changes in the operating environment, stakeholder expectations and industry developments. Identified risks are captured in the relevant risk registers to ensure enterprise-wide visibility and accountability.

Risk Assessment

Identified risks are assessed to determine their potential impact and likelihood, taking into account existing controls and the Group's risk appetite and tolerance. This assessment enables the prioritisation of risks based on their significance to the Group's strategic objectives and value creation. Risks are evaluated using a consistent methodology to ensure comparability and informed decision-making.

Risk mitigation and remediation

Risk mitigation focuses on the design and implementation of appropriate controls and management actions to reduce risks to acceptable levels. Where risks materialise or controls are ineffective, remediation plans are developed and executed within agreed timelines. Mitigation strategies are aligned to the Group's strategy, risk appetite and available resources to support sustainable outcomes.

Risk Monitoring

Risks and associated controls are continuously monitored to assess their effectiveness and to identify changes in the risk profile. Key risk indicators, incidents and emerging risks are tracked to ensure timely escalation and management intervention where required. This ongoing monitoring supports proactive risk management and responsiveness to a dynamic operating environment.

Risk reporting

Risk reporting provides transparent, accurate and timely information to management, Board committees and the Board to support oversight and decision-making. Reports include updates on key risks, emerging risks, control effectiveness, incidents and progress against mitigation actions. This reporting framework enhances accountability and ensures alignment between risk management, strategy execution and governance.

Overview of our top risks and related opportunities

Please see our material matter discussion on page 46 for information on how our top risks are integrated into our materiality process.

1. Execution risk – winning health offering

Source of risk: Internal and external Overall risk severity: Very high

Description

Failure to successfully scale the Group’s health offering, including Fedhealth, in line with strategic growth ambitions may constrain the achievement of Vision 2030 and limit the Group’s ability to build a sustainable, diversified revenue base.

Circumstances under which the risk may arise

This risk may arise due to challenges in product design and value proposition competitiveness, underperforming distribution channels, ineffective marketing or brand positioning, service delivery and customer experience constraints, affordability pressures in the market, regulatory complexity, heightened competition, and uncertainty related to the current form of NHI.

Impact on value creation/preservation and potential for value erosion

If not effectively managed, this risk could result in the non-achievement of strategic objectives, increased exposure to concentration risk, reduced revenue growth, and pressure on the Group’s long-term sustainability and scale.

Response and mitigating actions to preserve value:

The Group continues to strengthen its health offering through integrated distribution with Sanlam, enhanced marketing and brand repositioning initiatives, and disciplined execution supported by formal project governance. Strategic initiatives are prioritised through defined qualification criteria, with phased funding and disciplined capital allocation.

Identified opportunities to create value:

Effective execution of the health offering strategy enables membership growth, improved market penetration, enhanced customer experience and the development of a more resilient and diversified revenue base aligned with the Group’s long-term strategy.

How this risk is monitored

Membership growth and retention trends, product performance indicators, customer experience metrics, distribution effectiveness, and progress against key strategic initiatives.

Board committees overseeing the risk



Capitals impacted



Affected strategic priorities



2. Execution risk – managed care leadership through clinical innovation



Source of risk: Internal and external Overall risk severity: Very high

Description

Failure to successfully deliver and embed key clinical innovation initiatives may limit the Group’s ability to strengthen its managed care leadership position and achieve strategic objectives over the medium to long term.

Circumstances under which the risk may arise

This risk may arise due to weak concept development or validation, insufficient integration of clinical innovation with digital, technology-enabled and data-driven capabilities, ineffective project governance, limited client and provider buy-in, skills and capability constraints, inadequate monitoring of return on investment, insufficient investment in research and development, and uncertainty associated with the current form of the NHI.

Impact on value creation/preservation and potential for value erosion

If not effectively managed, this risk could result in the non-achievement of key Vision 2030 imperatives, reduced ability to future-proof the business, and a managed care offering that becomes less relevant or attractive to clients.

Response and mitigating actions to preserve value:

The Group applies defined strategic project qualification criteria to prioritise clinical innovation initiatives, supported by phased funding and disciplined capital allocation. Delivery is overseen through formal project governance structures, with regular progress reporting to management and the Board to ensure alignment with strategic outcomes.

Identified opportunities to create value:

Effective execution of clinical innovation initiatives strengthens the Group’s value-based care proposition, enhances differentiation in the market, and supports sustainable growth through improved clinical outcomes and operational efficiency.

How this risk is monitored

Progress against strategic clinical innovation initiatives, delivery milestones, client adoption indicators, benefits realisation trends and alignment with strategic performance objectives.

Board committees overseeing the risk



Capitals impacted



Affected strategic priorities



Trend improving

Trend worsening

Remains unchanged

3. Concentration risk

Source of risk: Internal and external **Overall risk severity:** Very high

Description

Dependence on a limited number of large client contracts exposes the Group to revenue volatility should these contracts be lost, restructured or not renewed, potentially constraining financial performance and medium-term sustainability.

Circumstances under which the risk may arise

This risk may arise from strained relationships with key schemes and clients, service delivery, or service-level agreement (SLA) performance issues, misalignment of strategic objectives and values, unsuccessful responses to tender or proposal processes, or changes in client procurement strategies.

Impact on value creation/preservation and potential for value erosion

If not effectively managed, the loss of key client contracts could result in a significant reduction in revenue, adverse impacts on financial performance, and heightened risk to the Group's sustainability in the short to medium term.

Response and mitigating actions to preserve value:

The Group continues to reduce concentration risk through revenue diversification initiatives, including the development of an integrated open-market medical scheme aligned with the AfroCentric and Sanlam value proposition. Additional focus areas include expanding revenue opportunities within existing schemes, intensifying growth in corporate schemes, and increasing investment in PHI, supported by active stakeholder relationship management.

Identified opportunities to create value:

Reducing reliance on individual contracts strengthens the resilience of the Group's revenue base, supports sustainable growth, and enables expansion into broader market segments with diversified income streams.

How this risk is monitored

Client concentration metrics, contract renewal and retention outcomes, revenue mix trends, service performance indicators, and stakeholder engagement feedback.

Board committees overseeing the risk



Capitals impacted



Affected strategic priorities



4. Execution risk – data strategy

NEW

Source of risk: Internal **Overall risk severity:** High

Description

Failure to successfully deliver and embed the Group's data strategy may limit the availability of accurate, timely and reliable information, constraining decision-making, reporting quality and service delivery to clients.

Circumstances under which the risk may arise

This risk may arise due to fragmented data sources and data lakes resulting in the absence of a single source of truth, data integrity challenges, an ineffective operating model for managing data demand, or reliance on outdated data platforms and technologies.

Impact on value creation/preservation and potential for value erosion

If not effectively managed, this risk could impair the Group's ability to provide high-quality reports and insights to management and clients, hinder the achievement of service-level commitments, result in inaccurate reporting, and expose the Group to potential penalties.

Response and mitigating actions to preserve value:

The Group is implementing a target operating model for data management, supported by improved ways of working and the modernisation of data platforms. Execution of the approved data strategy remains a strategic priority, with progress monitored through formal governance and management oversight.

Identified opportunities to create value:

Successful execution of the data strategy enhances decision-making, strengthens reporting capability, improves operational efficiency and enables greater value extraction from data assets across the Group.

How this risk is monitored

Data quality and availability indicators, reporting timeliness, service-level performance, progress against data platform modernisation initiatives and governance oversight of strategic delivery.

Board committees overseeing the risk



Capitals impacted



Affected strategic priorities



5. Stakeholder relationship management

Source of risk: Internal and external **Overall risk severity:** High

Description

Failure to build, maintain and strengthen constructive relationships with key stakeholders and clients may undermine trust, limit growth opportunities and adversely affect operational and financial performance.

Circumstances under which the risk may arise

This risk may arise from the absence of an effective stakeholder relationship management strategy, service delivery failures or underperformance against service-level commitments, and misalignment of values or strategic priorities between the Group and its stakeholders.

Impact on value creation/preservation and potential for value erosion

If not effectively managed, this risk could result in disengaged stakeholders, reduced ability to leverage relationships for business growth, loss of contracts, operational strain and increased exposure to penalties.

Response and mitigating actions to preserve value:

The Group has implemented a revised operating model aimed at reducing service failures and strengthening client relationships. Ongoing, structured engagement with stakeholders is maintained to proactively manage expectations, reinforce alignment and address emerging issues.

Identified opportunities to create value:

Strong stakeholder relationships enhance trust, support long-term partnerships, enable business growth opportunities and contribute to improved service delivery and organisational resilience.

How this risk is monitored

Stakeholder engagement feedback, service-level performance trends, contract retention outcomes, client experience indicators and escalation frequency.

Board committees overseeing the risk

Capitals impacted

Affected strategic priorities



6. Regulatory uncertainty – NHI Act

Source of risk: External **Overall risk severity:** Very high

Description

Ongoing uncertainty surrounding the implementation and timing of NHI presents a significant risk to the operating environment of medical aid administrators and related healthcare services.

Circumstances under which the risk may arise

This risk may arise due to health policy decisions, limited alignment between government and industry stakeholders, and the outcome of legal and regulatory processes related to the implementation of the NHI Act.

Impact on value creation/preservation and potential for value erosion

If not effectively managed, adverse regulatory developments could pose an existential threat to the medical aid industry and materially impact the sustainability of medical aid administrators and associated service providers.

Response and mitigating actions to preserve value:

The Group continues to engage with national and provincial health authorities, industry bodies and policymakers to contribute to constructive policy dialogue and advocate for sustainable healthcare outcomes. Parallel efforts focus on revenue diversification initiatives and structured communication with stakeholders to maintain transparency and organisational readiness.

Identified opportunities to create value:

Active participation in healthcare policy development positions the Group to adapt its operating model, explore alternative service offerings and identify new opportunities aligned with evolving public healthcare priorities.

How this risk is monitored

Regulatory and legislative developments, policy announcements, industry engagement outcomes and formal communications from relevant authorities.

Board committees overseeing the risk

Capitals impacted

Affected strategic priorities



7. Profit and earnings optimisation



Source of risk: Internal and external **Overall risk severity:** High

Description

Failure to achieve budgeted financial performance and earnings growth may constrain the Group's ability to deliver on strategic priorities, sustain investor confidence and reinvest in initiatives that support long-term value creation.

Circumstances under which the risk may arise

This risk may arise from adverse market conditions, competitive pricing pressures, loss of business or contracts, ineffective monitoring of performance against budget, or insufficient cost-containment measures.

Impact on value creation/preservation and potential for value erosion

If not effectively managed, underperformance against budget could result in reduced investor and shareholder confidence, slower business growth, constrained strategic investment, and negative impacts on employee remuneration and morale.

Response and mitigating actions to preserve value:

The Group maintains strong financial governance through regular review of financial performance by management, Board committees and the Board. Performance against budget and forecast is monitored on an ongoing basis, supported by disciplined cost management, focused performance oversight and timely corrective action where required.

Identified opportunities to create value:

Enhanced financial discipline and data-driven performance management support improved capital allocation, operational efficiency and the delivery of sustainable earnings growth.

How this risk is monitored

Budget and forecast performance trends, cost efficiency indicators, revenue performance by business cluster, and outcomes of Board and sub-committee financial reviews.

Board committees overseeing the risk

Capitals impacted

Affected strategic priorities



8. Regulatory burden and compliance risk

NEW

Source of risk: Internal and external **Overall risk severity:** High

Description

A complex and evolving regulatory environment increases the risk of non-compliance, which may disrupt operations and adversely affect the Group's ability to operate effectively and sustainably.

Circumstances under which the risk may arise

This risk may arise due to ongoing regulatory developments and reforms, increased scrutiny and oversight by regulatory authorities, ineffective compliance monitoring processes, or weaknesses in governance and oversight structures.

Impact on value creation/preservation and potential for value erosion

If not effectively managed, regulatory non-compliance could result in fines, sanctions or other enforcement actions, potential loss of operating licences or accreditations, increased regulatory scrutiny and reputational damage.

Response and mitigating actions to preserve value:

The Group continues to strengthen regulatory monitoring through enhanced compliance self-assessment processes, proactive compliance alerts and the execution of its compliance risk management plan. Ongoing engagement with regulators, lawmakers and industry bodies supports early identification of regulatory developments and alignment with evolving requirements.

Identified opportunities to create value:

A strong compliance culture and proactive regulatory engagement enhance organisational resilience, support sustainable operations and reinforce stakeholder confidence in the Group's governance practices.

How this risk is monitored

Compliance incident trends, outcomes of compliance self-assessments, regulatory feedback, audit findings and execution of compliance risk management initiatives.

Board committees overseeing the risk

Capitals impacted

Affected strategic priorities



9. System stability and availability



Source of risk: Internal and external **Overall risk severity:** High

Description

Unavailability or unreliability of critical IT systems may disrupt business operations, impair service delivery and negatively affect the Group's reputation and client relationships.

Circumstances under which the risk may arise

This risk may arise due to ageing infrastructure, reliance on unsupported hardware or software, ineffective licence management, third-party service dependencies, or insufficient proactive monitoring of system capacity and performance requirements.

Impact on value creation/preservation and potential for value erosion

If not effectively managed, system instability could result in business disruptions, failure to meet client service-level commitments, financial penalties, reputational harm and reduced client satisfaction and retention.

Response and mitigating actions to preserve value:

The Group has strengthened system resilience through the relocation of data centres to a more robust infrastructure environment, supported by ongoing monitoring of system performance and service dependencies. Focus remains on maintaining stability, managing third-party risks and ensuring continuity of critical services through appropriate oversight and controls.

Identified opportunities to create value:

Improved system stability enhances operational resilience, supports consistent service delivery, strengthens client trust and enables a more efficient use of technology to support business growth.

How this risk is monitored

System availability and performance indicators, service-level outcomes, incident and outage trends, third-party service performance and technology governance oversight.

Board committees overseeing the risk



Capitals impacted



Affected strategic priorities



10. Talent management



Source of risk: Internal and external **Overall risk severity:** Medium

Description

Inability to attract, develop and retain critical skills, including the maintenance of a robust leadership and specialist talent pipeline, may constrain the Group's capacity to execute its strategy and sustain long-term performance.

Circumstances under which the risk may arise

This risk may arise from challenges in employer brand positioning, an uncompetitive employee value proposition, remuneration structures that do not align with market expectations, organisational culture factors, perceptions of the healthcare administration industry, and uncertainty regarding the long-term sustainability of the sector.

Impact on value creation/preservation and potential for value erosion

If not effectively managed, talent constraints could result in insufficient future skills capacity, weakened strategic and operational execution, operational inefficiencies, and increased risk to business continuity.

Response and mitigating actions to preserve value:

The Group continues to strengthen its talent management approach through enhanced recruitment and retention frameworks, leadership and succession planning, targeted development programmes and ongoing culture and employee experience initiatives. Alignment with broader Group talent platforms supports access to wider skills pools and consistent people practices.

Identified opportunities to create value:

A strong talent pipeline and engaged workforce support strategic execution, leadership continuity, improved organisational performance and the development of a resilient, future-ready organisation.

How this risk is monitored

Talent attraction and retention trends, succession coverage, leadership development outcomes, employee engagement indicators and workforce capability assessments.

Board committees overseeing the risk



Capitals impacted



Affected strategic priorities



Our material matters

Why materiality matters to AfroCentric

Our materiality process identifies the matters that substantively affect AfroCentric's ability to create and preserve value and mitigate value erosion over the short, medium and long term, as well as the matters through which our business has the most significant impacts on the economy, society and the environment. These matters shape how we assess our operating context, make strategic decisions, allocate resources, manage risk and report on performance.

How we determine our material matters

For 2025, we refined our materiality assessment to strengthen alignment with current best practice and to ensure that this integrated report remains focused on the issues most relevant to our strategy, business model, stakeholders and long-term prospects. In doing so, we considered materiality through a double materiality lens. This means that we assessed both the significance of matters to AfroCentric's ability to create and preserve financial value over time, and the significance of AfroCentric's impacts on people, society and the environment.

This approach strengthens our transparency, supports responsible healthcare stewardship, and provides stakeholders with a richer understanding of AfroCentric's role and influence within the broader health ecosystem.

Our process can be summarised in three stages:

1

Identify

We began by assessing the external environment, stakeholder expectations, strategic priorities, and the risks and opportunities that may influence AfroCentric's ability to create value over time. Our assessment process drew on a range of internal and external inputs. These included our prior year material matters, the Group risk register, peer and sector analysis, investor and stakeholder perspectives, selected internal reports, media and market insights, macroeconomic and healthcare sector trends, the Group's refreshed strategy and business model, and applicable reporting guidance. This allowed us to assess if the issues disclosed in previous periods remained relevant, whether their context needed refinement, and if any additional matters should be included to better reflect current and future realities.

This step ensures that our materiality process is rooted in a comprehensive understanding of our context.

2

Prioritise

Potential material matters were reviewed, tested, and ranked based on their significance from both financial and impact perspectives. A structured materiality workshop formed a core part of this stage. Participants assessed each proposed matter for relevance, areas of overlap or omission, strategic connectivity and overall importance to decision-making. The exercise confirmed the continued relevance of previously disclosed matters and supported refinement of wording, emphasis and ordering.

3

Integrate

Once finalised, our material matters are embedded into our strategy, risk management processes and governance oversight. They serve as the organising lens for our integrated report and guide how we communicate AfroCentric's ability to create and preserve value over time.

The material matters identified for 2025 reflect the realities of AfroCentric's operating environment. They highlight the need to improve access to affordable, quality healthcare; respond to evolving market and regulatory dynamics; strengthen operational resilience; accelerate digital transformation; safeguard information assets; attract and retain critical skills; maintain stakeholder trust; and ensure financial resilience. They also acknowledge the importance of responsible environmental stewardship and the broader economic, political, geopolitical and societal developments shaping our business and the communities we serve.

How materiality shapes this report

In line with the principles of integrated reporting, we have used our material matters to shape the narrative of this report. For each material matter, we explain why it is important in the current year, how it links to strategy and risk, how we are responding, and where related information can be found elsewhere in the report. This is intended to strengthen connectivity across the report and to provide stakeholders with a clearer understanding of the issues that matter most to AfroCentric's ability to create and preserve value while mitigating value erosion where possible.

Description

Expanding access to quality healthcare services and affordable medicine remains AfroCentric's most material matter. It is central to the Group's purpose and strategy, particularly in a context of rising healthcare costs, affordability pressures, service gaps and the need for more integrated, outcomes-led care. This matter reflects AfroCentric's role in improving access, strengthening healthcare coordination and supporting better health outcomes across public and private settings.

Strategic objectives

Building a winning health offering in partnership with Sanlam; Strengthening managed care leadership through clinically driven innovation.

Related risks

Execution risk – winning health offering | Execution risk – managed care leadership through clinical innovation | Regulatory uncertainty – NHI.

Key stakeholders

Clients | Employees | Government and regulators | Industry bodies | Shareholders and investors.

Related capitals



How this matter is covered in this report

- Strengthening the integrated health platform (pages 54 and 58)
- Expanding Health Rewards by Sanlam (pages 54 and 58)
- Progressing value-based care initiatives (pages 54 and 58)
- Strengthening pharmacy benefit management (page 58)
- Maintaining affordability and continuity of care across schemes and public-sector medicine delivery (pages 6, 54 and 58)

2

Business continuity and adaptation

Description	Business continuity and adaptation reflects AfroCentric's need to remain resilient and responsive in a changing operating environment. This includes protecting service delivery, maintaining operational continuity and adapting to external developments, such as industry consolidation and market shifts. In 2025, this matter became more pronounced as the Group responded to a materially changed operating context and recalibrated elements of its strategic approach.
Strategic objectives	Building a winning health offering in partnership with Sanlam; Strengthening managed care leadership through clinically driven innovation; Investing in data, digital and technology; Evolving our culture and operating model to support the growth strategy.
Related risks	Concentration risk Stakeholder relationship management System stability and availability Profit and earnings optimisation.
Key stakeholders	Clients Employees Shareholders and investors.
Related capitals	
How this matter is covered in this report - Implementing the Board-approved turnaround programme (pages 6, 54 and 58) - Resetting the cost base (pages 54, 58 and 64) - Reorganising around a leaner operating model (pages 54, 58 and 64) - Simplifying the portfolio (pages 54, 58 and 64) - Protecting service delivery while preparing for revised scale (pages 54 and 58)	

3

Digital transformation and resilience

Description	Digital transformation and resilience remains a key material matter as digital, data and technology capabilities are increasingly foundational to cost-effective healthcare delivery, operational efficiency, innovation and client service. This matter reflects AfroCentric's continued investment in healthcare technology platforms, enterprise data capability and digital engagement, as well as the need to ensure these systems are resilient, scalable and aligned to a more integrated health platform model.		
Strategic objectives	Investing in data, digital and technology.		
Related risks	Execution risk – data strategy System stability and availability.		
Key stakeholders	Clients Employees Shareholders and investors.		
Related capitals		How this matter is covered in this report - Relocating data centres to strengthen resilience (pages 54 and 58) - Progressing data modernisation (page 58) - Establishing a single source of truth through enterprise data exchange (page 58) - Advancing AI-enabled automation (page 58) - Enhancing digital member and provider platforms (pages 54 and 58) - Improving hospital claims automation to 76% (Page 58)	

4

Legal, regulatory and compliance management

Description	Legal, regulatory and compliance management remains material given the highly regulated environment in which AfroCentric operates. This matter reflects the need to comply with laws, policies and regulatory requirements while proactively managing compliance risks, maintaining strong governance and responding to an evolving healthcare policy landscape. Its significance is heightened by ongoing regulatory developments and increased industry scrutiny.
Strategic objectives	Evolving our culture and operating model to support the growth strategy.
Related risks	Regulatory uncertainty – NHI Act Regulatory burden and compliance risk.
Key stakeholders	Clients Employees Government and regulators Industry bodies Shareholders and investors.
Related capitals	
How this matter is covered in this report - Ongoing engagement on NHI and LCBO developments (page 33) - Strengthened compliance monitoring (page 10) - Material enhancements implemented since 2021 noted as part of the Section 59 process (pages 6, 10 and 33) - Active legal stewardship on Bonitas and Neil Harvey and Associates matters (pages 6, 10 and 54)	

5

Sustainable financial performance

Description	Sustainable financial performance is critical to AfroCentric's ability to invest in growth, absorb change, protect resilience and continue delivering value to clients, members, shareholders and other stakeholders. This matter reflects the need to strengthen the Group's market position and financial sustainability by anticipating industry trends, meeting evolving client needs and differentiating services in a competitive and regulated environment.
Strategic objectives	Building a winning health offering in partnership with Sanlam; evolving our culture and operating model to support the growth strategy.
Related risks	Concentration risk Profit and earnings optimisation Execution risk – winning health offering.
Key stakeholders	Clients Employees Shareholders and investors.
Related capitals	
How this matter is covered in this report - Embedding cost containment measures (pages 54 and 64) - Applying capital discipline (page 64) - Resetting the cost base (pages 54 and 64) - Simplifying the portfolio (pages 54 and 64) - Focusing on profitable lines and sustainable growth areas (pages 54, 58 and 64)	

6

Cybersecurity and information security

Description	Cybersecurity and information security is material because AfroCentric's ability to deliver trusted, resilient healthcare services depends on secure systems, protected data and stable technology infrastructure. As the Group deepens digital enablement and data integration, the need to protect systems and data against breaches and disruptions becomes increasingly important. This matter underpins stakeholder trust, service continuity and operational resilience.
Strategic objectives	Investing in data, digital and technology.
Related risks	System stability and availability Execution risk – data strategy.
Key stakeholders	Clients Employees Government and regulators Industry bodies Shareholders and investors.
Related capitals	
How this matter is covered in this report - Strengthening infrastructure resilience (pages 54 and 58) - Relocating data centres (pages 54 and 58) - Improving platform stability pages 54 and 58) - Enhancing data governance and digital architecture (page 58) - Embedding secure and resilient information systems as a foundation for healthcare delivery (pages 54 and 58)	

7

Member satisfaction

Description	Member satisfaction remains a core material matter because the quality of AfroCentric's service delivery, healthcare experience and health outcomes directly influences loyalty, trust, competitiveness and long-term sustainability. This matter reflects the need to exceed member expectations by delivering high-quality service while improving health outcomes and promoting wellness through more integrated and preventative healthcare solutions.
Strategic objectives	Building a winning health offering in partnership with Sanlam; strengthening managed care leadership through clinically driven innovation.
Related risks	Stakeholder relationship management Execution risk – winning health offering System stability and availability.
Key stakeholders	Clients Employees Government and regulators Industry bodies Shareholders and investors.
Related capitals	
How this matter is covered in this report - Improving customer effort score trends (pages 54 and 58) - Maintaining service excellence during a challenging contract cycle (pages 54 and 58) - Enhancing digital engagement (page 58) - Driving provider portal adoption (page 58) - Recognition through the AskAfrica Orange Index Awards (page 58)	

8

Employee wellbeing and retention

Description	Employee wellbeing and retention is material because AfroCentric's ability to execute its strategy, innovate and maintain service quality depends on attracting, developing and retaining the right skills. In a healthcare environment shaped by digital change, transformation and rising service expectations, workforce capability, resilience and engagement remain essential. This matter also extends to building a workforce equipped for future healthcare needs.
Strategic objectives	Evolving our culture and operating model to support the growth strategy.
Related risks	Talent management.
Key stakeholders	Clients Employees Government and regulators Shareholders and investors.
Related capitals	   
How this matter is covered in this report - Refreshing Group values (page 76) - Using pulse surveys to monitor employee sentiment (page 76) - Providing wellbeing programmes and primary healthcare access (page 76) - Tracking Employee Assistance Programme utilisation (page 76) - Investing in learning and development (page 76) - Recognition as a Top Employer (pages 54 and 76)	

9

Ethical business conduct and good governance

Description	Ethical business conduct and good governance remain material because it underpins stakeholder trust, strategic oversight and the integrity of decision-making across the Group. This includes upholding high standards of ethical behaviour, transparency and governance, including Board effectiveness, remuneration and broader governance practices. In a complex and regulated operating environment, strong governance supports legitimacy, resilience and long-term sustainability.
Strategic objectives	Evolving our culture and operating model to support the growth strategy.
Related risks	Regulatory burden and compliance risk Stakeholder relationship management.
Key stakeholders	Clients Employees Government and regulators Industry bodies Shareholders and investors.
Related capitals	   
How this matter is covered in this report - Maintaining Board and committee oversight (pages 6 and 10) - Strengthening ethical and compliance frameworks (page 10) - Applying zero tolerance for unethical conduct (page 10) - Maintaining whistleblowing mechanisms (page 10) - Overseeing ESG and transformation matters (pages 10 and 76) - Disclosing key governance practices and Board deliberations (pages 6 and 10)	

Description	This material matter recognises that AfroCentric's performance and prospects are shaped not only by internal execution, but also by broader external forces. Economic pressures, transformation imperatives, political developments, geopolitical uncertainty and wider societal trends all influence affordability, demand, regulation, stakeholder expectations and business resilience. It reflects the need to remain responsive to the broader system in which AfroCentric operates.
Strategic objectives	Building a winning health offering in partnership with Sanlam; evolving our culture and operating model to support the growth strategy.
Related risks	Regulatory uncertainty – NHI Act Profit and earnings optimisation Concentration risk Regulatory burden and compliance risk Stakeholder relationship management Talent management related risk.
Key stakeholders	Clients Employees Government and regulators Industry bodies Shareholders and investors.

Related capitals**How this matter is covered in this report**

- Ongoing policy engagement (pages 33 and 40)
- Protecting affordability and access (pages 31 and 58)
- Maintaining Level 1 B-BBEE status (page 76)
- Advancing enterprise and supplier development (page 76)
- Driving a more disciplined operating model in response to macro and sector pressures (pages 54, 58 and 64)

Description	Pollution, waste and climate impacts remain material as AfroCentric continues to strengthen our understanding of the environmental impacts associated with our operations and value chain. They remain relevant to responsible stewardship, long-term resilience and the evolving expectations of stakeholders and reporting frameworks. This combined material matter reflects the need to manage pollution and waste responsibly while considering transition and physical climate-related impacts in an appropriate business context.
Strategic objectives	Evolving our culture and operating model to support the growth strategy.
Related risks	Climate change is recognised as a strategic risk that continues to be monitored; however, it currently falls outside the Group's top ten risks.
Key stakeholders	Clients Employees Government and regulators Industry bodies Shareholders and investors.

Related capitals**How this matter is covered in this report**

- Incorporating upstream pharmaceutical manufacturing and packaging emissions into the GHG inventory (page 86)
- Installing solar systems to supplement electricity usage (page 86)
- Investing in borehole infrastructure (page 86)
- Ensuring waste and recycling management (page 86)

Responding strategically



CEO's review

The year under review demanded clarity of purpose, resilience in execution and disciplined leadership. Guided by Vision 2030, we remained focused on transforming healthcare by expanding access, improving affordability and delivering better outcomes through integrated, value-driven care. We moved decisively from strategy to execution, making deliberate and at times difficult choices to strengthen the business's foundations for long-term sustainability.

Gerald van Wyk
Group CEO



The work we do carries real consequence. Every decision influences affordability, access and outcomes across a system under pressure. This responsibility shapes how we lead, how we invest and how we respond when conditions are challenging. It has reinforced our commitment to a clinically integrated, digitally enabled healthcare platform capable of delivering consistently and at scale.

Delivering resilient performance in a demanding operating environment

The year unfolded within a challenging context, characterised by margin pressure in parts of the pharmaceutical value chain, contract-cycle developments, and subdued activity across certain healthcare segments. Against this backdrop, AfroCentric delivered a resilient operational performance, supported by disciplined execution and proactive cost management.

Total revenue for the year declined by 10% to R7.3 billion compared to a comparable twelve months period to December 2024, largely due to contract losses, lower private patient script volumes following the discontinuation of certain designated service provider arrangements, margin pressure in selected pharmaceutical products and lower membership in some medical schemes. Management responded by implementing cost-containment measures across the Pharmacy Direct private business, helping stabilise operating margins and protect the Group's cash-generating capability.

Reported financial results were significantly impacted by once-off, non-cash adjustments relating to the impairment of goodwill. Following the announcement of the loss of the Bonitas Medical Scheme administration and managed care contracts effective 31 May 2026, the Group undertook a comprehensive impairment assessment of goodwill, resulting in impairments of R810.9 million. This non-cash impairment resulted in a reported loss from continuing operations of R645.9 million. The reported loss was further exacerbated by an R613.3 million loss from discontinued operations related to Activo's operational performance, which underperformed during the year. This loss was recognised under discontinued operations. It is a once-off event and does not affect the Group's future operating performance.

While these adjustments materially affected reported earnings for the year, they do not reflect the Group's underlying operating strength. On a normalised basis, excluding these once-off items, AfroCentric generated profit from continuing operations of R268.4 million, demonstrating the continued resilience of our core healthcare administration and pharmaceutical distribution businesses despite sector-wide pressures.

These adjustments represent an important reset of asset values and position the Group for long-term sustainability. Throughout the year, we maintained a disciplined focus on capital management and liquidity, strengthening the balance sheet, while continuing to invest in the capabilities required to support long-term growth.

Advancing clinically led innovation and value-based care

The shift to outcomes-based, coordinated care continues to accelerate, reinforcing our strategic direction. During the year, we refined our operating model to reduce fragmentation, strengthen end-to-end coordination and accelerate clinically led innovation. Targeted organisational changes were also implemented to address execution gaps, strengthen leadership accountability and enhance risk management.



Clinical innovation remains a key differentiator. Programmes such as the Orthocare Spinal Programme demonstrate how integrated, evidence-based care pathways can improve patient outcomes while reducing unnecessary procedures and associated costs.



Please see our strategy and approach discussion on page 58 and cluster review on page 72 for more information on this and other value-based care initiatives.

We also strengthened the integration of key capabilities across the Group. Scriptpharm was more closely integrated into Medscheme, enhancing alignment between managed care, pharmacy services and capitation functions.

These, among many other initiatives, support the practical realisation of value-based care through structured care pathways, coordinated provider engagement, and data-driven risk management, enabling us to improve access to appropriate, high-quality care for members across the healthcare continuum.

Building digital and data foundations for scale

Technology investment is not a support function; it is foundational to scaling cost-effective, high-quality healthcare delivery. We completed the relocation of our data centre infrastructure during the year, constituting one of the most complex technology programmes undertaken by the Group to date. The benefits were immediate, including improved system stability, resilience and responsiveness.

This migration provides the platform required to accelerate our data transformation journey and enable more advanced analytics and artificial intelligence capabilities. These capabilities will increasingly support improved risk management, care coordination and operational decision-making.

Beyond infrastructure, we advanced our Group data and platform architecture to enable secure, scalable integration across member, provider and scheme experiences. This supports outcomes-based contracting, risk management and a more connected healthcare journey.

Evolving our platform approach through partnerships and distribution

We continued to strengthen our partnership with Sanlam, recognising the importance of aligned capabilities across administration, managed care, data, technology, and clinical innovation. The integration of AfroCentric Distribution Services into Sanlam's distribution environment supports a more coordinated approach to taking healthcare solutions to market.

As part of this strategic alignment, AfroCentric concluded transactions to sell AfroCentric Distribution Services Group and Wellworx to Sanlam Life. These transactions support our strategic objectives and are intended to strengthen joint operational efficiency and market reach.

We also progressed Health Rewards by Sanlam for Fedhealth members through the Fedhealth member app, providing a digital platform for health activities, engagement and rewards. The roadmap continues to develop, with an enhanced rewards and loyalty programme milestone reflected for 1 January 2027.

Responding to strategic inflection points with realism and focus

The withdrawal of the Fedhealth and Medshield amalgamation represented an inflection point for the Group, reinforcing the complexity of scheme-level integration in the South African environment. Our strategic intent remains intact, but the pathway to scale has evolved, with increased emphasis on a broader health platform approach designed to enable participation across multiple schemes, employers and individuals.



We remain committed to delivering high-quality services to all our clients, and we continue to strengthen the capabilities that underpin our long-term strategy, including clinically led innovation, stronger digital and data enablement, and scalable partnership-led propositions.



For more information on the amalgamation discussions and outcome, please see our stakeholder matters in focus on page 33.

Managing contract-cycle developments with integrity and service continuity

Contract lifecycles are an inherent feature of our business model, requiring continuous focus on service excellence, client relationships and disciplined execution. During the year, several important contract developments occurred across the portfolio, reflecting the competitive dynamics and ongoing renewal cycles within the medical scheme environment.

As outlined in the Chairman's review, developments concerning the Bonitas Medical Scheme would have a significant impact on the Group. While the matter remains subject to legal and regulatory processes, management has remained focused on maintaining service continuity for members and clients to minimise disruption, while preparing the organisation for changes in the portfolio.

As we manage this uncertainty, service excellence remains non-negotiable. Throughout the year we continued to fulfil our contractual obligations and support members and clients with professionalism and care. This commitment was reflected in positive Customer Effort Score trends across the Group and in Bonitas taking first place in the Medical Aid category at the AskAfrica Orange Index Awards for a second consecutive year, demonstrating sustained strength in service experience outcomes.

In response to the Bonitas developments, a Board-approved programme was established to reposition the organisation around a leaner and more sustainable operating model. The programme focuses on resetting the cost base, stabilising core operations, simplifying the portfolio, and redesigning the operating model to ensure that our capabilities remain competitive and resilient.

The year also included other contract-cycle developments across the portfolio. Certain designated service provider contracts with Bonitas were not renewed, affecting aspects of the pharmaceutical cluster, including Pharmacy Direct, Activo and Curasana. In Namibia, the NHP managed care contract was also lost through a tender process and is currently transitioning to a new service provider.

At the same time, several important contract renewals and new business opportunities reinforced the strength of our core capabilities. The GEMS managed care, contribution and debt management contracts were successfully renewed, reflecting the scheme's continued confidence in our delivery capability. Subsequent to year-end, Medscheme was also awarded the full suite of administration and managed care contracts for Sisonke Health Medical Scheme.

While the developments relating to Bonitas have introduced near-term uncertainty, we remain confident in the strength and long-term resilience of our healthcare services platform. Medscheme continues to serve approximately 3.3 million beneficiaries across multiple schemes, supported by deep clinical expertise, advanced managed care capabilities and a long track record of delivering value to clients and members.

Strengthening our people, culture and leadership accountability

The year placed extraordinary demands on our people. Despite uncertainty and pressure, service levels were maintained, major programmes delivered, and members supported with professionalism and care. Recognition as a Top Employer for the third consecutive year reflects the strength of our culture and the dedication of colleagues across the Group.

This resilience is sustained through clarity of purpose, alignment and care for our people. As the Group moves forward, we remain focused on strengthening leadership capability, accountability and engagement, ensuring that the lived experience of our people continues to reflect the values that define AfroCentric.

Looking ahead with clarity and resolve

The year ahead will be defined by disciplined execution. Over the next 18 months, management will focus on implementing the turnaround actions required to stabilise the business while maintaining the service standards that underpin our reputation and client relationships.

This includes delivering the cost-optimisation measures already underway, embedding structural efficiencies across the organisation, and ensuring that these improvements translate

into a more sustainable operating model over time. At the same time, we remain focused on managing the transition of affected contracts responsibly and professionally, prioritising continuity of care for members and maintaining the trust of clients and partners.

As the organisation stabilises, we will continue progressing along the strategic pathway that differentiates AfroCentric in the healthcare ecosystem. Our long-term focus remains on clinically led innovation, value-based care models and the continued development of digital and data capabilities that enable better outcomes and more effective risk management.

In an increasingly competitive and outcomes-driven healthcare environment, sustainable growth is earned through measurable improvements in care delivery, affordability and member experience. Our priority is therefore clear: to stabilise the business, strengthen operational foundations and position AfroCentric to compete and succeed on the basis of demonstrable value.

For more detailed strategic outlook information, please refer to our strategy and approach section on page 58.

Appreciation

Our ability to execute with discipline is a direct result of the commitment and care our people bring to their work. In a year defined by complexity, pressure and strategic trade-offs, our teams continued to deliver with professionalism and an unwavering focus on achieving our purpose.

I am deeply grateful to our Board for their steadfast governance, thoughtful challenge and strategic guidance as we navigated a demanding operating environment.

Thank you to our clients, scheme members and partners for your trust, collaboration and shared dedication to improving healthcare access, quality and sustainability.

Together, we are shaping a system that better meets today's needs and is prepared for the challenges and opportunities of tomorrow.



Gerald van Wyk

Group CEO

Our strategy and approach

AfroCentric's long-term ambition remains focused on broadening access to affordable, quality healthcare and enhancing value across the healthcare value chain. While the strategic intent of Vision 2030 remains unchanged, 2025 marked an important turning point for the Group as changing market dynamics and recent developments highlighted the need for greater focus, operational discipline and strategic alignment across the business.

Against this backdrop, AfroCentric is executing its strategy through a health platform model that integrates administration, managed care, pharmaceutical and healthcare services capabilities to deliver improved healthcare outcomes and affordability.

Our strategy is anchored by two core growth drivers:

- Building a winning health offering in partnership with Sanlam
- Strengthening our managed care leadership through clinically driven innovation

Enabled by:

Investing in data, digital and technology

Evolving our culture and operating model to support the growth strategy

We aim to deliver long-term value through innovation, digital transformation, and a cohesive operating framework, directly contributing to SDG 3 and NDP Goal 8, prioritising access to high-quality, affordable healthcare for all South Africans.



Building a winning health offering in partnership with Sanlam



As part of the Sanlam Group, AfroCentric in partnership with Fedhealth is well positioned to deliver an integrated, holistic offering that aligns financial, wellness and medical services. In line with Sanlam's evolving health strategy, increased emphasis is being placed on a broader health platform approach, designed to enable participation across multiple schemes, employers and individuals through scalable distribution, rewards, digital capability, administration and managed care, rather than reliance on a single scheme outcome.

Performance

Central to this strategy is AfroCentric's partnership with Sanlam. Alignment across strategy, distribution and operating models has strengthened the Group's ability to deliver integrated healthcare and financial wellbeing solutions to employers, individuals and medical schemes.

Key developments during the year included:

- Selling AfroCentric Distribution Services to Sanlam to form Sanlam Health Distribution to focus on health distribution and expand access to Sanlam's broader distribution platform
- Expanding the health platform proposition through initiatives such as Health Rewards by Sanlam for Fedhealth members, promoting digital health engagement and incentivising healthy behaviour
- Delivering improved administration and managed care services to more than four million medical scheme beneficiaries through Medscheme's operating platform

Key performance indicator	Long-term target	2024 actual	2025 actual	Link to executive remuneration
EBIT margin growth	24% to 30%	7.5%	9%	✓ (Strategic)
Membership growth	5% to 8%	3%	2%	✓ (Strategic)

Strengthening our managed care leadership through clinically driven innovation



As a leading managed care organisation, clinical innovation remains a key differentiator for AfroCentric. The Group aims to transition traditional healthcare delivery models towards value-based care, with a stronger focus on healthcare outcomes, preventative care and improved member experiences.

Leveraging Medscheme's clinical expertise, Sanlam's risk management capabilities and strategic partnerships across the healthcare ecosystem, the Group continues to develop integrated healthcare solutions that place scheme members at the centre of care delivery.

Performance

Key developments during the year included:

- Advancing value-based care through coordinated care pathways, multidisciplinary provider engagement and outcomes-based contracting models
- Launching the Orthocare Spinal Programme in January 2025 for selected schemes, introducing a multidisciplinary rehabilitation pathway for chronic lower back pain aimed at improving functional recovery and reducing unnecessary surgery
- Strengthening collaboration between Medscheme's clinical teams and Scriptpharm's pharmaceutical risk management capabilities, enhancing the Group's integrated pharmacy benefit management model
- Establishing the groundwork for digital enablement of value-based care for diabetes (focusing on smart screening and risk stratification, intervention enrolment and medicine adherence)

Investing in data, digital and technology



We are committed to strengthening our leadership in healthcare by leveraging data-driven insights and adopting new technologies. By investing in data analytics, digital tools, and technology, we aim to improve operational efficiency and enhance the healthcare experience for our clients, empowering them with seamless digital solutions.

Performance

Digital and data capabilities are foundational to AfroCentric's ability to scale cost-effective healthcare delivery and support outcomes-based care models.

Key developments during the year included:

- Relocating the Group's data centre infrastructure to Sanlam's facilities in Cape Town, improving infrastructure resilience and system stability
- Advancing the Group's data modernisation programme, strengthening governance frameworks and enterprise data architecture
- Enhancing data capabilities to support improved decision-making, risk management and coordination across the healthcare journey
- Continuing to expand digital engagement platforms to enable more seamless interactions between members, healthcare providers and intermediaries

Key performance indicator	Long-term target	2024 actual	2025 actual	Link to executive remuneration
Hospital claims automation	75% to 85%	68%	76%	✓ (Strategic)

Evolving our culture and operating model to support the growth strategy



A key enabler of AfroCentric's strategy is the continued evolution of its culture and operating model. The Group seeks to foster innovation, collaboration and continuous learning while creating an inclusive environment that supports diversity and maximises employee potential.

This approach supports the development of a cohesive and integrated operating model that leverages the strengths of AfroCentric's diversified healthcare capabilities.

Performance

During the year, the Group implemented targeted organisational and structural changes to strengthen execution capability, governance and leadership accountability.

Key initiatives included:

- Realigning AfroCentric Technologies with core business activities and moving infrastructure to Sanlam Group Technology to improve stability and resilience
- Strengthening alignment with the Sanlam ecosystem to support a more integrated health platform across administration, managed care, distribution and technology
- Enhancing enterprise governance and risk oversight through leadership appointments, including the appointment of a Chief Risk Officer
- Continuing to embed an integrated operating model across administrative, clinical and pharmaceutical capabilities

Key performance indicator	Long-term target	2024 actual	2025 actual	Link to executive remuneration
Customer Effort Score (CES)	85% to 90%	83%	91%	✓ (Strategic)
Employee Net Promoter Score (eNPS)	30 to 50	21	9	✓ (Strategic)

Strategic outlook and interim focus

AfroCentric enters 2026 in a fundamentally different operating context. The outcome of the Bonitas RFP process represents a significant strategic and execution inflection point for the Group. These developments challenge some of the assumptions underpinning Vision 2030, particularly regarding the pace and sequencing of scale, investment and integration required to build a winning health offering. The strategy will therefore be recalibrated to address concentration risk, operate within a tighter capital envelope and reflect the practical constraints created by these developments.

Against this backdrop, management's priorities for 2026 are clear and execution-focused:

- **Cost reset:** Delivering a structural reduction and simplification of the cost-to-serve to align with the new revenue base (excluding Bonitas)
- **Protecting the core business:** We continue to serve 3.3 million lives while ensuring the enablement structure is fit for the revised revenue base
- **Reorganised operating model:** Establishing a lean, accountable structure designed to protect the existing client base
- **Portfolio simplification:** Focusing on core capabilities and removing organisational complexity that no longer provides value or impact in the new operating reality

Service excellence remains non-negotiable and will be protected through disciplined operational execution and continued investment in the systems, processes and capabilities that support client outcomes.

Improving the sustainability of the pharmaceutical cluster also remains a priority. Management is focused on stabilising profitability through:

- Active management of margin pressures by leveraging digital tools and robotic medication dispensing to lower transaction costs and offset the impact of declining private script volumes and right-sizing organisational structures
- Optimised working capital management through executing stricter inventory protocols, specifically reducing stock holdings within the private pharmacy and Activo businesses
- A sharper focus on the most appropriate product lines and contracts, simplifying its portfolio through the disposal of the Activo Group and concentrating resources on high-margin product lines and the most viable remaining contracts
- Continued scale and impact through Pharmacy Direct's participation in the public-sector CCMD programme. Pharmacy Direct continues to serve as a critical strategic partner to the Department of Health, expanding its reach to 2.4 million patients and delivering 1.4 million scripts monthly

Regulatory developments continue to shape the healthcare landscape and require agile strategic responses. These include ongoing discussions on the NHI Act and the CMS Low-Cost Benefit Options report, which recommends phasing out primary health insurance products.

Despite these challenges, the Group's core business remains sound and competitive. AfroCentric's diversified healthcare assets, strong partnerships and prudent capital management provide a solid foundation to respond to market shifts and pursue new opportunities.

The Group remains committed to strengthening an integrated healthcare value proposition that leverages its clinical, administrative and pharmaceutical capabilities in a more cohesive and scalable way, ensuring the continued delivery of sustainable value to shareholders.

Understanding our strategic trade-offs

Robust strategic decision-making requires careful consideration of which trade-offs to pursue and which to avoid, particularly when these decisions may negatively impact certain stakeholder groups while creating long-term value for others.

AfroCentric's strategic decisions are assessed through the lens of the six capitals, with an emphasis on their impact on long-term value creation. In 2025, the operating environment required the Group to make several difficult but necessary decisions to stabilise the business following contract losses and changing market conditions.

While some decisions reduced short-term profitability or required structural adjustments, they were made to preserve the Group's long-term sustainability and its ability to deliver affordable, high-quality healthcare.

Balancing short-term resilience with long-term strategic direction remains central to AfroCentric's decision-making.

1. Stabilising the business while restructuring the operating model

Analysing the trade-off

The outcome of the Bonitas procurement process represents a significant shift in AfroCentric's operating environment and requires a reassessment of the Group's operating model. While the process remains subject to ongoing regulatory and legal developments, the Group has prudently begun preparing for a potential change in the scale of services provided to Bonitas. Following the announcement of Bonitas' request for proposals in 2025, the Board undertook a series of deliberate and structured strategic assessments to evaluate the appropriateness of continuing the Group's long-standing relationship with the scheme. These considerations encompassed commercial sustainability, operational capability, and the broader social impact of AfroCentric's role within the healthcare ecosystem.

The Board supported participation of the Group in the Bonitas RFP process, recognising the strategic importance of Bonitas as a key client and the Group's capacity to deploy additional strategic and operational resources in support of the scheme. Concurrently, the Board conducted comprehensive scenario planning to assess the risks linked with an unsuccessful outcome, acknowledging that the potential loss of Bonitas would have material implications for the Group's operating model and cost structure. The Board appreciated that this would require timely and measured responses to safeguard long-term sustainability and the Group's ability to continue serving more than three million beneficiaries across its remaining scheme clients.

Central capital trade-off

Short-term impact on **human capital** through organisational restructuring



Long-term protection of **financial, intellectual and social and relationship capital** through a sustainable operating model



2. Simplifying the portfolio to strengthen strategic focus

Analysing the trade-off

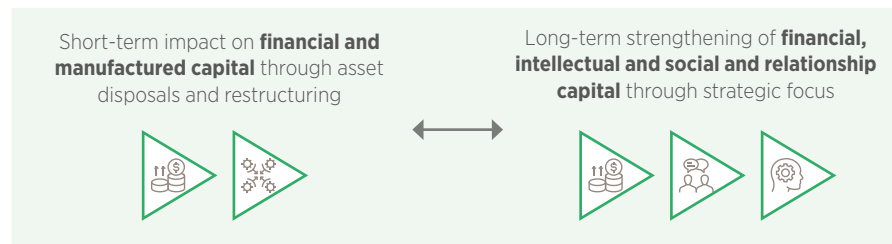
The Group took decisive steps to simplify its portfolio and address underperformance in the pharmaceutical cluster.

Declining private script volumes, margin pressure in certain pharmaceutical product lines and the loss of designated service provider contracts placed significant pressure on the business. In response, the Group initiated targeted restructuring actions, including cost right-sizing, improved inventory management and a sharper focus on profitable product lines.

The disposal of the Activo Group forms part of this portfolio simplification strategy.

Although these decisions reduce short-term scale and revenue diversification, they enable AfroCentric to concentrate resources on its core health platform capabilities, including scheme administration, managed care and pharmacy benefit management. At the same time, the Group continues to deliver meaningful healthcare impact through Pharmacy Direct's participation in the public-sector CCMD programme, which expanded significantly during the year.

Central capital trade-off



3. Investing in integrated health capabilities despite financial pressure

Analysing the trade-off

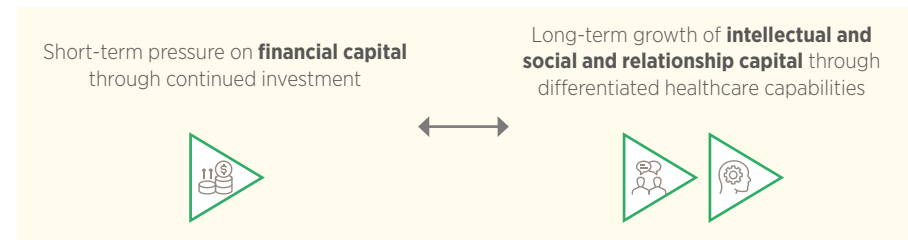
Even in a challenging financial year, AfroCentric continued investing in capabilities that differentiate its health platform.

One example is the integration of Scriptpharm into the Medscheme environment, strengthening the Group's integrated pharmacy benefit management proposition. This integration allows the Group to deliver coordinated pharmaceutical risk management and managed care services to scheme clients.

Similarly, the Group continued investing in clinically led innovation and value-based care models designed to improve health outcomes while controlling healthcare costs.

While these initiatives require investment in clinical, data, and technology capabilities, they are essential for maintaining AfroCentric's competitive position and improving healthcare affordability for scheme members.

Central capital trade-off



Our performance



Economic performance

CFO's review

The year under review unfolded against a challenging healthcare market backdrop, both locally and abroad. The South African healthcare market continues to operate under sustained pressure, characterised by elevated medical inflation, constrained scheme affordability, regulatory complexity, heightened public scrutiny on cost efficiency across the healthcare value chain, and modest overall membership growth.

Thato Moloele
Group CFO



These dynamics have continued to place pressure on medical schemes, administrators, managed care providers, and distribution partners alike, with knock-on effects on revenue growth, pricing flexibility, and margin sustainability across the sector.

Against this backdrop, the Group delivered resilient operational performance across its core South African and international businesses, maintaining high service delivery standards despite operating in a constrained and highly competitive healthcare environment. Management's focus during the year was firmly anchored on delivering value through our core business components, unlocking efficiencies for our scheme partners through our specialist managed care interventions, preserving balance sheet resilience, protecting liquidity, and proactively reshaping the Group's portfolio to simplify our operational structure and refocus on higher margin specialist service offerings.

While reported financial performance was impacted by revenue contraction and impairment charges, these outcomes must be understood within the context of market-driven changes, most notably the regression of Bonitas-related income, impacting current and expected earnings across the services, private courier, wholesale, and pharmaceutical marketing and distribution lines of our business. Accordingly, total impairment charges of R1.37 billion were recognised during the 2025 financial year, spread across our continuing and discontinued operations.

Notwithstanding the accounting loss reported for the year, which was largely driven by non-cash impairment charges, the Group delivered R679 million in operational earnings and R914 million in operating cash flows. These results underscore the underlying profitability, economic performance and cash-generative strength of the core businesses, which were supported by resilient services performance and exceptional outcomes in capitation risk management and the public courier operations.

Medical scheme administration, risk management and technology cluster

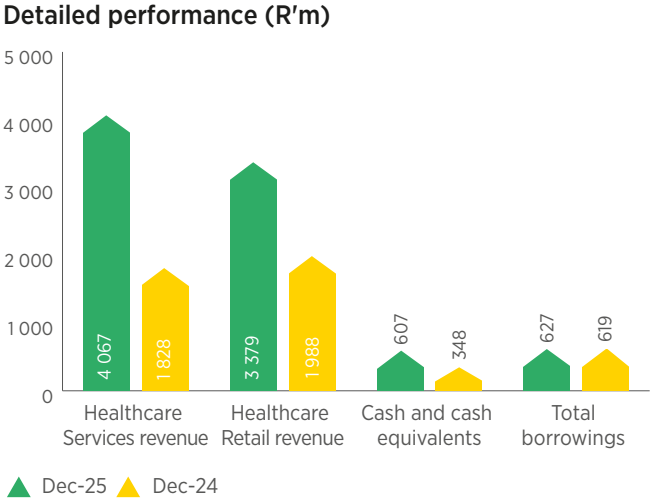
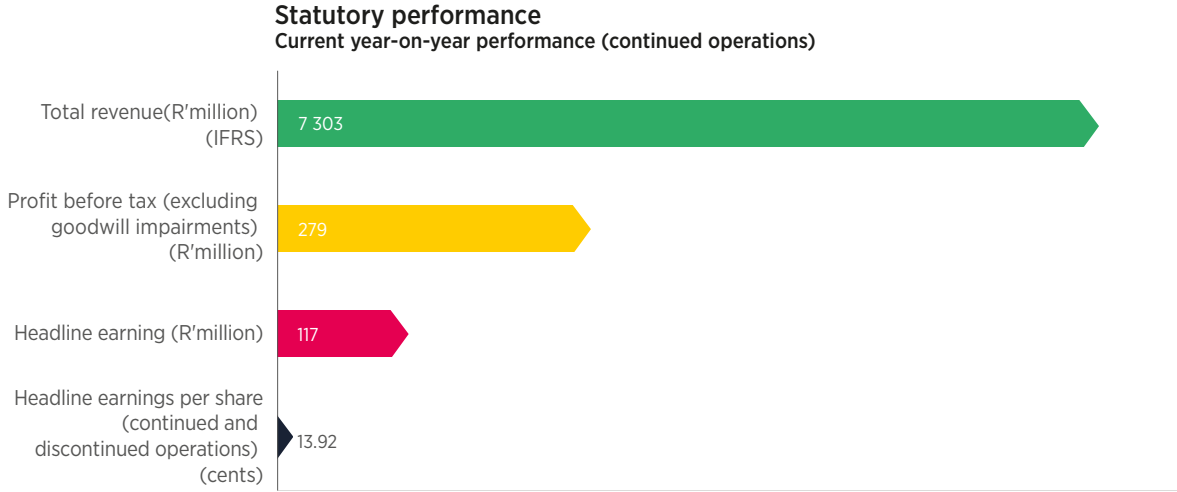
The medical scheme administration, risk management and technology cluster delivered resilient performance and contributed materially to the Group's financial performance. This contribution was delivered through disciplined execution across administration, managed care and capitation risk management activities both locally and abroad.

Financial outcomes in the South African market were primarily driven by contract losses linked to Bonitas, related to the administration of the lower-income BonCap option, and by marketing revenue. These events were partially offset by muted membership growth, averaging 2% locally, driven primarily by closed medical scheme membership growth. This continues to accelerate ahead of local open scheme growth, while the Group's Namibia operations delivered average membership growth of approximately 2% for the year.

In combination, these dynamics resulted in total lives under management increasing to 4.09 million, reinforcing the scale, relevance and defensive characteristics of the Group's core services platform.

The Group's Africa operations continued to make a meaningful and increasingly important contribution to the cluster. These businesses delivered stable performance in line with prior-year expectations, generating R261 million in revenue for the year, equivalent to approximately 6.5% of the cluster's revenue. The growing contribution of these markets relative to total Group revenue, together with disciplined cost management across South Africa and the rest of Southern Africa markets, enhanced the quality of earnings within the cluster and supported an improvement in its operating profit margin to 14.2%, strengthening AfroCentric's overall earnings profile.

A key strategic differentiator within the cluster remains the Group's capitation risk management capabilities, particularly across specialist healthcare risk management and oncology (reported under the pharmaceutical cluster) and dental lines, which delivered strong profitability during the year. These outcomes reflect robust underwriting discipline, effective claims management processes, and mature clinical risk governance frameworks, all of which remain central to the Group's value proposition to schemes operating in an increasingly constrained funding environment.



The dental capitation business, DENIS, delivered a particularly improved financial performance within the cluster, increasing its operational earnings by 26.2% for the year, supported by favourable utilisation trends and a tightly controlled claims experience. As a specialist line of business, dental capitation provides the Group with a clear and defensible strategic competitive advantage, requiring deep actuarial, clinical and operational expertise that is not easily replicated. This capability positions the Group as a preferred partner to medical schemes seeking predictable cost outcomes, sustainable benefit design and improved member experience, while generating attractive risk-adjusted returns.

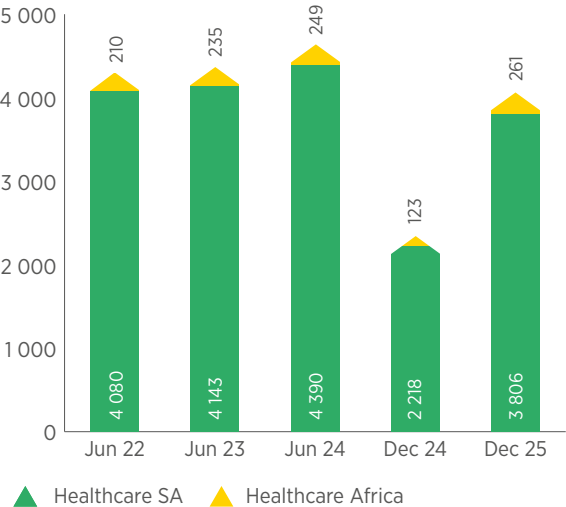
The year marked an important milestone in the evolution of the Group's managed care model, with the successful rollout of the spinal care risk transfer pilot within Fedhealth and Medshield, which performed ahead of expectations, achieving break-even economics during its initial pilot phase. This performance demonstrated the commercial viability of applying capitation-based risk management principles to high-cost musculoskeletal cases.

The success of this pilot, together with the improved performance of the Group's dental and oncology capitation businesses, provides tangible validation of the Group's strategic intent to transition toward a value-based care model, where specialist clinical capabilities, proactive cost management and aligned incentives support sustainable financial outcomes for our scheme clients, our members and shareholders. These developments reinforce management's confidence in progressively evolving the managed care offering away from traditional utilisation-based approaches toward scalable, data-enabled value-based care solutions across additional chronic and specialist conditions.

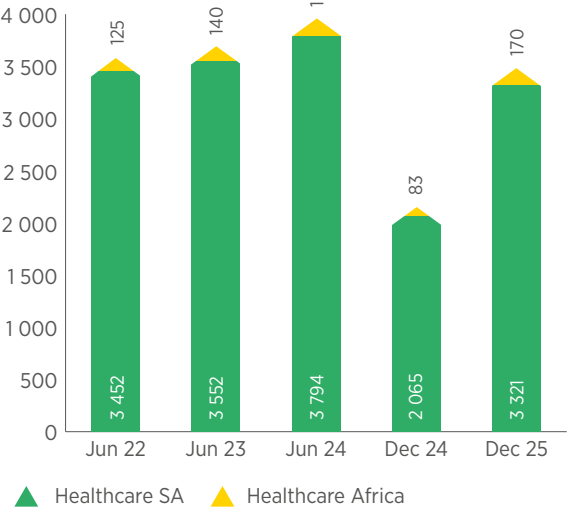
A key strategic milestone achieved during the year was the successful completion of the sale of AfroCentric Distribution Services, together with its wholly owned subsidiaries Tendahealth, AfroCentric Financial Services (**the ADS Group**) and Wellworx, to Sanlam Life in July 2025 for R13 million, net the indemnity claim paid to Sanlam Life. This transaction formed part of the Group's broader strategy to sharpen focus on core healthcare servicing capabilities, while leveraging partnerships to drive distribution scale more efficiently and sustainably.

The disposal has allowed AfroCentric to embed health sales and distribution within a well-capitalised, scalable ecosystem, enabling a more integrated healthcare platform for medical schemes, gap cover products, primary healthcare insurance and wellness solutions. Importantly, the transaction lowered operational risk and complexity, allowing AfroCentric to generate liquidity from the ADS Group while maintaining access to enhanced critical distribution capabilities.

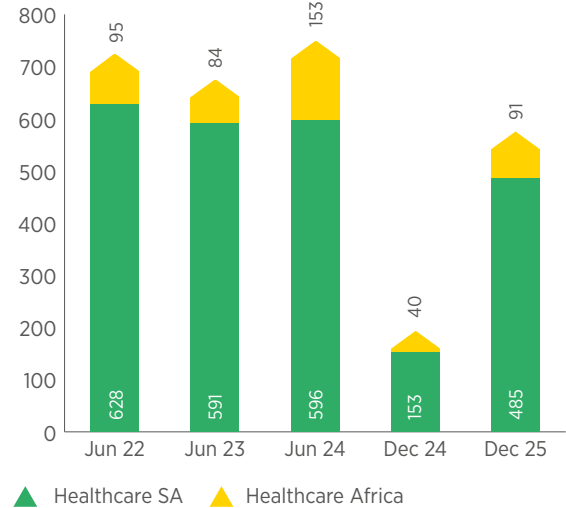
Revenue (R'm)



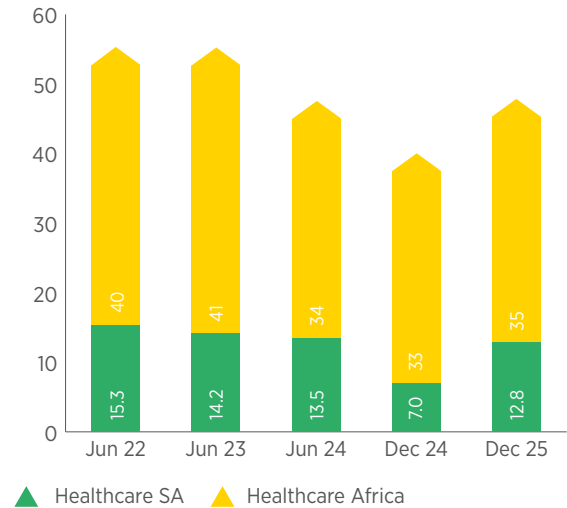
Operating costs (R'm)



Operating profit (R'm)



Operating margin (%)



Pharmaceutical cluster

This cluster comprises three linked but separate businesses, namely:

- The courier business – Pharmacy Direct, which delivers medicines to medical scheme members and the public through a partnership with the Department of Health
- The pharmaceutical business – Activo, a medicine manufacturer and distributor selling directly into the open pharmacy market
- The managed care risk management business – Scriptpharm provides risk management and pharmaceutical value chain optimisation services to schemes through an integrated patient-focused approach. These services are delivered through a risk-based capitation model

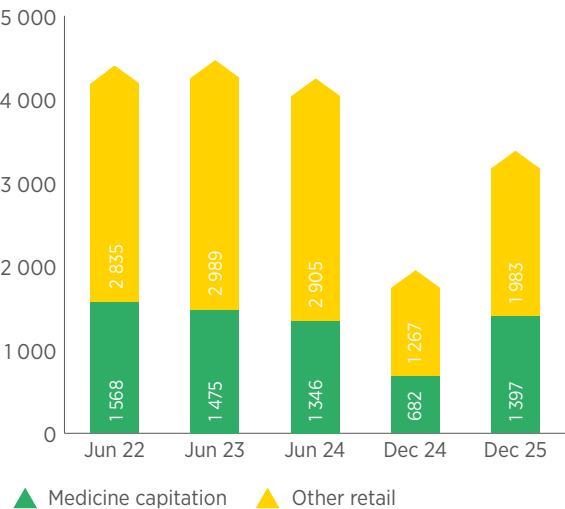
Performance within the pharmaceutical cluster during the year was materially impacted by the termination of three Bonitas designated service provider arrangements linked to the BonCap, Primary and Primary Select options, which took effect from February 2025.

These terminations had a direct and adverse effect on revenue generation and trading volumes across Pharmacy Direct Private, Curasana Wholesale and Activo, and represent the primary driver of the underperformance reported within this cluster for the year.

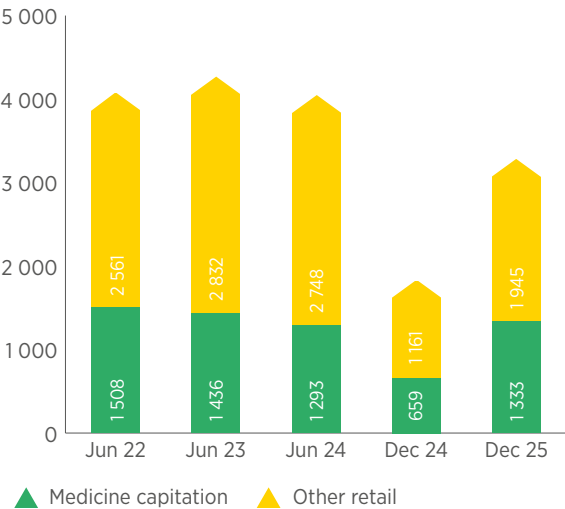
The loss of designated service provider status for these three options resulted in a sharp contraction in private-market script volumes, with average monthly scripts declining to 105 560, down from 157 759 in the prior period. This represents a 33% reduction in private-market volumes, reflecting both the immediate impact of the termination and the channel's structural sensitivity to scheme-mandated distribution arrangements. In addition, the designated service provider terminations adversely affected antiretroviral product category volumes (down 40.5% year on year), further weighing on trading performance and profitability margins across the wholesale and marketing components of the pharmaceutical cluster.

While private-market activity contracted materially, the year was also characterised by a record-breaking performance from Pharmacy Direct's public sector courier business, which continues to demonstrate undeniable scale, relevance, and reach in advancing South Africa's national healthcare objectives. The public courier business plays a critical role in the national healthcare ecosystem by supporting the Department of Health in providing access to essential chronic medication for over 2.4 million patients across several provinces, primarily through the CCMDD programme.

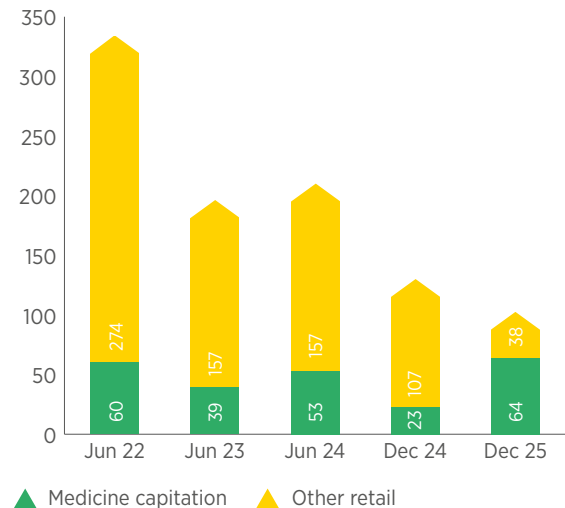
Revenue (R'm)



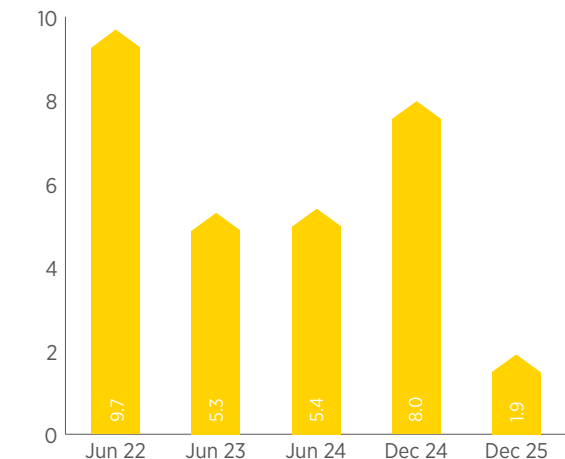
Operating costs (R'm)



Operating profit (R'm)



Operating margin (excluding medicine capitation) (%)



During the year, the public courier business increased average monthly script deliveries to 1.417 million, up from 1.329 million in the prior period. This represents a 6.6% increase in monthly delivery volumes, marking the strongest operational performance the business has achieved to date. The growth reflects the continued expansion of the public-sector programme, the operational reliability delivered by Pharmacy Direct, and the Group's ability to execute at scale in a highly regulated, service-driven environment.

The shift in business mix toward a higher proportion of public-sector volumes, while positive from a scale and access perspective, has had a dilutive effect on overall courier trading margins (down from 6.8% in 2024 to 2.2% in 2025). Notwithstanding this margin impact, the public courier operation delivered a strong absolute operating profit contribution, supported by operational efficiencies and improved organic market penetration, reinforcing its role as a stable, strategically important component of the pharmaceutical cluster and contributing positively to Group earnings.

Similar to the performance of our dental capitation business (reported in the medical scheme administration, risk management and technology cluster section), Scriptpharm's managed care offerings, specifically in oncology lines of expertise, delivered a 29% improvement in profitability for the year, driven by disciplined risk selection, effective claims adjudication and the successful execution of profit-sharing arrangements with scheme clients.

These results underscore the scalability and durability of the Group's specialist risk management platforms and highlight their role as long-term value drivers within the AfroCentric portfolio.

In parallel with the operational refresh underway, the Group has entered into a sale and purchase agreement relating to the potential disposal of the Activo Group, subject to the fulfilment of customary external approvals. This transaction represents a deliberate step to divest from pharmaceutical manufacturing and distribution activities, release capital tied up in non-core, working capital-intensive operations, and allow the Group to sharpen its focus on capital-light, higher-return healthcare activities.

As a result, the Activo Group has been classified as a discontinued operation, and its assets and liabilities have been reclassified as Held for Sale, in accordance with our applicable financial reporting standards. This classification appropriately reflects the Group's strategic intent to realise short- to medium-term value from assets that are no longer aligned with the Group's long-term operating model.

In aggregate, the pharmaceutical cluster's performance reflects a year of transition, marked by structural revenue losses in private-market channels alongside exceptional execution and growth in public-sector distribution, as well as healthcare risk management and the strategic decision to exit the Activo Group. As a management team, we intend to continue actively reshaping the retail portfolio to align with sustainable margin profiles, reduce earnings volatility, and ensure strategic relevance within the evolving healthcare landscape.

Balance sheet, cash flow and capital discipline

The balance sheet for the year reflects the recognition of material impairment charges, primarily relating to goodwill and intangible assets within Medscheme, Pharmacy Direct and Activo Group. These impairments were driven by a reassessment of AfroCentric's future earnings expectations linked to Bonitas following its decision to award several contracts to other service providers.

The recognition of these impairments represents a prudent, non-cash reset of asset values in line with AfroCentric's current operating realities. Importantly, the impairments do not impact the Group's liquidity and were undertaken to ensure that the balance sheet appropriately reflects sustainable earnings assumptions going forward. As a result, the Group enters the next financial year with a more conservative capital base, better aligned to its revised strategic focus and medium-term outlook.

The Group's cash flow management remained efficient throughout the year, supported by improved working capital discipline and sound operational execution against considerable headwinds. Key contributors included:

- Stock procurement interventions, aligned to reduced private-market volumes
- Improved profitability within core service and capitation businesses
- Containment of discretionary expenditure
- Continued focus on operational efficiency and cost control

As a result, liquidity and solvency metrics remained sound, providing the Group with the flexibility required to navigate the post-Bonitas transition period.

Capital preservation and outlook

In light of the challenging operating environment, recent and forthcoming structural changes, the Board has taken a deliberate decision to preserve short to medium-term capital. This approach is intended to prudently manage the balance sheet, protect liquidity, and provide the Group with the capacity to execute a cost and organisational reset during 2026, while maintaining service delivery and stakeholder confidence.

Management believes that this disciplined approach to capital allocation, combined with the strategic actions already taken, positions the Group to emerge from the Bonitas transition period leaner, more focused and better aligned to sustainable long-term value creation.

Appreciation

Management extends its sincere gratitude to our people for their continued commitment, resilience and collaboration throughout a challenging year. Your unwavering dedication, professionalism and shared belief in our purpose have been instrumental in enabling the Group to navigate significant market headwinds while continuing to deliver meaningful value. We look forward to building on this momentum as we position the Group for sustainable, long-term growth.



Thato Moloele
Group CFO

Summary annual financial results

Results at a glance

for the year ended 31 December 2025

		Year ended 31 December 2025	Excluding impair- ments 2025	Six months ended 31 December 2024	Excluding impair- ments 2024	Year ended 30 June 2024	Excluding impair- ments 2024	Year ended 30 June		% change			
Key features								2023	2022	Dec25/ Dec24	Dec24/ Jun24	Jun24/ Jun23	Jun23/ Jun22
Healthcare service revenue	(R'000)	4 067 007		1 828 230		4 639 131		4 378 300	4 290 086	122.5%	(60.6%)	6.0%	2.1%
Healthcare service operating profit	(R'000)	576 865		279 862		679 697		686 571	787 983	106.1%	(58.8%)	(1.0%)	(12.9%)
Healthcare retail revenue	(R'000)	3 379 833		1 988 267		4 251 575		4 464 341	4 403 131	70.0%	(53.2%)	(4.8%)	1.4%
Healthcare retail operating profit	(R'000)	101 918		92 530		210 646		196 323	333 317	10.1%	(56.1%)	7.3%	(41.1%)
Headline earnings	(R'000)	117 054		31 930		334 808		217 258	315 667	266.6%	(90.5%)	54.1%	(31.2%)
Normalised headline earnings	(R'000)	107 571		22 447		348 655		193 494	324 956	379.2%	(93.6%)	80.2%	(40.5%)
Cash generated from operations	(R'000)	914 799		343 158		1 027 896		796 143	1 101 850	166.6%	(66.6%)	29.1%	(27.7%)
Statutory earnings per shares													
– Basic	(cents)	(151.50)	11.87	(14.96)	(16.09)	6.67	34.47	30.70	54.32	(732.0%)	(146.7%)	(78.3%)	(43.5%)
Statutory headline earnings per shares													
– Basic	(cents)	13.92	7.74	3.8	(20.23)	40.32	40.32	36.34	54.81	(158.0%)	(150.2%)	11.0%	(33.7%)
Dividends paid per ordinary share	(cents)	0		6		11		0	34.00				
Share price as at 31 December	(rands)	1.33		1.88		2.88		4.15	4.39				
Return on assets		(30.38%)	1.72%	(6.55%)	(2.21%)	1.20%	5.24%	5.47%	8.83%				
Return on assets (excl. Goodwill)		(30.90%)	2.15%	(8.34%)	(2.94%)	1.59%	7.24%	7.60%	12.38%				
Return on equity		(64.81%)	2.89%	(10.32%)	(3.41%)	1.91%	8.14%	8.69%	14.02%				
Current ratio		1.21		1.58		1.52		1.27	1.30				
Cash generated from operating activities	(R'000)	684 903		249 258		695 368		381 694	527 018				
Capital expenditure	(R'000)	223 407		295 142		262 152		257 103	295 386				
Number of lives managed	('000)	4 081		4 075		4 010		3 871	3 790				
Group membership		1 682 372		1 660 350		1 642 673		1 653 477	1 652 035				
Open schemes		454 600		486 192		486 262		481 127	492 283				
Closed and Corporate schemes		281 546		246 299		249 924		251 516	259 103				
GEMS		898 755		880 565		859 666		818 953	784 431				
Namibia		47 471		47 294		46 821		101 881	116 218				
Number of scripts		18 268 514		9 346 556		17 097 640		14 847 876	13 911 074				
NDoH		17 001 798		8 415 348		15 268 885		13 045 738	12 196 969				
Pharmacy Direct		1 266 716		931 208		1 828 755		1 802 138	1 714 105				

Summarised consolidated statement of financial position

	Audited Year ended 31 December 2025 R'000	Audited six months ended 31 December 2024 R'000	Audited year ended 30 June 2024 R'000	Restated Audited year ended 30 June 2023 R'000	Restated Audited year ended 30 June 2022 R'000
ASSETS					
Non-current assets	2 085 668	3 643 099	3 790 706	4 141 570	4 115 232
Property and equipment	315 691	362 597	290 230	340 819	381 057
Right of use Assets	89 884	140 721	142 572	191 065	147 964
Land and buildings	339 493	298 677	301 422	307 052	316 561
Investment property	11 670	12 500	12 500	10 731	7 631
Intangible assets	1 047 635	2 541 437	2 804 199	3 045 541	3 076 336
Other financial assets	94	20 209	27 719	39 928	19 117
Investment in associates and joint ventures	12 922	6 259	8 732	49 148	33 340
Insurance contract assets	83 344	71 250	67 644	69 978	56 154
Deferred tax assets	119 708	121 127	132 015	82 881	77 072
Deferred payment asset	3 890	3 778	3 673	4 427	-
Receivables – Sanlam RSP and PDSP	61 337	64 544	-	-	-
Current assets	1 528 763	1 519 428	1 618 849	1 418 660	1 321 909
Trade and other receivables	521 082	602 303	786 276	740 695	724 321
Inventory	107 305	440 837	449 079	444 562	431 764
Current tax assets	273 544	128 492	53 235	43 640	27 235
Other financial assets	19 473	-	-	-	-
Cash, managed funds and deposits	607 359	347 796	330 259	189 763	138 589
Assets Held for Sale	566 023	-	-	-	-
Total assets	4 180 454	5 162 527	5 409 555	5 560 230	5 437 141
EQUITY AND LIABILITIES					
Capital and reserves	1 902 784	3 245 743	3 369 996	3 443 302	2 438 244
Issued ordinary share capital	8 211	8 184	21 324	21 294	18 909
Share premium	2 553 665	2 550 551	2 537 411	2 525 687	1 094 876
Share-based payment reserve	5 948	17 646	24 468	34 115	30 465
Treasury shares	(1 162)	(2 240)	(1 162)	(1 162)	(2 324)

	Audited Year ended 31 December 2025 R'000	Audited six months ended 31 December 2024 R'000	Audited year ended 30 June 2024 R'000	Restated Audited year ended 30 June 2023 R'000	Restated Audited year ended 30 June 2022 R'000
Foreign currency translation reserve	(6 793)	3 905	(5 650)	2 201	(5 056)
Capital contribution by Non controlling interest	-	-	-	-	55 874
Cash flow hedge reserve	-	-	-	1 530	-
Distributable reserve	(657 085)	667 697	793 605	859 637	1 245 500
Non-controlling interest	40 225	33 350	31 189	55 950	987 772
Total equity	1 943 009	3 279 093	3 401 185	3 499 252	3 426 016
Non-current liabilities	880 282	923 655	942 911	946 614	995 923
Deferred tax liabilities	245 669	263 581	259 628	223 131	242 367
Non-current borrowings	574 119	556 722	569 853	528 005	531 082
Non-current provisions	-	-	-	16 970	16 970
Post-employment medical obligations	1 658	1 631	1 680	1 825	1 916
Lease liabilities	58 836	101 721	111 750	176 683	127 790
Contingent consideration	-	-	-	-	75 798
Current liabilities	1 265 387	959 779	1 065 459	1 114 364	1 015 202
Provisions	15 053	13 059	16 792	16 695	15 216
Borrowings	53 126	62 373	58 553	120 000	120 000
Lease liabilities	57 611	69 985	65 336	67 644	68 610
Trade and other payables	712 248	628 211	733 057	610 588	632 604
Current tax liabilities	230 207	72 748	9 374	9 031	19 161
Employment benefit provisions	194 605	111 400	182 347	106 276	143 122
Deferred payment liability Sanlam Performance	-	-	-	-	14 139
Deferred Shares – IFRS 2 liability	2 537	2 003	-	-	-
Bank overdraft	-	-	-	104 007	2 350
Contingent consideration	-	-	-	80 123	-
Liabilities Held for Sale	91 776	-	-	-	-
Total liabilities	2 237 445	1 883 434	2 008 370	2 060 978	2 011 125
Total equity and liabilities	4 180 454	5 162 527	5 409 555	5 560 230	5 437 141

Summarised consolidated statement of comprehensive income

	Audited year ended 31 December 2025 R'000	Re-presented* Audited six months ended 31 December 2024 R'000	Audited year ended 30 June 2024 R'000	Restated Audited year ended 30 June 2023 R'000	Audited year ended 30 June 2022 R'000		% change					
						Dec25/ Dec24	Dec24/ Jun24	Jun24/ Jun23	Jun23/ Jun22	Jun22/ Jun21	CAGR	
Healthcare service revenue	4 067 007	1 828 230	4 639 131	4 378 300	4 290 086	122.5%	(60.6%)	6.0%	2.1%	11.9%	7.2%	
Healthcare service operating costs	(3 571 133)	(1 588 292)	(4 038 306)	(3 766 970)	(3 577 908)							
IFRS 16: Lease reversals	80 991	39 924	78 872	75 241	75 805							
Healthcare service operating profit	576 865	279 862	679 697	686 571	787 983	106.1%	(58.8%)	(1.0%)	(12.9%)	(3.0%)	1.5%	
Healthcare retail revenue	3 379 833	1 988 267	4 251 575	4 464 341	4 403 131	70.0%	(53.2%)	(4.8%)	1.4%	3.8%	6.3%	
Healthcare retail cost of sales	(2 171 506)	(1 175 573)	(3 370 496)	(3 590 850)	(3 592 571)							
Healthcare retail operating costs	(1 106 409)	(720 164)	(670 433)	(677 168)	(477 243)							
Healthcare retail operating profit	101 918	92 530	210 646	196 323	333 317	10.1%	(56.1%)	7.3%	(41.1%)	8.7%	(3.7%)	
Total healthcare operating profit	678 783	372 392	890 343	882 894	1 121 300	82.3%	(58.2%)	0.8%	(21.3%)	0.2%	0.2%	
Other income	50	236	5 186	2 570	563							
(Loss)/Profit on sale of investments	(830)	-	(5)	-	-							
Profit/(loss) on disposal of subsidiary	5 101	-	-	(198)	-							
Compensation for impairment of property and equipment	-	64 402	-	-	-							
Loss on disposal of intangible assets	-	-	(6 060)	(5 415)	(635)							
Fair value (losses)/gains	(89)	380	1 769	3 669	6 369							
Impairment of assets and loans	(824 914)	(2 958)	(274 265)	(27 115)	(3 203)							
Net finance and investment income	(13 059)	(12 709)	(58 227)	(68 252)	(60 834)							
- Finance and investment income	62 445	25 123	38 008	25 941	14 996							
- Finance costs : Lease liabilities	(13 273)	(7 897)	(19 190)	(16 964)	(18 781)							
- Finance cost	(62 231)	(29 935)	(77 045)	(77 229)	(57 049)							
Share-based payment income/(expense)	2 564	(6 487)	(3 133)	(17 837)	(13 848)							
Share of associate profits/(losses)	6 663	(2 199)	(9 045)	14 051	6 991							
(Loss)/profit before depreciation and amortisation	(145 731)	413 057	546 563	784 565	1 056 703	(135.28%)	(24.43%)	(30.3%)	(25.8%)	1.7%	(8.1%)	
Right of use assets depreciation	(60 090)	(30 377)	(52 747)	(65 380)	(61 606)							
Depreciation	(123 999)	(55 797)	(94 142)	(82 976)	(87 138)							
Amortisation of intangible assets	(202 081)	(101 786)	(192 690)	(208 823)	(231 269)							
(Loss)/profit before income taxation from continuing operations	(531 901)	225 097	206 984	427 386	676 690	(336.30%)	8.75%	(51.6%)	(36.8%)	(3.3%)	(17.4%)	

* The summarised consolidated statement of comprehensive income has been re-presented to separately disclose discontinued operations that were previously included in continuing operations for comparative periods presented.

Cluster performance

During the year, the Group continued to reposition around an integrated health offering in collaboration with Sanlam, while navigating a demanding client contract cycle and heightened industry scrutiny. Key focus areas included strengthening the core services platform, simplifying parts of the portfolio, and accelerating digital enablement to protect service quality and improve efficiency at scale.

The operating environment remained complex, characterised by rising healthcare funding pressures, tighter procurement cycles in medical scheme administration, and increased stakeholder attention on claims governance and fraud prevention. In response, the Group focused on protecting service excellence, strengthening integrated capabilities across administration and pharmaceutical services, and enhancing operational resilience.

Executing our strategy through cluster initiatives

Our strategy is structured around four key objectives: building a winning health offering; driving clinical innovation; investing in data, digital and technology; and evolving our culture and operating model. Each of our business clusters actively contributes to these objectives, collectively advancing our strategic ambition.

Through closer collaboration across administrative, clinical and pharmaceutical capabilities, the Group continues to strengthen its integrated healthcare platform while enhancing affordability, improving outcomes and expanding access across the healthcare value chain.

Building a winning health offering

We are committed to transforming healthcare access and affordability. Our approach is anchored in strategic partnerships, advanced healthcare models and innovative service delivery to create sustainable, value-driven healthcare solutions. By prioritising quality outcomes, cost efficiency and seamless service experiences, we continue to enhance member wellbeing while supporting long-term sustainability for healthcare funders and employers.

Medical scheme administration, risk management, and technology cluster

Our partnership with Sanlam continued to gain momentum as the Group progressed its Health Platform strategy. During the year, Health Rewards by Sanlam was launched for Fedhealth members, supporting preventative health engagement and strengthening the integration between healthcare services and financial wellbeing solutions.

Operational integration across the healthcare value chain also advanced. The Group continued aligning administrative, clinical and pharmaceutical capabilities, including further integration between Medscheme and Scriptpharm to strengthen the Group's pharmacy benefit management capability. This integration enables more coordinated benefit design, improved cost management and a more seamless experience for members and healthcare providers.

Distribution alignment also progressed during the year, with AfroCentric Distribution Services rebranded and integrated into Sanlam Health Distribution within Sanlam Corporate Distribution. This supports a more coordinated go-to-market approach for health solutions across the Sanlam ecosystem.

Service excellence remained a key differentiator. Customer effort score (customer satisfaction survey) trends improved, reflecting sustained service experience.

The cluster continued to manage a complex contract lifecycle environment. Several key contracts approached renewal during the year, requiring focused engagement with scheme clients and continued demonstration of service excellence. At the same time, Medscheme successfully secured new business through a competitive tender process.

For more information on contract management, please see our Chairman's and CEO's reviews on pages 6 and 54, respectively, as well as our Stakeholder matters in focus section on page 33.

Pharmaceutical cluster

The pharmaceutical cluster continues to participate across the medicine value chain to improve affordability, strengthen adherence and expand access to essential medicines.

Pharmacy Direct further strengthened its role in the public healthcare sector, supporting the CCMD programme and increasing medicine deliveries despite continued price compression across the sector. Improvements in operational processes and delivery coordination contributed to a strong recovery in public sector programme performance.

In the private market, operations were impacted by lower script volumes following the loss of designated service provider arrangements across certain Bonitas options. In response, the cluster implemented a cost rationalisation programme in April 2025 and strengthened working capital discipline, improving stock management and distribution efficiency.

Strategically, the cluster continues to drive affordability through increased generic utilisation and improved compliance management. Scriptpharm's compliance management model achieved over 90% generic substitution, and capitation-based contracting delivered material savings, including R69 million and R240 million on chronic and HIV medicines respectively for two client schemes, and R65 million in out-of-hospital oncology medicines and consumables for a large scheme. Through direct engagement with prescribers and patients, adherence support programmes continue to reduce avoidable hospital admissions and support improved long-term treatment outcomes.

Corporate solutions cluster

The corporate solutions cluster enables employers to improve employee health and productivity through integrated solutions combining preventative healthcare, mental health support and financial wellbeing.

Nurse-led onsite clinics, mobile nurse services and targeted digital wellness tools support early detection, reduce absenteeism and improve access to care within the workplace. By combining Fedhealth Elect and Savvy options with Sanlam Gap Cover and Sanlam Primary Care, the cluster provides cost-effective healthcare alternatives that enable unlimited primary care benefits outside hospital settings.

Automated gap claims processing, integrated with Medscheme-administered schemes, has improved claims turnaround times and enhanced the overall client experience.

The cluster also continued strengthening its alignment with the Sanlam Health platform, enabling the development of coordinated health and financial wellbeing solutions for employer groups. These integrated offerings support improved affordability while expanding access to preventative healthcare services.

Driving clinical innovation

AfroCentric continues to redefine healthcare by shifting from traditional, transactional models to a patient-centred, value-based approach to care. Our focus on clinical innovation ensures that interventions improve health outcomes while supporting affordability and sustainability.

Through collaborative contracting models, coordinated care pathways and proactive disease management, the Group is reshaping healthcare delivery to be more integrated, preventative and responsive to member needs.

Medical scheme administration, risk management, and technology cluster and pharmaceutical cluster

The Group continues to advance patient-centred, value-based models of care that reward improved health outcomes, coordinated care and responsible cost management. Clinical, managed care and pharmaceutical teams collaborate to align benefit design, provider networks and contracting models with evidence-based treatment pathways.

Several key initiatives progressed during the year.

The Orthocare Spinal Risk Transfer Model, launched on 1 January 2025, introduced a multidisciplinary biopsychosocial pathway for chronic lower back pain. The programme focuses on improving functional recovery and coping skills while reducing unnecessary spinal surgery through conservative, evidence-based care.

The Oncology Performance-Based Reimbursement Model continues to advance treatment pathways that balance clinical outcomes with responsible cost management.

Integrated Pharmacy Benefit Management remains a key priority. The alignment of Medscheme's clinical and operational capabilities with Scriptpharm's pharmaceutical risk management and network expertise strengthens the Group's integrated Pharmacy Benefit Management (PBM) solution, supporting value-based care and improved medicine management.

Digital transformation of disease management programmes also continued, using data and AI-enabled screening to support earlier intervention, particularly in conditions such as diabetes and obesity.

The Group also strengthened fraud, waste, abuse and error detection programmes, enhancing claims oversight and improving provider billing behaviour to support scheme sustainability.

Corporate solutions cluster

A healthy workforce is fundamental to organisational performance, and our approach to corporate health continues to evolve through clinically led, data-driven innovation.

Mental health support remains a key component of workforce wellbeing. During the year, the cluster expanded access to mental health professionals, digital wellness tools and preventative resources, supporting early identification and intervention while reducing reliance on traditional employee assistance programmes.

Financial wellbeing also plays a critical role in employee health. The launch of Credit Gateway in 2025 provides employees with structured support to manage debt, improve financial planning and strengthen long-term financial resilience.

Innovation at the point of care is further demonstrated through the mobile nurse service, which brings clinical services directly into the workplace. Upcoming enhancements, including an online booking platform and onsite InBody scanners, will further strengthen clinical assessment capabilities and improve operational efficiency.

Investing in data, digital, and technology

The healthcare industry continues to undergo rapid digital transformation. AfroCentric is investing in data analytics, automation and advanced decision technologies to improve service delivery, enhance member experiences and support operational efficiency at scale.

Medical scheme administration, risk management, and technology cluster

Core administration and decisioning

Core administration platforms continue to be modernised through the replacement of legacy claims and pre-authorisation systems with a modern decision engine and data architecture capable of real-time, rules-based decision-making at the point of service.

The platform processes approximately 10.2 million associated provider claim lines each month, with more than 99% processed automatically, demonstrating the scale and efficiency of the Group's administration capability. Of approximately 150 000 hospital and facility claims processed monthly, automation reached 76%, representing a 28% improvement on legacy systems and an 8% year-on-year increase, and exceeded 80% automation for two months following system enhancements.

These decision technologies also support pre-authorisation and case management processes, improving responsiveness and enabling better after-hours access to hospital care.

Intelligent automation and data ingestion

Automation continues to expand across operational processes. The Group now supports 87 business processes through robotic process automation (RPA) and AI-enabled agents, handling approximately 240 000 work items per month across 130 software bots, with a 95% completion rate.

Tools developed to convert unstructured inputs, including approximately 70 000 emails and 22 000 portal requests each month, many containing handwritten or scanned attachments, now achieve more than 90% conversion accuracy. This capability enables automation of complex workflows such as PMB email processing, mental health admissions, chronic medicine management and member onboarding, increasing overall end-to-end automation levels to approximately 89%.

Digital engagement

Digital engagement capabilities continue to expand to improve service accessibility for members, healthcare providers and intermediaries. The Customer Engagement Modernisation programme informed a contact centre modernisation market review, with implementation of a new AI-enabled engagement platform.

The provider portal, launched in July 2024, has gained strong adoption and is now used by more than 35 775 healthcare practices and 245 bureaus for claims review, benefits verification and remittance management.

Digital tools at the point of care also expanded during the year. QR code functionality was deployed across 516 healthcare practice locations, generating 3 251 scans that enabled members to access practice information, check benefits, nominate preferred providers and submit referrals digitally.

The broker portal was redesigned during the year and is scheduled for launch in 2026. The upgraded platform will introduce enhanced reporting capabilities, improved product mapping and streamlined onboarding, with future enhancements planned to enable real-time quotations, integrated underwriting and simplified broker servicing.

Data modernisation

Data modernisation initiatives also progressed during the year to strengthen the Group's digital foundation. Migration of procedure files to a centralised data management platform improved data accuracy and scalability, while the implementation of a single Enterprise Data Exchange platform now manages industry master and reference data across systems, including the decision engine.

This architecture establishes a single source of truth for key data sets, reducing duplication and operational errors while improving decision accuracy.

In addition, the Group relocated its data centres and AfroCentric Technologies adopted a new platform, strengthening system integration, resilience and readiness for future AI-driven capabilities.

Pharmaceutical cluster

Digital integration across the pharmaceutical environment continued to advance, improving coordination between medicine management, scheme administration and patient support programmes.

Pharmacy Direct Private remains fully integrated with Medscheme and Aid for AIDS, enabling real-time chronic medicine authorisations and benefit validation at the point of dispensing, which improves efficiency for pharmacists while reducing administrative friction for members.

Integration with the SyNCH platform for the CCMDD programme has further streamlined prescription processing and reduced administrative burden for healthcare providers. These capabilities also enable improved monitoring and reporting across the programme.

Scriptpharm continues strengthening its clinical data capabilities through an oncology data model that supports treatment planning and cost management. Rule-based approvals for supportive oncology medicines ensure appropriate treatment protocols while supporting responsible cost management.

Corporate solutions cluster

The corporate solutions cluster continued expanding its digital wellness platform to improve accessibility and engagement for employer clients and employees.

AI-enabled tools, automated workflows and WhatsApp-based service channels were introduced to improve response times and support more personalised interactions with employees accessing wellness services.

Employers also benefit from anonymised data analytics that provide insights into programme utilisation and outcomes, enabling organisations to assess programme performance and target interventions more effectively.

To increase adoption of digital wellness solutions, the cluster implemented focused education campaigns, gamification initiatives and incentive-based engagement programmes, improving awareness and participation across employer groups.

Evolving our culture and operating model

AfroCentric continues to embed a more integrated and agile operating model that aligns clinical, administrative and pharmaceutical capabilities with client needs and regulatory expectations. This transformation is supported by strengthened leadership accountability, governance frameworks and operational discipline across the organisation.

Medical scheme administration, risk management, and technology cluster

The cluster continued transitioning towards a capability-based delivery model anchored in core services. Workforce planning improvements and a more structured hiring process reduced recruitment lead times and improved talent quality.

To strengthen operational enablement, new capabilities focused on product and service implementation were established, while existing functions were reorganised to strengthen innovation, business architecture and service governance.

The business enablement capability now incorporates Business Engineering and Solution Design, a Service Support Hub and a Functional Enablement capability. This structure is designed to support the organisation's transformation and ensure that the capabilities required for future healthcare delivery are embedded within the operating model.

Pharmaceutical cluster

The cluster continues to strengthen its operating model to improve integration across the Group, supporting the delivery of affordable medicines, efficient supply chains and consistent service standards. The integration of Scriptpharm has remained a central priority, enhancing cost efficiency and service performance. By aligning procurement and formulary strategies with AfroCentric's broader healthcare goals, the cluster supports scheme sustainability and improved access to medicines for members.

A formal performance management framework has been embedded to reinforce accountability and operational excellence. At Pharmacy Direct Private, real-time performance dashboards provide employees with clear visibility of their individual contributions, ensuring alignment with service targets and strengthening delivery standards.

Corporate solutions cluster

The Cluster streamlined entities with complementary offerings, improving efficiency and pricing consistency. Under the Sanlam Health brand, coordinated financial and healthcare solutions enable employers to access integrated offerings that support improved health outcomes while strengthening affordability for employees.

A strong performance culture is supported through structured training programmes, clearly defined service level agreements and independent client feedback mechanisms, ensuring that the cluster continues to respond effectively to employer and employee needs.

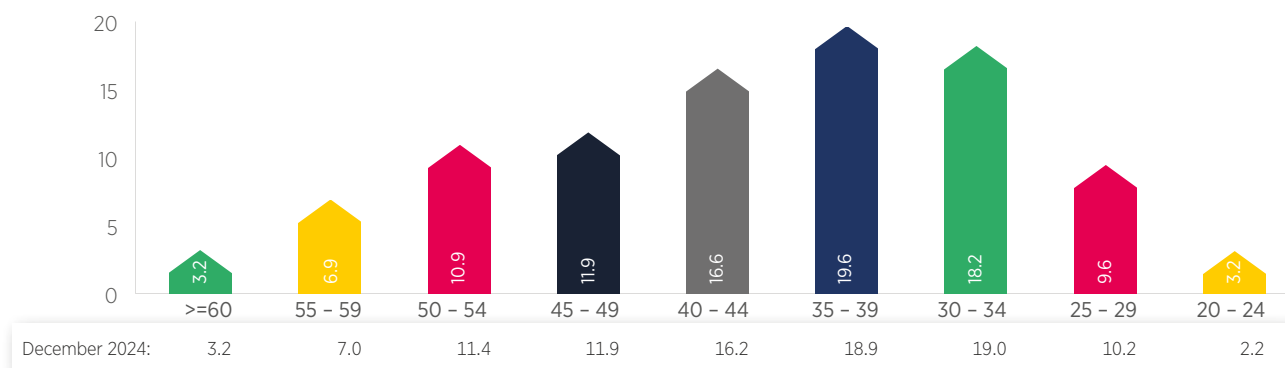
Social performance

Our people remain central to delivering sustainable healthcare solutions and upholding our social licence to operate. In 2025, we continued to strengthen our workforce profile, deepen transformation, and invest meaningfully in culture, wellbeing and capability. With a total South African headcount of 5 061 employees, a predominantly female workforce (69.5%), and sustained progress in recruiting previously disadvantaged groups, we have advanced our commitment to diversity and inclusive growth. At the same time, we embedded a refreshed values-led culture, expanded employee listening mechanisms, and enhanced integrated wellbeing and development initiatives to support performance, resilience and long-term organisational sustainability.

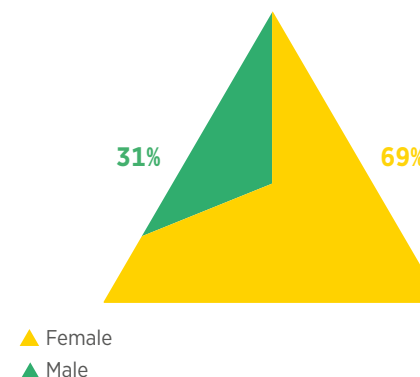
SA Head count totals	Dec 2025*							Dec 2024*						
	Male	Female	Black	White	Foreign	Total	%	Male	Female	Black	White	Foreign	Total	%
Total	1 419	3 515	4 391	628	42	5 061	100	1 545	3 497	4 336	673	33	5 042	100
Per employee category														
Permanent	1 419	3 324	4 087	615	41	4 743	94	1 501	3 406	4 216	659	32	4 907	97
Temporary	127	191	304	13	1	318	6	44	91	120	14	1	135	3
Total	1 419	3 515	4 391	628	42	5 061	100	1 545	3 497	4 336	673	33	5 042	100

* These are SA employees, not the total Group including Namibia and Mauritius.

Age range (%) – December 2025



Gender (%) – December 2025



Union members

In line with their constitutional rights, we uphold our employees' right to freedom of association and collective bargaining, the protection of employee representatives, and the right to be a trade union member.

	December 2025	December 2024
NEHAWU	1 822	1 964
Total	1 822	1 964

Staff appointments

In the past period, AfroCentric has seen an increase in top management from previously disadvantaged groups.

Occupation level	Dec 2025*							Dec 2024*						
	Male	Female	Black	White	Foreign	Total	%	Male	Female	Black	White	Foreign	Total	%
Top management	1	1	2	0	0	2	0.4	0	0	0	0	0	0	0
Senior management	0	3	1	2	0	3	0.6	1	2	3	0	0	3	1.3
Professionally qualified	15	27	37	5	0	42	8.3	13	13	21	5	0	26	11.3
Skilled	76	195	262	8	1	271	53.5	22	64	83	3	0	86	37.2
Semiskilled	89	100	174	14	1	189	37.3	32	83	113	2	0	115	49.8
Unskilled	0	0	0	0	0	0	0	1	0	1	0	0	1	0.4
Grand total	181	326	476	29	2	507	100	69	162	221	10	0	231	100
	35.7%	64.3%	93.9%	5.7%	0.4%	100%		29.9%	70.1%	95.7%	4.3%	0.0%	100%	

* These are SA employees, not the total Group including Namibia and Mauritius.

As part of our efforts to advance diversity and transformation, 93.9% of new appointments by December 2025 were from previously disadvantaged groups. Despite the ongoing war on talent, we attracted an additional 21 employees (81% at the professionally qualified level). This is crucial to our succession strategy to create a talent pool for the next level.

Employee hires and turnover

By gender

	Male	Female	Total*
Total number of new employees	181	326	507
New employee hire rate	35.7%	64.3%	100%
Total employee turnover	5.9%	11.3%	17.2%

* These are SA employees, not the total Group including Namibia and Mauritius.

By age

	Under 30	30 – 50	Over 50	Total*
Total number of new employees	305	197	5	507
New employee hire rate	60.2%	38.9%	0.9%	100%
Total employee turnover	5.4%	9.2%	2.6%	17.2%

By race

	African	Indian/ Asian	Coloured	White	Foreign	Total*
Total number of new employees	407	23	46	29	2	507
New employee hire rate	80.3%	4.5%	9.1%	5.7%	0.4%	100%
Total employee turnover	11.8%	0.9%	2.5%	1.9%	0.1%	17.2%

* These are SA employees, not the total Group including Namibia and Mauritius.

Our culture journey – refreshing and reconnecting to a values-driven culture

In 2025, AfroCentric reached a significant milestone by launching a structured Values Refresh Programme to cultivate a shared, future-ready, values-driven culture centred on Integrity, Care, Innovation, and Collaboration. The refresh was intentionally co-created to ensure authenticity, relevance, and ownership, with nearly 50% of AfroCitizens contributing to defining what the values mean and how they should look in action.

Engagement occurred through multiple channels and deep listening moments, including a digital values survey (capturing 17% of organisation-wide insights), 56 virtual workshops (totalling 84 hours of co-creation), and four senior leadership immersion sessions to secure leadership alignment and commitment. Insights from these engagements were consolidated into a practical Values Framework that describes the values “in action,” observable behaviours and cultural expectations aligned with our purpose.

The Values Refresh formed part of the broader 2025 Culture Refresh Programme. It was delivered through a phased approach in the second half of 2025, moving from introducing and launching the values to build awareness to landing the values with colleagues to encourage engagement, deepen understanding, embed meaning and ensure leadership alignment. This approach ensured that values were not treated as a once-off communication event, but as an evolving culture shift supported by leadership advocacy, enabling tools and measurement.

To translate the refreshed values into lived culture, AfroCentric embedded a phased programme approach with a range of practical interventions aligned with clear objectives and deliverables.

Phase I – Launch (June – July): The organisation introduced the refreshed values and built early awareness through a leadership roadshow kick-off (24 June), desktop lockscreen rollout (July), and a communication campaign featuring CEO messaging, Exco value videos, and an integration narrative aligned with the broader strategic direction. Supporting assets included infographics, digital collateral and invitations to participation activities.

Phase II – Land (August – December): Focus shifted to deepening engagement through immersion and co-creation workshops, alongside development and validation of the strategic Values Framework. This phase also provided an opportunity for our Senior Leadership team to participate in a Values Immersion Experience, a dedicated space for reflection, connection, and growth as they explored the refreshed values and AfroCentric Leadership Ethos, amplifying and strengthening the message that values-led leadership is not just important, it is essential. Refreshed values messaging remained a strong feature throughout the remainder of the year in our weekly Connexion newsletter and other Executive communication.

Employee listening strategy – Pulse survey

In October 2025, we conducted our fourth pulse survey, which highlighted significant progress in fostering a positive workplace culture.

Our pulse data

	Pulse I April 2024	Pulse II October 2024	Pulse III March 2025	Pulse IV October 2025
Response rate	77%	78%	83%	81%
Pulse score	81%	82%	81%	80%
Flight risk	11%	11%	13%	14%
eNPS	N/A	21	14	9

Employee wellbeing

Employees are our most valued asset, and we therefore prioritise integrated wellness programmes that promote wellbeing and foster a culture of care. Recognising that wellness drives productivity and organisational performance, we provide employees with the support they need to navigate challenges and reach their full potential. Our executive leaders visibly champion these initiatives, demonstrating strong organisational commitment and encouraging meaningful employee engagement.

Our organisation’s employee wellbeing strategy is underpinned by an integrated Five-Star Model comprising physical, emotional and mental, financial, social wellbeing, and organisational effectiveness. This model enables a coordinated, preventative approach to workforce wellbeing, supporting early intervention, sustained access to care, and organisational performance.

A key focus on prevention and early intervention through ongoing access to care is achieved through primary healthcare clinics, which provide continuous, year-round support, enabling employees to access assessment, treatment, and referral services without delay. 3 990 employees accessed clinic services in 2025, with health education, family planning, and respiratory and gastrointestinal conditions remaining the most common reasons for utilisation. This consistent uptake reflects sustained reliance on accessible primary care. The always-on clinic model was complemented by Wellness Week, during which 1 366 employees completed health risk assessments, and 833 participated in voluntary HIV testing. Medical scheme data continues to indicate diabetes, hypertension and hyperlipidaemia as the most prevalent chronic conditions, reinforcing the importance of ongoing screening and early detection initiatives.

As a healthcare organisation, it is crucial that all our employees enjoy access to quality healthcare through the three medical scheme options we have made available. Beyond medical aid offerings, primary health insurance continues to gain traction among employees earning below a certain threshold, with enrolment steadily increasing from 595 in 2024 to 716 in 2025.

The Employee Assistance Programme (**EAP**) remained a central component of the wellbeing ecosystem, with utilisation increasing steadily year on year. Total engagement increased from 1 028 participants in 2023 to 2 027 in 2024, reaching 4 788 in 2025. Individual employee cases increased from 534 (2023) to 1 166 (2024) and remained at 1 166 in 2025, including 102 dependant cases. The Afrocentric Group engagement rate of 73.1% exceeds the healthcare sector benchmark of 54.4%, reflecting programme maturity, accessibility and effective awareness. Emotional and mental health concerns accounted for 55.7% of cases, followed by financial wellbeing (36.2%) and workplace wellbeing (31.2%). Based on savings of R18.8 million against an investment of R1.4 million, the programme delivered a return on investment of 22.20, demonstrating sustained organisational and financial value.

Mental health support was strengthened through structured and responsive initiatives informed by EAP trends. Mental Health Round Tables have become a sustained programme that was established in 2023 to address stress, family issues, bereavement and anxiety risks identified in the EAP data. Topics tackled in this platform ranged from imposter syndrome, Gen Z and mental health, black tax, and money relationship and mental health.

Research shows that men are less likely than women to seek both general health and mental health support due to stigma and traditional masculine norms¹. We therefore introduced the Quarterly Men's Fireside Conversation Programme to encourage men to talk openly about both their health and mental health in a supportive, stigma-free environment. In 2025, this forum focused on topics that resonate with our male population, including destigmatising mental health dialogue with men, the effects of substance abuse in the workplace, men's health and empowering fathers to partner in their children's wellbeing. Our CEO, Gerald van Wyk, is the main executive sponsor for this programme, demonstrating our leadership buy-in of wellness initiatives. Participation in this forum has grown over time, reflecting increased trust, relevance and early engagement with preventative support.

In 2025, we bolstered the existing financial wellness services, which included onsite financial coaching clinics, debt counselling, and budget streamlining, by incorporating adverse or default removals, credit report fixes, and consolidated loans. The programme focused on budgeting, debt management, rehabilitation and savings behaviour, resulting in average monthly savings of R12 711, average new monthly restructured amount paid towards debt after debt review of R3 956 down from R16 667; 342 total accounts were settled with a total settlement paid of R2 377 274, with a total settling saving of R896 025, reduced averaged loan instalments from R13 210 to R2 295, resulting in average monthly affordability increasing by R10 915. Measurable impact was demonstrated through reduced debt exposure and increased financial literacy. These outcomes reinforce the organisation's commitment to financial resilience as a core component of holistic wellbeing.

Creating a safe and inclusive work environment remained a priority, with utilisation trends informing continued investment. Mommy rooms are in place to support employees returning from maternity leave by enabling dignified and practical workplace reintegration. In response to the impact of gender-based violence, in 2025, we implemented an integrated gender-based violence (**GBV**) awareness and prevention campaign aimed at fostering a safe, supportive and trauma-informed workplace. Branded as "Calling a Family Meeting," the initiative unified the Women's Health Programme, Men's Fireside Conversation and the Mental Health Round Table to address GBV holistically. Key objectives included raising awareness, educating employees on recognising abuse, reducing stigma and ensuring access to support services. With Exco sponsorship, the project featured a flagship webinar and an organisation-wide communication campaign led by our virtual mascot, Zola the Brave. It continued access to a wellness service through our EAP. Through this multi-layered approach, the campaign strengthened collective responsibility, promoted psychological safety, and reinforced the organisation's commitment to protecting and supporting employees affected by GBV.

Wellbeing was further embedded at the organisational health level through leadership and team health interventions, as well as targeted counselling and referral support. Uncertainty created by organisational changes can cause stress and anxiety among employees. During acquisitions and amalgamations in 2025, employees received support that not only helped them cope with change but also built resilience. Resilience, stress awareness, change management, and onsite counselling support were among the training interventions provided to employees and leaders, equipping them with the necessary skills while creating a safe space to process and express their emotions.

Additional wellness education sessions rolled out across the business in response to departmental unique needs and trends covered topics such as psychological safety, bullying in the workplace, emotional intelligence, work-life balance, and disability sensitisation. Through these sessions, we reached 5 932 employees and leaders.

Overall utilisation trends across wellbeing programmes reflect increasing engagement, earlier access to support, and growing confidence in organisational care structures.

Over recent years, the Group has recognised individual and team achievements through the Ignite Recognition Programme. As the organisation aligns initiatives to its refreshed values, recognition efforts are being repositioned, with an interim programme implemented in 2025 to sustain engagement, while a comprehensive Group-wide framework is being developed. The interim programme builds on existing awareness, centres on the Afrocentric Group Citizen Individual Award, and reinforces a culture of belonging, innovation and excellence. It is leader-led, people-centred, inclusive and values-driven, anchored in the Group values of Collaboration, Innovation, Care and Integrity. Five award categories were offered: Customer Champion; Living our Values; Innovation and Creativity, Leader of the Year and Collaboration Excellence (Team award). In addition, a discretionary Chief Executive Award will be conferred, and the selected Group Exco will consider individual award recipients for participation in a prestigious corporate recognition experience.

¹ Banks & Collier, 2025; Mursa et al., 2022, as cited in Sagar Ouriaghli et al., 2020

Recordable incidents rates

	January 2025 to December 2025	July 2024 to December 2024
Fatalities	0	0
Lost day cases	103	25
Restricted duty cases	0	0
Medical treatment cases	40	5
Hours worked	11 039 318	5 804 568
TRIR*	2.59	1.03

* Total recordable incidence rate (TRIR).

Work arrangements fit for our business

To support our transition to a hybrid workplace, we introduced an updated working arrangements policy. Alternatives such as fully remote and hybrid roles form part of our new ways of working as we strive to attract and retain the best talent in the healthcare industry.

The policy allows staff to choose from various modalities in line with operational requirements.

Top Employer 2024/2025

After AfroCentric's first measurement, we are proud to announce that we have attained the status of one of South Africa's Top Employers, as designated by the globally recognised Top Employers Institute for the third year.

The Top Employers Institute's rigorous certification process evaluates organisations based on their performance in crucial human capital domains, including people strategy, work environment, talent acquisition, learning, diversity, equity and inclusion, wellbeing and more.

The Top Employers Institute Certification Programme has certified and recognised over 2 300 Top Employers in 121 countries/regions, positively impacting the lives of over 12 million employees globally.

Online learning

The AfroCentric Online Academy offers online training. Employees have constant virtual access to legislative courses, Company policies and functional training courses.



3 663

online courses were undertaken by

962

employees

(December 2024: 2 601 courses undertaken by 440 employees)

Learnerships

These programmes help candidates earn qualifications and gain valuable work experience.



201

unemployed youth participated in learnership programmes

(December 2024: 8 unemployed youth)



77

learnerships catered to people with disabilities

(December 2024: 57)

Skills development

Sector Education and Training Authority and industry-funded learnership initiatives were implemented and prioritised across the Group.



R6.8 million

(Course cost R3.5 million and stipends R3.3 million)

invested towards developing unemployed learners with disabilities

(December 2024: R1.4 million invested)

Bursaries



We offered study assistance to **30** employees to the value of **R0.9 million**

(December 2024: six employees at R0.2 million)

Internships

These programmes help candidates gain valuable work experience after obtaining a qualification.



32

other learners participated in internship programmes

(December 2024: 32)

40

learnerships formed part of our new employee value proposition, enabling employees' children to participate in learnerships

Participation of staff in the Sanlam Leadership development programmes

Zero

Sanlam Junior Management Programme (SJMP)

Three

Sanlam Middle Management Programme (SMMP)

Two

Sanlam Senior Management Programme (SSMP)

Five

Advanced Leadership Certificate in Design Thinking

Nine

Lead for Performance & Growth (Harvard Programme)

Performance management

AfroCentric uses a balanced scorecard (**BSC**) approach for performance management. The BSC provides management with the tools to drive competitive success. It translates the organisation's strategy into performance measures that provide the framework for a strategic measurement and management system.

When managing individual performance, we aim for a mutually beneficial environment where managers and employees share clear expectations and objectives. Employees participate in all phases of the performance management process and share ownership of the outcomes with the organisation.

Participation in internal programmes

Presentation skills – **eight** participants

Self-mastery – **one** participant

Telephone techniques – **five** participants

Business writing skills – **10** participants

Coaching for superior performance – **53** participants

Stress management – **nine** participants

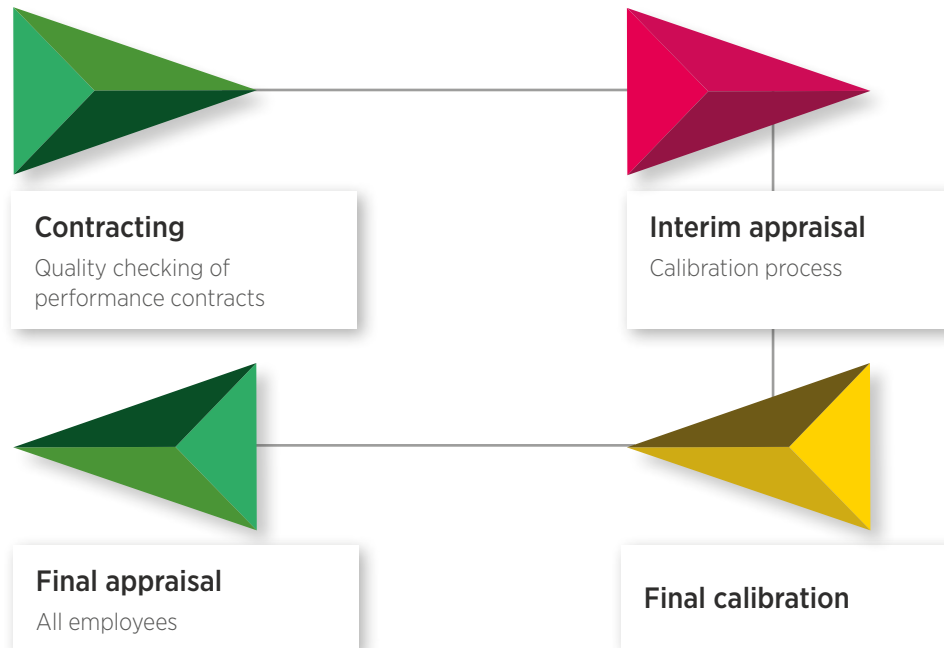
External bursaries

Five medical students

10 nursing students

R2.5 million in total funding

Continuous conversations, feedback and development



Managing performance is not an annual event but a continual process of feedback and collaboration. Performance discussions are therefore expected to cover more than simply performance ratings. Instead, leaders should have regular, meaningful conversations to close performance gaps and enable individuals to understand how their performance and behaviour link to the organisational strategy and how their contributions drive business growth.

Our performance management system aims to enable:

- A streamlined performance management process to direct line managers' time from administrative activities to ongoing performance conversations
- High-quality performance discussions, giving employees more frequent feedback and addressing performance gaps in real time
- The creation of a high-performance culture within the organisation
- Employee-led performance management
- Difficult conversations that drive positive change

Transformation and B-BBEE

As the most transformed health-related business listed in South Africa, we are committed to corporate citizenship, driving transformation and upholding our social licence to operate. The Group invests in healthcare-related companies, and our contribution to healthcare extends beyond our products and services. Our social impact starts within the Group and extends to the suppliers and communities we directly impact, as well as the broader society affected by our health management initiatives.

We value diversity, equity and inclusion and aim to create an environment where differences are valued, as demonstrated by our diverse workforce and supply chain.

B-BBEE creates opportunity while mitigating operational risk



AfroCentric's transformation framework supports national strategic priorities and our purpose of creating sustainable healthcare solutions to enhance our stakeholders' quality of life.

The degree of the Group's alignment of B-BBEE policies and practices to the requirements of the different transformation-related legislation in support of the South African Government's overall economic transformation includes, but is not limited to:

- The NDP 2030
- The National Strategy for Broad-Based Black Economic Empowerment
- The Revised Broad-Based Black Economic Empowerment Act 53 and related amendments

- The Amended Codes of Good Practice on Broad-Based Black Economic Empowerment of 2013
- The Employment Equity Amendment Act of 2014 and associated regulations
- Skills Development Act and related Skills Development Levies Act
- The Preferential Procurement Act

AfroCentric Investment Corporation Limited BEE status

Element	Element total points	FYE 31 December 2024	FYE 30 June 2024
Equity ownership	25.00	25.00	25.00
Management control	19.00	13.92	14.62
Skills development	20.00 (+5 bonus points)	16.58	18.54
Enterprise and supplier development	40.00 (+4 bonus points)	41.27	39.72
SED	5.00	5.00	5.00
Total	109 (+ bonus = 118)	101.77	102.88
B-BBEE level		Level 1	Level 1
AfroCentric Investment Corporation Limited BEE status		135 %	135%

AfroCentric Health (RF) Proprietary Limited BEE status

Element	Element total points	FYE 31 December 2024	FYE 30 June 2024
Equity ownership	25.00	25.00	24.93
Management control	19.00	14.67	14.14
Skills development	20.00 (+5 bonus points)	18.50	18.39
Enterprise and supplier development	40.00 (+4 bonus points)	41.48	44.05
SED	5.00	5.00	5.00
Total	109 (+ bonus = 118)	104.65	102.58
B-BBEE level		Level 1	Level 1
AfroCentric Health (RF) Proprietary Limited BEE Status		135%	135%

Pharmacy Direct Proprietary Limited BEE status

Element	Element total points	FYE 31 December 2024	FYE 30 June 2024
Equity ownership	25.00	25.00	24.93
Management control	19.00	18.14	17.27
Skills development	20.00 (+5 bonus points)	14.86	13.38
Enterprise and supplier development	40.00 (+4 bonus points)	38.36	39.96
SED	5.00	5.00	5.00
Total	109 (+ bonus = 118)	101.37	100.54
B-BBEE level		Level 1	Level 1
Pharmacy Direct Proprietary Limited BEE status		135%	135%

Enterprise development in action

Our enterprise and supplier development initiatives are designed to strengthen the healthcare ecosystem, expand access to quality services, and support sustainable Black-owned businesses that contribute meaningfully to South Africa's health infrastructure. Through targeted financial support and strategic partnership, we enable enterprises to scale, innovate and deliver broader societal impact.

Molecular Imaging and Therapy Limpopo Proprietary Limited (MI&T)

Expanding access to advanced cancer diagnostics in underserved regions



MI&T provides nuclear medicine diagnostics and therapy, delivering critical radiation services to patients living with cancer. The business plays a vital role in expanding access to specialised nuclear treatment for patients in the northern regions of Limpopo and Mpumalanga, reducing the need for long-distance travel to major metropolitan centres.

MI&T holds the distinction of being recognised as the first Black woman-owned PET/CT scan facility globally, marking an important milestone in both healthcare transformation and gender empowerment within highly specialised medical services.

Through an enterprise development grant of R5 247 958.48, AfroCentric is supporting the sustainability and growth of this 100% Black female-owned enterprise, strengthening regional oncology services and contributing to more equitable access to life-saving diagnostics and treatment.

Alma Clinics Proprietary Limited

Bringing affordable primary healthcare closer to communities

Alma Clinics is a nurse-led primary healthcare provider, remotely supported by a doctor and enabled by modern technology to deliver accessible, affordable care. The model is designed to extend quality primary healthcare services to underserved communities while maintaining clinical oversight and efficiency.

Funding support is enabling the establishment of a new clinic in the Hammanskraal area and the expansion of dentistry and renal services at two existing facilities, where demand remains high. This expansion enhances community-level access to preventative and essential healthcare services, reducing pressure on public health facilities.

As a 100% Black-owned enterprise with 75% Black female ownership, Alma Clinics reflects our commitment to inclusive economic participation. Grants of R274 466.67 (enterprise development) and R5 650 696.96 (supplier development) are strengthening both operational capacity and long-term sustainability, supporting expanded reach and improved healthcare outcomes.



MIFS Solutions CC

Driving digital efficiency in healthcare and beyond

MIFS Solutions provides information and communications technology (ICT) services that help businesses manage their finances, stock and ordering processes more efficiently. Its digital tools support accounting, warehouse management and online ordering, enabling businesses to operate more accurately and productively.

Through an enterprise development grant of R808 737.00, AfroCentric is supporting this 100% Black-owned business to expand its capacity and strengthen its service offering. By enabling smoother day-to-day operations, MIFS contributes to improved efficiency and reliability within the broader healthcare value chain.



Ndala ICT Solutions Proprietary Limited

Innovating digital service delivery

Ndala ICT Solutions provides end-to-end ICT services to small, medium and large enterprises across the private and public sectors. The company supports digital transformation through tailored technology solutions that enhance efficiency and service delivery.

Enterprise development funding of R2 580 362.00 is being directed toward the development and piloting of the ConnectPod platform. This innovation aims to improve service delivery by providing clients with access to digital remote consultation facilities in secure and private environments. The initiative has the potential to enhance access to professional services, particularly in contexts where privacy and connectivity present barriers.

As a 100% Black-owned enterprise, Ndala represents the role of digital innovation in strengthening healthcare access and operational effectiveness.



YMS Medical Supplies Proprietary Limited



MEDICAL SUPPLIES (PTY) LTD.
Celebrating 60 years and counting

YMS Medical Supplies operates as a medical devices supplier and pharmacy wholesaler, providing essential health products utilised in healthcare facilities. Reliable supply of quality medical equipment and consumables is fundamental to patient care and operational continuity.

Funding of R2 005 054.95 is being used to acquire demo machines to expand sales across multiple product lines, strengthening long-term commercial sustainability.

As a 100% Black-owned enterprise, YMS contributes to supplier diversity within the healthcare sector while reinforcing the resilience of the medical supply chain.

Environmental performance

Climate change

Climate change poses the greatest health risk to humanity. The Intergovernmental Panel on Climate Change states that to avoid severe health consequences and save millions of lives, global temperature rise must be kept within 1.5°C.

To reduce our environmental impact, we focus on reducing resource use, minimising consumption, and preventing pollution.

Greenhouse gas (GHG) emissions

The following table provides a detailed overview of AfroCentric Group's January to December 2025 GHG inventory, including the percentage contribution of each category and sub-category to the total GHG inventory. The scope of this assessment includes 26 office clusters distributed across the country.

This report does not provide the Mandatory GHG Reporting emissions information in the format required by the South African Department of Forestry, Fisheries and the Environment (**DFFE**) in terms of the National GHG Emission Reporting Regulations. In previous reporting periods, our operations were below the department's reporting thresholds for listed activities. Following the integration of the Group's activities, we have been engaging with the department to determine whether the reporting thresholds for DFFE reporting requirements have been triggered.

The combustion of diesel in back-up generators is the listed activity most likely to trigger reporting under the regulations. The reporting threshold is based on the total installed capacity of 10 MWh for back-up diesel generators across the South African office clusters.

This is the first year in which emissions associated with the upstream manufacturing of pharmaceutical products and related packaging materials have been comprehensively quantified and included. The total GHG inventory for January to December 2025 amounted to 94 126 tCO₂e. Within this period, an additional 102 tCO₂e was emitted from fugitive emissions (R22 refrigerant gas), which accounts for less than 1% of total emissions and is reported outside of scope according to the GHG Protocol.

A material increase in reported emissions is observed when compared to the previous reporting year. Total emissions increased when upstream pharmaceutical emissions were included, representing a 267% increase. This increase is not attributable to proportional growth in operational activity, but rather to a significant enhancement in the completeness of Scope 3 reporting within the pharmaceutical cluster.

Direct (Scope 1) GHG emissions amounted to 188 tCO₂e. Total indirect emissions amounted to 93 835 tCO₂e in the reporting period, including emissions related to electricity purchased, transportation, waste management, and the purchase of goods used by the organisation. However, emissions from purchased electricity (Scope 2) contributed 7 648 tCO₂e to the overall indirect emissions for the reporting period.

Emission category	Emission source	Jan 2025 – Dec 2025 emissions	% Contribution
Scope 1	Stationary Diesel Combustion	34	0
	Mobile Petrol Combustion	153	0
	Stationary LPG	2	0
Total Scope 1 emissions		188	0
Scope 2	Purchased electricity	7 648	8
Total Scope 2 emissions		7 648	8
Scope 3 – Category 1: Purchased goods and services	Water	0	0.0
	Food	151	0
	Stationary	59	0
	Packaging Material	5 027	5
	Pharmaceutical Products	55 629	59
Scope 3 – Category 3: Fuel- and energy-related activities	Production of Diesel	0	
	Production of Petrol	0	
	Production of LPG	0	
	Fuels used in the Production of Electricity	651	1
	Transmission & Distribution Losses	0	
Scope 3 – Category 4: Upstream Transportation and Distribution	Transportation of Pharmaceuticals	5 601	6
Scope 3 – Category 5: Waste generated in operations	General Waste – Sent to Landfill	102	0
	Recycled Waste – Paper, Plastic, Cardboard, Cans, etc.	1	
	Food Waste – Bokashi	0	
	Sewerage	35	0
Scope 3 – Category 6: Business travel	Domestic Flights – Economy	125	0
	International Flights – Economy	448	0
	Domestic Flights – Business	929	1
	International Flights – Business	215	0
	Domestic Connecting Flights – First Class	1	–
	International Flights – First Class	2	
	Accommodation	87	0
	Rental Vehicles	55	0
Scope 3 – Category 7: Employee Commuting	Commuting via Bus	1 869	2
	Commuting via Car	13 992	15
	Commuting via Mini-Bus Taxi	704	1
	Commuting via Train	17	0
	Commuting via Motorcycle	35	0
	Commuting via SUV/Bakkie	450	0
Total Scope 3 emissions		86 187	92
Outside of scope	Fugitive Emissions (R22 refrigerant)	102	0
Total emissions		94 126	100

Emission category mapping between GHG Protocol and SANS 14064-1:2021

SANS 14064-1:2021	GHG protocol	Description
1. Direct GHG emissions and removals	Scope 1	Direct emissions
2. Indirect GHG emissions from imported energy emissions	Scope 2 Scope 3(3)	Energy indirect emissions Fuel and energy-related activities
3. Indirect GHG emissions from transportation	Scope 3(4) Scope 3(6) Scope 3(7) Scope 3(9)	Upstream transportation and distribution Business travel Employee commuting Downstream transportation and distribution
4. Indirect GHG emissions from products used by the organisation	Scope 3(1) Scope 3(2)	Purchased goods and services Capital goods
5. Indirect GHG emissions associated with the use of products from the organisation	Scope 3(10) Scope 3(11) Scope 3(12)	Processing of sold products Use of sold products End-of-life treatment of sold products
6. Indirect GHG emissions from other sources	Scope 3(5) Scope 3(8) Scope 3(13) Scope 3(14) Scope 3(15)	Waste generated in operations Upstream leased assets Downstream leased assets Franchises Investments

Energy mix

While our current energy mix is anchored in non-renewable sources, as we chart our path forward, we are committed to transitioning to utilise more renewable sources, aligning ourselves with global sustainability standards, and playing our role in mitigating the impacts of climate change.

In pursuit of a diversified and sustainable energy mix, solar installations have been implemented to supplement electricity usage within our owned buildings, DENIS and Block E in Roodepoort. In alignment with agreements established with the landlord, the leased buildings in Roodepoort also incorporate solar power into their energy supply. The cumulative solar energy generation for the period January to December 2025 amounted to 707.83 MWh, underscoring our commitment to renewable energy integration and reduced reliance on conventional sources.

Water

We recognise that water is a finite resource and that many of our operations are located in water-stressed areas. Responsible water stewardship is therefore both an ethical obligation and a business imperative. By actively managing our water consumption, we mitigate operational, regulatory and reputational risks associated with water use, while also improving efficiency and reducing operating costs.

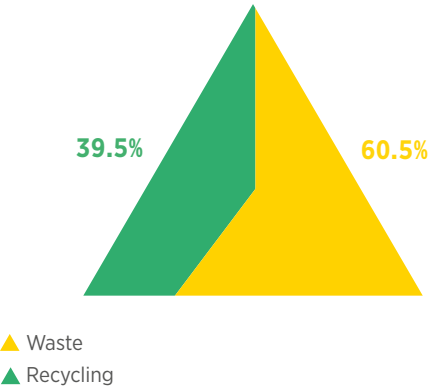
Water (KL)	2025
Withdrawal (municipal)	63 268
Groundwater (borehole) withdrawals	8 983
Recycled water (greywater)	0
Total water consumption from water-stressed areas	72 251
Total water consumption	72 251
Average water intensity ratio (KL/m ²)	28.55

Our borehole water system has been successfully installed and is fully operational at the Pharmacy Direct Louwlandia facility as well as the Roodepoort Head Office, supplying Blocks A, B, C, and D. From January to December 2025, a total of 8 983 kilolitres of borehole water was used for domestic supply at these facilities, ensuring a sustainable and reliable water source. Applications for the required water-use licences have been submitted to the local municipalities and the National Water custodians, and we are currently awaiting their decisions. This initiative represents a significant step toward enhancing resource efficiency and sustainability across our operations.

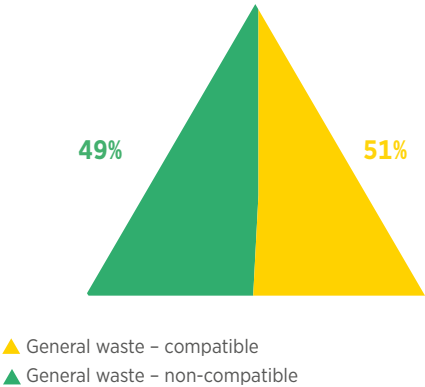
Pollution and waste

We strive to reduce our impact on the natural environment through effective waste management, which can also reduce operational and capital costs through improved efficiencies. In contrast, a failure to manage waste can result in reputational damage and increase potential financial and legal liability costs.

Waste vs recycling – 2025 (%)



Breakdown of non-recyclables – 2025 (%)



Breakdown of recyclables – 2025 (%)

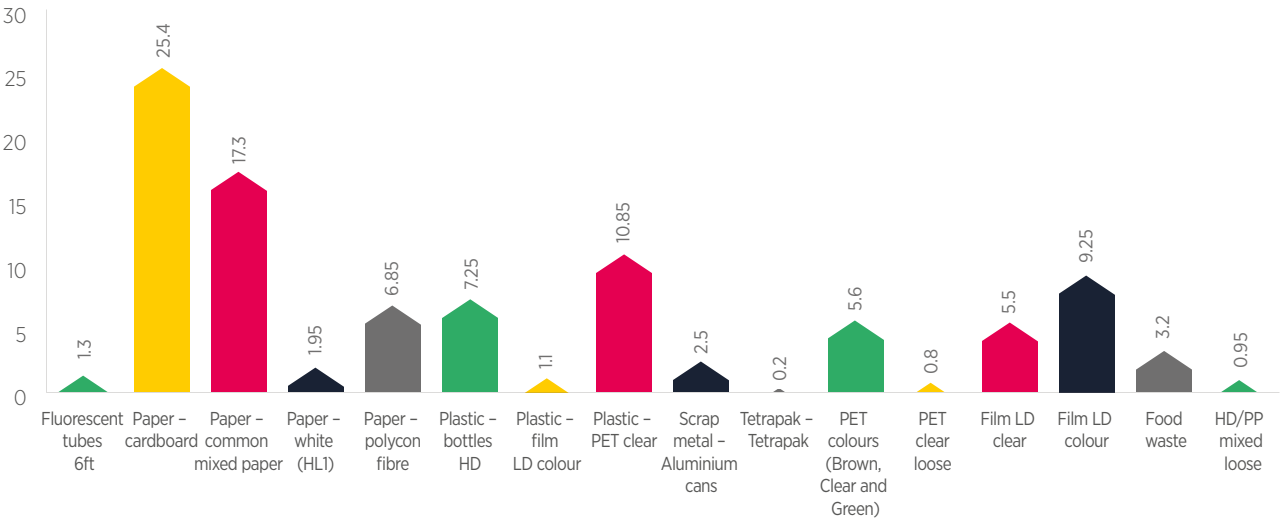


Table of abbreviations

ADS	AfroCentric Distribution Services
AI	Artificial intelligence
AIDS	Acquired immune deficiency syndrome
B-BBEE	Broad-Based Black Economic Empowerment
BHF	Board of Healthcare Funders
BLSA	Business Leadership South Africa
BUSA	Business Unity South Africa
CCMDD	Central Chronic Medicines Dispensing and Distribution
CEO	Chief Executive Officer
CES	Customer Effort Score
CFO	Chief Financial Officer
CIO	Chief Information Officer
CMS	Council for Medical Schemes
COSO	Committee of Sponsoring Organizations of the Treadway Commission
CRO	Chief Risk Officer
DBC	Document-based care
DENIS	Dental Information Systems
DFFE	Department of Forestry, Fisheries and the Environment
EAP	Employee assistance programme
ERM	Enterprise risk management
ESG	Environmental, social and governance
FWA	Fraud, waste and abuse
GBV	Gender-based violence
GEMS	Government Employees Medical Scheme

GHG	Greenhouse gas
HCP	Healthcare Professional Portal
HEPS	Headline earnings per share
HIV	Human immunodeficiency virus
ICT	Information and communications technology
IFRS	International Financial Reporting Standards
ISO	International Organization for Standardization
IT	Information technology
JSE	Johannesburg Stock Exchange
KPMG	KPMG Incorporated
LCBOs	Low Cost Benefit Options
NDoh	National Department of Health
NEHAWU	National Education, Health and Allied Workers' Union
NHA	Neil Harvey and Associates
NHI	National Health Insurance
PBM	Pharmacy Benefit Management
PHI	Primary Health Insurance
POLMED	South African Police Service Medical Scheme
RFP	Request for proposal
ROE	Return on equity
ROIC	Return on invested capital
SALU	South African Legal Union
SENS	Stock Exchange News Service
VBC	Value-Based-Care

Notes

Company information

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1988/000570/06

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